Paper 2(midwifery), FQE, Kechn, 260 MCQS, 70 SAQS, 70 LAQS, Answers and Rationales, Jan 2024 predictions

Kenya Registered Community Health Nursing (Kenya Medical Training College)

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Subject 1: Reproductive Health

Q1: During a reproductive health workshop, a participant asks about the significance of family planning in preventing maternal mortality. Which of the following statements best explains the relationship between family planning and maternal mortality reduction?

A. Family planning directly reduces maternal mortality by addressing complications during childbirth.

B. Family planning indirectly contributes to maternal mortality reduction by promoting healthy spacing of pregnancies.

C. Family planning has no impact on maternal mortality as it primarily focuses on contraception.

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D. Family planning only reduces maternal mortality in low-income countries.

Rationale: Option B is correct. Healthy spacing of pregnancies through family planning reduces the risk of complications, ensuring maternal well-being. While family planning may indirectly impact maternal mortality, it doesn't directly address complications during childbirth (Option A).

Q2: A community health nurse is educating adolescents about sexually transmitted infections (STIs). What counseling strategy is most effective in preventing STIs among this population?

A. Encouraging abstinence-only education.

B. Promoting consistent and correct condom use.

C. Advising on the use of emergency contraception.

D. Providing information on alternative reproductive health methods.

Rationale: Option B is correct. Promoting consistent and correct condom use is an effective strategy to prevent STIs among adolescents. Abstinence-only education (Option A) may not be realistic, and emergency contraception (Option C) does not protect against STIs.

Q3: A married couple seeks advice on choosing a contraceptive method after having two children. The wife is concerned about long-term fertility implications. Which contraceptive method would be most suitable for them?

A. Oral contraceptives.

B. Intrauterine device (IUD).

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C. Condoms.

D. Tubal ligation.

Rationale: Option C is correct. Condoms provide effective contraception without affecting long-term fertility. Tubal ligation (Option D) is a permanent method, while oral contraceptives (Option A) and IUDs (Option B) can influence fertility temporarily.

Q4: In a community health outreach program, a nurse encounters resistance to family planning methods due to cultural beliefs. How can the nurse effectively address this challenge?

A. Implementing the chosen family planning method despite cultural objections.

B. Collaborating with community leaders to understand and respect cultural perspectives.

C. Ignoring cultural beliefs and focusing on medical evidence.

D. Educating the community on the superiority of modern medicine.

Rationale: Option B is correct. Collaborating with community leaders demonstrates cultural sensitivity and facilitates understanding. Ignoring cultural beliefs (Option C) may lead to resistance, while imposing chosen methods (Option A) disregards cultural autonomy.

Q5: A teenage girl is seeking reproductive health advice. Which legal and ethical principle guides the healthcare provider's response in maintaining confidentiality?

A. Autonomy.

B. Beneficence.

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C. Justice.

D. Fidelity.

Rationale: Option A is correct. Respecting autonomy involves maintaining confidentiality, especially in sensitive matters like reproductive health for adolescents. Beneficence (Option B) and justice (Option C) are important but not directly related to confidentiality in this context.

Q6: A community health nurse is conducting a reproductive health workshop and emphasizes the importance of preconception care. What key component should be included in preconception care?

A. Routine vaccination.

B. Genetic counseling.

C. Weight loss programs.

D. Immediate initiation of prenatal vitamins.

Rationale: Option B is correct. Genetic counseling is a crucial component of preconception care to assess and manage genetic risks. While routine vaccination (Option A) is essential, it may not be specific to preconception. Weight loss programs (Option C) and immediate prenatal vitamin initiation (Option D) may not address individual genetic concerns.

Q7: A pregnant woman is diagnosed with gestational diabetes. What is the primary goal of managing gestational diabetes during pregnancy?

A. Eliminating the need for insulin therapy.

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B. Minimizing fetal growth.

C. Preventing maternal hypertension.

D. Maintaining optimal blood glucose levels for maternal and fetal health.

Rationale: Option D is correct. The primary goal in managing gestational diabetes is to maintain optimal blood glucose levels for the well-being of both the mother and the fetus. Eliminating the need for insulin therapy (Option A) may not be achievable in all cases, and minimizing fetal growth (Option B) is not the primary goal.

Q8: A pregnant woman is experiencing severe morning sickness. What non-pharmacological intervention is most appropriate for managing this condition?

A. Administering antiemetic medication.

B. Encouraging increased fluid intake.

C. Recommending bed rest.

D. Advising a high-protein diet.

Rationale: Option B is correct. Increasing fluid intake is a non-pharmacological intervention to manage severe morning sickness. While antiemetic medication (Option A) may be prescribed in severe cases, bed rest (Option C) and a high-protein diet (Option D) are not primary interventions for morning sickness.

Q9: A pregnant woman reports persistent back pain during the second trimester. What exercise recommendation would be most beneficial for alleviating back pain?

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A. High-intensity aerobic exercises.

B. Heavy weightlifting.

C. Low-impact exercises such as swimming or walking.

D. Avoiding all physical activity.

Rationale: Option C is correct. Low-impact exercises, such as swimming or walking, are beneficial for alleviating back pain during pregnancy. High-intensity aerobic exercises (Option A) and heavy weightlifting (Option B) may exacerbate back pain. Avoiding all physical activity (Option D) is not recommended.

Q10: A pregnant woman with a history of preterm birth is at increased risk for a recurrent preterm birth. What intervention is most effective in reducing the risk of preterm labor in this population?

A. Early initiation of prenatal care.

B. Bed rest throughout pregnancy.

C. Prophylactic administration of tocolytic drugs.

D. Continuous electronic fetal monitoring.

Rationale: Option C is correct. Prophylactic administration of tocolytic drugs can be effective in reducing the risk of preterm labor in women with a history of preterm birth. Early initiation of prenatal care (Option A) is important, but tocolytic drugs are more specific to preventing preterm labor. Bed rest (Option B) is not universally recommended, and continuous electronic fetal monitoring (Option D) is not a preventive measure for preterm labor.

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Q11: A pregnant woman presents with hypertension. What dietary advice is most appropriate to manage hypertension during pregnancy?

A. High-sodium diet.

B. Low-calorie diet.

C. Adequate calcium intake.

D. Reduced caffeine consumption.

Rationale: Option C is correct. Adequate calcium intake is associated with a lower risk of developing hypertension during pregnancy. High-sodium diets (Option A) can exacerbate hypertension, and low-calorie diets (Option B) may not provide essential nutrients. While reduced caffeine consumption (Option D) is generally advisable, it is not the primary dietary recommendation for managing hypertension.

Q12: A pregnant woman with a history of recurrent miscarriages is concerned about the risk of another miscarriage. What intervention is most appropriate to address this concern?

A. Prophylactic administration of progesterone.

B. Early induction of labor.

C. Continuous fetal monitoring.

D. Strict bed rest throughout pregnancy.

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Rationale: Option A is correct. Prophylactic administration of progesterone is a recognized intervention to reduce the risk of recurrent miscarriages. Early induction of labor (Option B), continuous fetal monitoring (Option C), and strict bed rest (Option D) are not established interventions for preventing recurrent miscarriages.

Q13: A pregnant woman in her third trimester reports edema in her ankles and hands. What nursing intervention is most appropriate to manage edema during pregnancy?

A. Encouraging increased sodium intake.

B. Elevating the legs when sitting or lying down.

C. Administering diuretic medications.

D. Initiating immediate labor induction.

Rationale: Option B is correct. Elevating the legs when sitting or lying down is a non-pharmacological intervention to manage edema during pregnancy. Increased sodium intake (Option A) may exacerbate edema, diuretic medications (Option C) are generally avoided during pregnancy, and immediate labor induction (Option D) is not indicated for mild edema.

Q14: A pregnant woman requests information on the use of herbal supplements for improving fertility. What is the nurse's most appropriate response?

A. Encouraging the use of herbal supplements to boost fertility.

B. Advising against the use of herbal supplements due to potential risks.

C. Recommending a specific herbal supplement without consulting a healthcare provider.

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D. Suggesting a trial of multiple herbal supplements simultaneously.

Rationale: Option B is correct. Advising against the use of herbal supplements is the most appropriate response due to potential risks and lack of regulation. Encouraging their use (Option A), recommending a specific supplement without consulting a healthcare provider (Option C), and suggesting multiple supplements simultaneously (Option D) can pose health risks.

Q15: A pregnant woman with a history of multiple cesarean sections expresses interest in attempting a vaginal birth after cesarean (VBAC). What information should the nurse provide regarding the risks and benefits of VBAC?

A. Emphasizing the high success rate and minimal risks associated with VBAC.

B. Discouraging any consideration of VBAC due to inherent risks.

C. Providing balanced information on both the benefits and risks of VBAC.

D. Encouraging immediate scheduling of a repeat cesarean section.

Rationale: Option C is correct. Providing balanced information on both the benefits and risks of VBAC allows the woman to make an informed decision. Emphasizing only the success rate and minimal risks (Option A) or discouraging consideration altogether (Option B) lacks a comprehensive approach. Encouraging immediate scheduling of a repeat cesarean section (Option D) may not align with the woman's preferences and autonomy.

Q16: A pregnant woman reports experiencing "lightning crotch" or sharp, shooting pains in the pelvic area. What is the most accurate explanation for this phenomenon?

A. Sign of impending labor.

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B. Compression of pelvic nerves.

C. Fetal movement causing discomfort.

D. Effect of hormonal changes on ligaments.

Rationale: Option B is correct. "Lightning crotch" is often attributed to the compression of pelvic nerves, causing sharp, shooting pains. While it can occur in late pregnancy, it is not a direct sign of impending labor (Option A). Fetal movement (Option C) and hormonal changes affecting ligaments (Option D) may contribute to pelvic discomfort but are not specific to "lightning crotch."

Q17: A pregnant woman in her second trimester is diagnosed with placenta previa during a routine ultrasound. What nursing intervention is a priority in managing this condition?

A. Encouraging increased physical activity.

B. Preparing for immediate cesarean section.

C. Advising on the use of tocolytic drugs.

D. Strict bed rest until delivery.

Rationale: Option D is correct. Strict bed rest until delivery is a priority in managing placenta previa to reduce the risk of bleeding. Increased physical activity (Option A) and tocolytic drugs (Option C) are generally avoided. Immediate cesarean section (Option B) may be necessary in case of severe bleeding but is not the primary intervention.

Q18: A pregnant woman at 28 weeks' gestation reports feeling dizzy and lightheaded when lying on her back. What is the most likely explanation for this symptom?

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A. Orthostatic hypotension.

B. Gestational diabetes.

C. Supine hypotensive syndrome.

D. Iron-deficiency anemia.

Rationale: Option C is correct. Supine hypotensive syndrome, caused by pressure on the vena cava when lying on the back, can result in dizziness and lightheadedness. Orthostatic hypotension (Option A), gestational diabetes (Option B), and iron-deficiency anemia (Option D) may cause different symptoms but are not directly related to the described symptom when lying on the back.

Q19: A pregnant woman is diagnosed with cholestasis of pregnancy. What fetal complication is associated with this condition?

A. Macrosomia.

B. Preterm birth.

C. Neural tube defects.

D. Intrauterine growth restriction.

Rationale: Option D is correct. Cholestasis of pregnancy is associated with an increased risk of intrauterine growth restriction. Macrosomia (Option A), preterm birth (Option B), and neural tube defects (Option C) are not commonly linked to this condition.

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Q20: A pregnant woman at term presents with decreased fetal movements. What nursing action is a priority in assessing fetal well-being?

A. Immediate induction of labor.

B. Nonstress test (NST).

C. Administering tocolytic drugs.

D. Performing a biophysical profile.

Rationale: Option D is correct. Performing a biophysical profile is a comprehensive assessment that includes fetal movements, amniotic fluid levels, and other parameters, providing a more thorough evaluation of fetal well-being. Immediate induction of labor (Option A), NST (Option B), and tocolytic drugs (Option C) are not primary interventions for assessing fetal well-being.

Subject 2: Normal Pregnancy

Q21: A pregnant woman in her first trimester is concerned about the risk of miscarriage. What information should the nurse prioritize in educating the woman about the factors influencing the risk of miscarriage?

A. Importance of bed rest.

B. Genetic factors.

C. Maternal age only.

D. Dietary habits.

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Rationale: Option B is correct. Genetic factors play a significant role in the risk of miscarriage. Maternal age (Option C) is important but not the sole determinant. Bed rest (Option A) is not proven to prevent miscarriage, and while dietary habits (Option D) are relevant, genetic factors have a more direct impact.

Q22: A pregnant woman at 32 weeks' gestation is concerned about the possibility of preterm labor. What symptom is most indicative of preterm labor in this woman?

A. Occasional back pain.

B. Braxton Hicks contractions.

C. Increased vaginal discharge.

D. Regular, painful contractions.

Rationale: Option D is correct. Regular, painful contractions are indicative of preterm labor. Occasional back pain (Option A), Braxton Hicks contractions (Option B), and increased vaginal discharge .

Q23: A pregnant woman in her third trimester experiences sudden swelling in her face and hands. What assessment finding is most concerning for preeclampsia?

A. Mild headache.

B. Elevated blood pressure.

C. Presence of pedal edema.

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D. Increased fetal movement.

Rationale: Option B is correct. Elevated blood pressure is a key indicator of preeclampsia, a serious hypertensive disorder during pregnancy. Mild headache (Option A), pedal edema (Option C), and increased fetal movement (Option D) are not specific to preeclampsia and may occur in normal pregnancies.

Q24: A pregnant woman with a history of gestational diabetes is concerned about the impact on her baby. What is the primary fetal risk associated with gestational diabetes?

A. Neural tube defects.

B. Macrosomia.

C. Preterm birth.

D. Intrauterine growth restriction.

Rationale: Option B is correct. Gestational diabetes is associated with an increased risk of macrosomia (large birth weight), which can pose delivery complications. Neural tube defects (Option A), preterm birth (Option C), and intrauterine growth restriction (Option D) are not typically associated with gestational diabetes.

Q25: A pregnant woman in her second trimester reports sudden abdominal pain and vaginal bleeding. What complication should the nurse consider as a potential cause?

A. Placental abruption.

B. Ectopic pregnancy.

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C. Uterine fibroids.

D. Round ligament pain.

Rationale: Option A is correct. Sudden abdominal pain and vaginal bleeding are concerning for placental abruption, a serious complication during pregnancy. Ectopic pregnancy (Option B), uterine fibroids (Option C), and round ligament pain (Option D) typically do not present with both symptoms simultaneously.

Q26: A pregnant woman with a history of preterm birth is prescribed 17-alpha hydroxyprogesterone caproate (17P) injections. What is the primary purpose of this medication?

A. Fetal neuroprotection.

B. Prevention of gestational diabetes.

C. Reduction of preterm birth risk.

D. Enhancement of fetal lung development.

Rationale: Option C is correct. 17-alpha hydroxyprogesterone caproate (17P) injections are administered to reduce the risk of preterm birth in women with a history of preterm birth. Fetal neuroprotection (Option A), prevention of gestational diabetes (Option B), and enhancement of fetal lung development (Option D) are not primary indications for this medication.

Q27: A pregnant woman in her second trimester is diagnosed with gestational hypertension. What is the primary goal of management for gestational hypertension?

A. Achieving normoglycemia.

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B. Maintaining optimal fetal heart rate.

C. Preventing maternal seizures.

D. Controlling blood pressure within normal limits.

Rationale: Option D is correct. The primary goal in managing gestational hypertension is to control blood pressure within normal limits to prevent maternal complications. Achieving normoglycemia (Option A), maintaining optimal fetal heart rate (Option B), and preventing maternal seizures (Option C) are not the primary objectives in the management of gestational hypertension.

Q28: A pregnant woman at term presents with decreased fetal movements. What nursing action is a priority in assessing fetal well-being?

A. Immediate induction of labor.

B. Nonstress test (NST).

C. Administering tocolytic drugs.

D. Performing a biophysical profile.

Rationale: Option D is correct. Performing a biophysical profile is a comprehensive assessment that includes fetal movements, amniotic fluid levels, and other parameters, providing a more thorough evaluation of fetal well-being. Immediate induction of labor (Option A), NST (Option B), and tocolytic drugs (Option C) are not primary interventions for assessing fetal well-being.

Q29: A pregnant woman in her third trimester reports frequent episodes of heartburn. What non-pharmacological advice is most appropriate to manage this symptom?

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A. Avoiding all spicy foods.

B. Consuming a large glass of milk before bedtime.

C. Lying down immediately after meals.

D. Eating small, frequent meals.

Rationale: Option D is correct. Eating small, frequent meals is a non-pharmacological approach to manage heartburn during pregnancy. Avoiding all spicy foods (Option A), consuming a large glass of milk before bedtime (Option B), and lying down immediately after meals (Option C) may not effectively alleviate heartburn symptoms.

Q30: A pregnant woman with a history of recurrent miscarriages is concerned about the risk of another miscarriage. What intervention is most appropriate to address this concern?

A. Prophylactic administration of progesterone.

B. Early induction of labor.

C. Continuous fetal monitoring.

D. Strict bed rest throughout pregnancy.

Rationale: Option A is correct. Prophylactic administration of progesterone is a recognized intervention to reduce the risk of recurrent miscarriages. Early induction of labor (Option B), continuous fetal monitoring (Option C), and strict bed rest (Option D) are not established interventions for preventing recurrent miscarriages.

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Q31: A primigravida in early labor expresses concerns about the pain associated with childbirth. What non-pharmacological comfort measure should the nurse recommend to help manage pain during early labor?

A. Epidural anesthesia.

B. Breathing exercises.

C. Intramuscular analgesics.

D. Continuous fetal monitoring.

Rationale: Option B is correct. Breathing exercises, such as slow and rhythmic breathing, can be effective in managing pain during early labor. Epidural anesthesia (Option A) and intramuscular analgesics (Option C) are pharmacological interventions. Continuous fetal monitoring (Option D) is a monitoring technique, not a pain management measure.

Q32: A pregnant woman in active labor requests to change positions frequently. What is the primary benefit of encouraging position changes during labor?

A. Reduction of maternal fatigue.

B. Prevention of postpartum hemorrhage.

C. Facilitation of continuous fetal monitoring.

D. Promotion of epidural anesthesia effectiveness.

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Rationale: Option A is correct. Encouraging position changes during labor helps reduce maternal fatigue, enhance comfort, and promote effective contractions. Prevention of postpartum hemorrhage (Option B), facilitation of continuous fetal monitoring (Option C), and promotion of epidural anesthesia effectiveness (Option D) are not the primary benefits of position changes during labor.

Q33: A pregnant woman in active labor presents with meconium-stained amniotic fluid. What nursing intervention is a priority in managing this situation?

A. Administering oxytocin to augment labor.

B. Placing the woman in a lateral position.

C. Initiating continuous electronic fetal monitoring.

D. Preparing for immediate cesarean section.

Rationale: Option C is correct. Initiating continuous electronic fetal monitoring is a priority in managing meconium-stained amniotic fluid to closely monitor fetal well-being. Administering oxytocin (Option A), placing the woman in a lateral position (Option B), and preparing for immediate cesarean section (Option D) are not the initial interventions for this situation.

Q34: A laboring woman is experiencing back pain during contractions. What nursing intervention is most effective in providing relief for this type of pain?

A. Administering intramuscular analgesics.

B. Encouraging the woman to lie on her back.

C. Applying counterpressure to the lower back.

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D. Recommending frequent position changes.

Rationale: Option C is correct. Applying counterpressure to the lower back can be effective in providing relief for back pain during contractions. Intramuscular analgesics (Option A) are pharmacological options. Lying on the back (Option B) may exacerbate back pain. Frequent position changes (Option D) are generally helpful but may not specifically address back pain.

Q35: A laboring woman at 8 cm dilation requests an epidural for pain relief. What is the nurse's most appropriate response based on the woman's cervical dilation?

A. Administering the epidural immediately.

B. Explaining that epidural anesthesia is not effective at this stage.

C. Offering alternative pain management options.

D. Suggesting relaxation techniques instead.

Rationale: Option A is correct. At 8 cm dilation, the woman is in active labor, and it is appropriate to administer an epidural for pain relief. Explaining that epidural anesthesia is not effective at this stage (Option B) is inaccurate. Offering alternative pain management options (Option C) may be appropriate, but the woman's request for an epidural should be addressed. Suggesting relaxation techniques (Option D) may not provide sufficient pain relief at this stage.

Q36: A laboring woman is not progressing in the second stage of labor, despite adequate contractions. What nursing intervention is most appropriate to facilitate descent?

A. Encouraging the woman to push harder.

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B. Administering intravenous opioids for relaxation.

C. Recommending a change in position.

D. Initiating oxytocin infusion to increase contractions.

Rationale: Option C is correct. Recommending a change in position, such as hands and knees or side-lying, can facilitate descent in the second stage of labor. Encouraging the woman to push harder (Option A) may not be effective and can lead to exhaustion. Administering intravenous opioids for relaxation (Option B) may hinder effective pushing. Initiating oxytocin infusion (Option D) is not indicated for inadequate descent without evidence of inadequate contractions.

Q37: A laboring woman with an epidural reports sudden shortness of breath and difficulty breathing. What is the priority nursing action?

A. Administering oxygen via mask.

B. Assessing blood pressure and heart rate.

C. Notifying the healthcare provider.

D. Increasing the rate of the epidural infusion.

Rationale: Option B is correct. Assessing blood pressure and heart rate is a priority to determine if the symptoms are related to a potential complication such as hypotension, a common side effect of epidural anesthesia. Administering oxygen (Option A) may be necessary, but assessment takes precedence. Notifying the healthcare provider (Option C) should occur after initial assessment. Increasing the rate of the epidural infusion (Option D) is not appropriate without first assessing the woman's vital signs.

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Q38: A laboring woman with a previous cesarean section expresses a desire for a trial of labor after cesarean (TOLAC). What key factor should the nurse consider in assessing the woman's eligibility for TOLAC?

A. Number of previous cesarean sections.

B. Maternal age.

C. Presence of diabetes.

D. Type of uterine incision from the previous cesarean.

Rationale: Option D is correct. The type of uterine incision from the previous cesarean section is a key factor in assessing eligibility for TOLAC. The number of previous cesarean sections (Option A), maternal age (Option B), and the presence of diabetes (Option C) are important considerations but not the primary factors in determining eligibility.

Q39: A laboring woman experiences a uterine rupture. What clinical manifestation is most indicative of this serious complication?

A. Intermittent contractions.

B. Prolonged deceleration of fetal heart rate.

C. Vaginal bleeding.

D. Maternal hypotension and shock.

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Rationale: Option D is correct. Maternal hypotension and shock are indicative of uterine rupture, a serious complication during labor. Intermittent contractions (Option A), prolonged deceleration of fetal heart rate (Option B), and vaginal bleeding (Option C) may occur with other complications but are not specific to uterine rupture.

Q40: A laboring woman in the second stage suddenly experiences shoulder pain. What nursing action is a priority in managing this situation?

A. Assessing for fetal distress.

B. Preparing for an emergency cesarean section.

C. Encouraging the woman to push harder.

D. Applying suprapubic pressure.

Rationale: Option B is correct. Sudden shoulder pain may indicate shoulder dystocia, a potentially serious complication. Preparing for an emergency cesarean section is a priority in managing this situation. Assessing for fetal distress (Option A), encouraging the woman to push harder (Option C), and applying suprapubic pressure (Option D) are important but may not address the potential urgency of shoulder dystocia.

Subject 4: Normal Baby

Q41: A newborn is observed to have acrocyanosis. What nursing intervention is most appropriate for managing this common condition?

A. Initiating immediate phototherapy.

B. Administering intravenous antibiotics.

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C. Encouraging skin-to-skin contact.

D. Administering supplemental oxygen.

Rationale: Option C is correct. Encouraging skin-to-skin contact helps regulate the newborn's temperature and is an appropriate intervention for acrocyanosis. Initiating immediate phototherapy (Option A) is not necessary for acrocyanosis. Administering intravenous antibiotics (Option B) and supplemental oxygen (Option D) are not indicated for this benign condition.

Q42: A newborn exhibits jaundice within the first 24 hours of life. What type of jaundice is most likely present?

A. Physiological jaundice.

B. Breastfeeding jaundice.

C. Hemolytic jaundice.

D. Biliary atresia.

Rationale: Option C is correct. Hemolytic jaundice, caused by excessive breakdown of red blood cells, can present within the first 24 hours. Physiological jaundice (Option A) typically appears after 24 hours. Breastfeeding jaundice (Option B) and biliary atresia (Option D) are not common within the first 24 hours.

Q43: A newborn is assessed to have a single umbilical artery. What congenital anomaly is commonly associated with this finding?

A. Down syndrome.

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B. Tetralogy of Fallot.

C. Trisomy 18.

D. Renal anomalies.

Rationale: Option D is correct. A single umbilical artery is commonly associated with renal anomalies. Down syndrome (Option A), Tetralogy of Fallot (Option B), and Trisomy 18 (Option C) are not typically linked to this umbilical cord anomaly.

Q44: A newborn is diagnosed with a cephalohematoma. What complication is most concerning for the nurse to monitor in this infant?

A. Hypertension.

B. Hypotension.

C. Jaundice.

D. Hyperactivity.

Rationale: Option C is correct. Cephalohematoma can lead to jaundice due to the breakdown of blood within the hematoma. Hypertension (Option A), hypotension (Option B), and hyperactivity (Option D) are not typically associated with cephalohematoma.

Q45: A newborn is identified with a positive Barlow test during a routine examination. What is the primary concern associated with a positive Barlow test?

A. Developmental dysplasia of the hip (DDH).

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B. Spina bifida.

C. Cleft lip and palate.

D. Patent ductus arteriosus.

Rationale: Option A is correct. A positive Barlow test is concerning for developmental dysplasia of the hip (DDH). Spina bifida (Option B), cleft lip and palate (Option C), and patent ductus arteriosus (Option D) are not assessed through the Barlow test.

Q46: A newborn has a visible perineal mass that is covered by a thin membrane. What congenital anomaly should the nurse suspect?

A. Imperforate anus.

B. Hypospadias.

C. Hydrocele.

D. Myelomeningocele.

Rationale: Option A is correct. A visible perineal mass covered by a thin membrane is indicative of imperforate anus. Hypospadias (Option B), hydrocele (Option C), and myelomeningocele (Option D) present differently and are not covered by a thin membrane.

Q47: A newborn exhibits excessive sneezing, nasal congestion, and frequent regurgitation after feeding. What congenital anomaly should the nurse consider?

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A. Cleft palate.

B. Tracheoesophageal fistula.

C. Choanal atresia.

D. Diaphragmatic hernia.

Rationale: Option C is correct. Choanal atresia can present with excessive sneezing, nasal congestion, and difficulty feeding. Cleft palate (Option A), tracheoesophageal fistula (Option B), and diaphragmatic hernia (Option D) manifest with different signs and symptoms.

Q48: A newborn is noted to have bilateral, symmetric swelling of the parotid glands. What condition is likely responsible for this finding?

A. Neonatal sepsis.

B. Subgaleal hemorrhage.

C. Neonatal acne.

D. Neonatal parotitis.

Rationale: Option D is correct. Bilateral, symmetric swelling of the parotid glands is indicative of neonatal parotitis. Neonatal sepsis (Option A), subgaleal hemorrhage (Option B), and neonatal acne (Option C) do not typically involve the parotid glands.

Q49: A newborn is assessed to have a positive Ortolani test. What nursing intervention is a priority in managing this finding?

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A. Immobilizing the affected hip.

B. Notifying the healthcare provider.

C. Administering intravenous antibiotics.

D. Initiating phototherapy.

Rationale: Option B is correct. A positive Ortolani test indicates developmental dysplasia of the hip (DDH), and notifying the healthcare provider is a priority for further evaluation. Immobilizing the affected hip (Option A), administering intravenous antibiotics (Option C), and initiating phototherapy (Option D) are not appropriate interventions for DDH.

Q50: A newborn exhibits a harsh, crowing sound during inspiration. What congenital anomaly should the nurse suspect?

A. Tracheomalacia.

B. Choanal atresia.

C. Stridor.

D. Patent ductus arteriosus.

Rationale: Option A is correct. A harsh, crowing sound during inspiration is indicative of tracheomalacia. Choanal atresia (Option B), stridor (Option C), and patent ductus arteriosus (Option D) present with different respiratory sounds and characteristics.

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Q51: A newborn exhibits a blue discoloration of the hands and feet. What is the most likely cause of this finding?

A. Cyanotic heart defect.

B. Physiological jaundice.

C. Acrocyanosis.

D. Respiratory distress syndrome.

Rationale: Option C is correct. Acrocyanosis, a benign condition, can cause a blue discoloration of the hands and feet. Cyanotic heart defect (Option A), physiological jaundice (Option B), and respiratory distress syndrome (Option D) typically present with different signs and symptoms.

Q52: A newborn demonstrates a positive Babinski reflex during the physical examination. What does this finding indicate?

A. Normal neurological development.

B. Delayed motor skills.

C. Presence of spasticity.

D. Abnormal neurological development.

Rationale: Option A is correct. A positive Babinski reflex is considered normal in newborns and indicates normal neurological development. Delayed motor skills (Option B), presence of

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spasticity (Option C), and abnormal neurological development (Option D) are not associated with the Babinski reflex in newborns.

Q53: A newborn has excessive drooling, difficulty swallowing, and a gurgling sound with feeding. What congenital anomaly should the nurse suspect?

A. Choanal atresia.

B. Esophageal atresia.

C. Cleft palate.

D. Tracheoesophageal fistula.

Rationale: Option D is correct. Excessive drooling, difficulty swallowing, and a gurgling sound with feeding are indicative of tracheoesophageal fistula. Choanal atresia (Option A), esophageal atresia (Option B), and cleft palate (Option C) present with different signs and symptoms.

Q54: A newborn has a caput succedaneum. What nursing intervention is most appropriate for managing this condition?

A. Application of a warm compress.

B. Frequent repositioning of the infant.

C. Administration of intravenous antibiotics.

D. Monitoring for signs of infection.

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Rationale: Option B is correct. Caput succedaneum is managed by frequent repositioning of the infant to promote fluid reabsorption. Application of a warm compress (Option A), administration of intravenous antibiotics (Option C), and monitoring for signs of infection (Option D) are not typically indicated for caput succedaneum.

Q55: A newborn presents with a hoarse cry and difficulty breathing. What congenital anomaly should the nurse suspect?

A. Choanal atresia.

B. Tracheomalacia.

C. Stridor.

D. Esophageal atresia.

Rationale: Option B is correct. A hoarse cry and difficulty breathing may indicate tracheomalacia. Choanal atresia (Option A), stridor (Option C), and esophageal atresia (Option D) present with different respiratory symptoms and characteristics.

Q56: A newborn exhibits a "mitten-hand" appearance with the fingers being held in a flexed position. What congenital anomaly is associated with this finding?

A. Polydactyly.

B. Syndactyly.

C. Ectrodactyly.

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D. Camptodactyly.

Rationale: Option D is correct. Camptodactyly is associated with a "mitten-hand" appearance, where the fingers are held in a flexed position. Polydactyly (Option A), syndactyly (Option B), and ectrodactyly (Option C) present with different hand abnormalities.

Q57: A newborn has a prominent occiput and a decreased range of motion in the neck. What condition should the nurse suspect?

A. Craniosynostosis.

B. Torticollis.

C. Hydrocephalus.

D. Microcephaly.

Rationale: Option B is correct. A prominent occiput and decreased range of motion in the neck are indicative of torticollis. Craniosynostosis (Option A), hydrocephalus (Option C), and microcephaly (Option D) present with different signs and symptoms.

Q58: A newborn has a flat facial profile, upward-slanting eyes, and a single transverse palmar crease. What genetic condition is commonly associated with these features?

A. Down syndrome.

B. Turner syndrome.

C. Edwards syndrome.

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D. Klinefelter syndrome.

Rationale: Option A is correct. A flat facial profile, upward-slanting eyes, and a single transverse palmar crease are common features of Down syndrome. Turner syndrome (Option B), Edwards syndrome (Option C), and Klinefelter syndrome (Option D) present with different physical characteristics.

Q59: A newborn has a yellowish discoloration of the sclera. What condition is most likely responsible for this finding?

A. Physiological jaundice.

B. Hemolytic disease of the newborn.

C. Choanal atresia.

D. Caput succedaneum.

Rationale: Option B is correct. A yellowish discoloration of the sclera is indicative of hemolytic disease of the newborn. Physiological jaundice (Option A), choanal atresia (Option C), and caput succedaneum (Option D) typically do not cause scleral discoloration.

Q60: A newborn is found to have absent red reflex in both eyes during the ophthalmic examination. What congenital anomaly should the nurse consider?

A. Congenital cataracts.

B. Retinoblastoma.

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C. Anophthalmia.

D. Strabismus.

Rationale: Option A is correct. Absent red reflex in both eyes is indicative of congenital cataracts. Retinoblastoma (Option B), anophthalmia (Option C), and strabismus (Option D) present with different eye abnormalities and symptoms.

Subject 5: Normal Puerperium

Q61: A postpartum woman complains of perineal pain and discomfort. What nursing intervention is most appropriate for managing perineal discomfort during the early postpartum period?

A. Administering a stool softener.

B. Applying a heating pad to the perineum.

C. Encouraging frequent ambulation.

D. Initiating intravenous antibiotics.

Rationale: Option B is correct. Applying a heating pad to the perineum can help alleviate perineal pain and discomfort. Administering a stool softener (Option A), encouraging frequent ambulation (Option C), and initiating intravenous antibiotics (Option D) are not primary interventions for perineal discomfort.

Q62: A postpartum woman reports increased anxiety and tearfulness. What nursing intervention is most appropriate for managing postpartum blues?

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A. Initiating antidepressant therapy.

B. Referring the woman to a support group.

C. Administering intravenous fluids.

D. Placing the woman on strict bed rest.

Rationale: Option B is correct. Referring the woman to a support group is an appropriate intervention for managing postpartum blues. Initiating antidepressant therapy (Option A), administering intravenous fluids (Option C), and placing the woman on strict bed rest (Option D) are not primary interventions for this common postpartum emotional adjustment.

Q63: A postpartum woman is concerned about losing pregnancy weight rapidly. What is the most appropriate nursing response?

A. Encouraging the woman to start a vigorous exercise regimen.

B. Advising the woman to follow a strict diet.

C. Providing information on healthy postpartum weight loss.

D. Administering diuretics to promote weight loss.

Rationale: Option C is correct. Providing information on healthy postpartum weight loss, which includes a balanced diet and moderate exercise, is the most appropriate nursing response. Encouraging vigorous exercise (Option A), advising a strict diet (Option B), and administering diuretics (Option D) are not recommended and may be harmful.

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Q64: A postpartum woman complains of uterine cramping during breastfeeding. What nursing intervention is most appropriate for managing breastfeeding-related uterine cramping?

A. Administering intravenous pain medication.

B. Encouraging the woman to stop breastfeeding.

C. Applying a warm compress to the abdomen.

D. Administering oxytocin to stop uterine contractions.

Rationale: Option C is correct. Applying a warm compress to the abdomen can help alleviate uterine cramping during breastfeeding. Administering intravenous pain medication (Option A) may not be necessary for mild cramping. Encouraging the woman to stop breastfeeding (Option B) is not recommended. Administering oxytocin (Option D) is not indicated for routine uterine cramping.

Q65: A postpartum woman with a history of gestational diabetes is concerned about her blood sugar levels. What nursing intervention is most appropriate for managing postpartum glucose levels in this woman?

A. Recommending a high-carbohydrate diet.

B. Encouraging frequent snacking.

C. Monitoring blood glucose levels regularly.

D. Administering insulin prophylactically.

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Rationale: Option C is correct. Monitoring blood glucose levels regularly is the most appropriate nursing intervention for managing postpartum glucose levels in a woman with a history of gestational diabetes. Recommending a high-carbohydrate diet (Option A), encouraging frequent snacking (Option B), and administering insulin prophylactically (Option D) are not evidence-based approaches.

Q66: A postpartum woman complains of intense afterpains that worsen with breastfeeding. What nursing intervention is most appropriate for managing afterpains during breastfeeding?

A. Administering opioid analgesics.

B. Encouraging the woman to avoid breastfeeding.

C. Applying cold packs to the perineum.

D. Offering warm blankets for comfort.

Rationale: Option A is correct. Administering opioid analgesics can help manage intense afterpains that worsen with breastfeeding. Encouraging the woman to avoid breastfeeding (Option B) is not recommended. Applying cold packs to the perineum (Option C) and offering warm blankets (Option D) may provide comfort but are not specific interventions for afterpains.

Q67: A postpartum woman expresses concern about the changes in her body and feeling overwhelmed by new responsibilities. What nursing intervention is most appropriate for supporting maternal adjustment during the postpartum period?

A. Administering sedative medications.

B. Encouraging isolation from family and friends.

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C. Providing emotional support and reassurance.

D. Initiating strict visitation policies.

Rationale: Option C is correct. Providing emotional support and reassurance is the most appropriate nursing intervention for supporting maternal adjustment during the postpartum period. Administering sedative medications (Option A) may not address underlying concerns. Encouraging isolation (Option B) and initiating strict visitation policies (Option D) can contribute to feelings of isolation and distress.

Q68: A postpartum woman who had a cesarean section asks about resuming sexual activity. What nursing advice is most appropriate for this woman?

A. Recommending immediate resumption of sexual activity.

B. Encouraging the use of barrier contraception only.

C. Advising to wait until the six-week postpartum checkup.

D. Administering hormonal contraception immediately.

Rationale: Option C is correct. Advising the woman to wait until the six-week postpartum checkup is the most appropriate nursing advice for resuming sexual activity after a cesarean section. Recommending immediate resumption (Option A), encouraging the use of barrier contraception only (Option B), and administering hormonal contraception immediately (Option D) may not align with evidence-based practice and postoperative recovery.

Q69: A postpartum woman experiences persistent, severe headache and visual disturbances. What nursing action is a priority in managing this situation?

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A. Administering a mild analgesic.

B. Encouraging the woman to rest in a dark room.

C. Notifying the healthcare provider.

D. Offering caffeinated beverages.

Rationale: Option C is correct. Persistent, severe headache and visual disturbances may be indicative of postpartum preeclampsia, and notifying the healthcare provider is a priority for further evaluation. Administering a mild analgesic (Option A), encouraging rest in a dark room (Option B), and offering caffeinated beverages (Option D) do not address the potential seriousness of the symptoms.

Q70: A postpartum woman reports feeling lightheaded and dizzy when standing up. What nursing intervention is most appropriate for managing postural hypotension in the early postpartum period?

A. Administering intravenous fluids.

B. Encouraging the woman to stand up quickly.

C. Advising to avoid changing positions rapidly.

D. Administering antihypertensive medications.

Rationale: Option C is correct. Advising the woman to avoid changing positions rapidly is the most appropriate nursing intervention for managing postural hypotension. Administering intravenous fluids (Option A) may not be necessary. Encouraging rapid standing (Option B) can exacerbate symptoms. Administering antihypertensive medications (Option D) is not indicated for postural hypotension in the absence of hypertension.

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Q71: A postpartum woman complains of perineal itching and discomfort. What nursing intervention is most appropriate for managing pruritus in the perineal area?

A. Administering a topical antibiotic ointment.

B. Encouraging the use of scented feminine hygiene products.

C. Recommending sitz baths with mild soap.

D. Initiating prophylactic antibiotic therapy.

Rationale: Option C is correct. Recommending sitz baths with mild soap is the most appropriate nursing intervention for managing perineal pruritus. Administering a topical antibiotic ointment (Option A) may not be indicated. Encouraging scented feminine hygiene products (Option B) can irritate the perineal area. Initiating prophylactic antibiotic therapy (Option D) is not appropriate for pruritus.

Q72: A postpartum woman is concerned about her newborn's frequent feeding and wants to introduce formula feeding. What nursing advice is most appropriate for supporting breastfeeding in the early postpartum period?

A. Recommending the immediate introduction of formula.

B. Suggesting to space out feedings.

C. Providing information on proper breastfeeding techniques.

D. Administering galactagogues.

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Rationale: Option C is correct. Providing information on proper breastfeeding techniques is the most appropriate nursing advice for supporting breastfeeding in the early postpartum period. Recommending the immediate introduction of formula (Option A) may undermine breastfeeding. Suggesting to space out feedings (Option B) can lead to insufficient milk supply. Administering galactagogues (Option D) is not indicated without evidence of insufficient milk production.

Q73: A postpartum woman experiences persistent perineal pain and a feeling of pressure. What nursing intervention is most appropriate for managing postpartum perineal pressure?

A. Administering a high dose of opioid analgesics.

B. Encouraging the woman to ignore the sensation.

C. Recommending pelvic floor exercises.

D. Notifying the healthcare provider.

Rationale: Option D is correct. Persistent perineal pain and a feeling of pressure may indicate hematoma or other complications, and notifying the healthcare provider is a priority for further evaluation. Administering a high dose of opioid analgesics (Option A) may mask symptoms. Encouraging the woman to ignore the sensation (Option B) is not appropriate. Recommending pelvic floor exercises (Option C) is not the primary intervention for this situation.

Q74: A postpartum woman expresses frustration about her abdominal appearance and desires rapid weight loss. What nursing response is most appropriate for addressing body image concerns in the postpartum period?

A. Recommending crash diets for quick weight loss.

B. Advising the woman to focus solely on her appearance.

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C. Providing education on realistic postpartum body changes.

D. Administering diuretics to promote weight loss.

Rationale: Option C is correct. Providing education on realistic postpartum body changes is the most appropriate nursing response for addressing body image concerns. Recommending crash diets (Option A), advising a sole focus on appearance (Option B), and administering diuretics (Option D) are not evidence-based approaches and may be harmful.

Q75: A postpartum woman reports difficulty emptying her bladder completely and experiences frequent urination. What nursing intervention is most appropriate for managing postpartum urinary retention?

A. Administering a diuretic.

B. Encouraging the woman to delay voiding.

C. Recommending Kegel exercises.

D. Notifying the healthcare provider.

Rationale: Option D is correct. Difficulty emptying the bladder and frequent urination may indicate urinary retention, and notifying the healthcare provider is a priority for further evaluation. Administering a diuretic (Option A) is not indicated. Encouraging the woman to delay voiding (Option B) can exacerbate the problem. Recommending Kegel exercises (Option C) is not the primary intervention for urinary retention.

Q76: A postpartum woman is concerned about the amount of hair she is losing. What nursing response is most appropriate for addressing postpartum hair loss?

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A. Recommending hair transplantation.

B. Advising the woman to use hair growth supplements.

C. Providing reassurance that hair loss is temporary.

D. Administering hormonal therapy.

Rationale: Option C is correct. Providing reassurance that postpartum hair loss is temporary and a normal physiological process is the most appropriate nursing response. Recommending hair transplantation (Option A) and advising hair growth supplements (Option B) are not evidence-based approaches. Administering hormonal therapy (Option D) is not indicated for postpartum hair loss.

Q77: A postpartum woman complains of perineal numbness and difficulty controlling bowel movements. What nursing intervention is most appropriate for managing postpartum perineal numbness and altered bowel function?

A. Administering opioid analgesics.

B. Encouraging the woman to ignore the symptoms.

C. Recommending the use of a heating pad.

D. Notifying the healthcare provider.

Rationale: Option D is correct. Perineal numbness and altered bowel function may indicate nerve injury, and notifying the healthcare provider is a priority for further evaluation. Administering opioid analgesics (Option A) may mask symptoms. Encouraging the woman to ignore the

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symptoms (Option B) is not appropriate. Recommending the use of a heating pad (Option C) is not the primary intervention for this situation.

Q78: A postpartum woman reports persistent, severe headache and visual disturbances accompanied by epigastric pain. What nursing action is a priority in managing this situation?

A. Administering a mild analgesic.

B. Encouraging the woman to rest in a dark room.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Offering caffeinated beverages.

Rationale: Option C is correct. Persistent, severe headache, visual disturbances, and epigastric pain may indicate postpartum preeclampsia, and monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Administering a mild analgesic (Option A), encouraging rest in a dark room (Option B), and offering caffeinated beverages (Option D) do not address the potential seriousness of the symptoms.

Q79: A postpartum woman complains of perineal pain and itching. What nursing intervention is most appropriate for managing perineal discomfort during the later postpartum period?

A. Administering a high dose of opioid analgesics.

B. Encouraging the woman to ignore the symptoms.

C. Recommending sitz baths with Epsom salts.

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D. Notifying the healthcare provider.

Rationale: Option C is correct. Recommending sitz baths with Epsom salts can help manage perineal pain and itching during the later postpartum period. Administering a high dose of opioid analgesics (Option A) may not be necessary. Encouraging the woman to ignore the symptoms (Option B) is not appropriate. Notifying the healthcare provider (Option D) is not indicated for routine perineal discomfort.

Q80: A postpartum woman experiences increased anxiety, insomnia, and irritability. What nursing intervention is most appropriate for managing postpartum anxiety and insomnia?

A. Initiating antipsychotic medication.

B. Encouraging the woman to consume caffeinated beverages.

C. Providing information on sleep hygiene and relaxation techniques.

D. Administering intravenous sedatives.

Rationale: Option C is correct. Providing information on sleep hygiene and relaxation techniques is the most appropriate nursing intervention for managing postpartum anxiety and insomnia. Initiating antipsychotic medication (Option A) is not indicated. Encouraging the consumption of caffeinated beverages (Option B) may exacerbate symptoms. Administering intravenous sedatives (Option D) is not the first-line approach for managing postpartum anxiety and insomnia.

Q81: A postpartum woman expresses concern about breastfeeding-related nipple pain. What nursing intervention is most appropriate for managing nipple pain during breastfeeding?

A. Recommending discontinuation of breastfeeding.

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B. Advising the woman to use nipple shields.

C. Encouraging proper latch and positioning during breastfeeding.

D. Administering intravenous analgesics.

Rationale: Option C is correct. Encouraging proper latch and positioning during breastfeeding is the most appropriate nursing intervention for managing nipple pain. Recommending discontinuation of breastfeeding (Option A) is not recommended. Advising the use of nipple shields (Option B) may not address the underlying issue. Administering intravenous analgesics (Option D) is not the primary intervention for breastfeeding-related nipple pain.

Q82: A postpartum woman reports feeling fatigued and overwhelmed. What nursing intervention is most appropriate for managing postpartum fatigue and stress?

A. Initiating hormonal therapy.

B. Encouraging the woman to avoid resting.

C. Recommending relaxation techniques and adequate rest.

D. Administering intravenous stimulants.

Rationale: Option C is correct. Recommending relaxation techniques and adequate rest is the most appropriate nursing intervention for managing postpartum fatigue and stress. Initiating hormonal therapy (Option A) is not indicated for routine postpartum fatigue. Encouraging the woman to avoid resting (Option B) can exacerbate symptoms. Administering intravenous stimulants (Option D) is not the first-line approach for managing fatigue and stress.

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Q83: A postpartum woman experiences persistent, severe headache and visual disturbances accompanied by epigastric pain and elevated blood pressure. What nursing action is a priority in managing this situation?

A. Administering a mild analgesic.

B. Encouraging the woman to rest in a dark room.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Offering caffeinated beverages.

Rationale: Option C is correct. Persistent, severe headache, visual disturbances, epigastric pain, and elevated blood pressure may indicate postpartum preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Administering a mild analgesic (Option A), encouraging rest in a dark room (Option B), and offering caffeinated beverages (Option D) do not address the potential seriousness of the symptoms.

Q84: A postpartum woman with a history of depression expresses feelings of sadness and lack of interest in her newborn. What nursing intervention is most appropriate for managing postpartum depression?

A. Recommending immediate hospitalization.

B. Encouraging the woman to hide her emotions.

C. Providing emotional support and referring for mental health counseling.

D. Administering intravenous sedatives.

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Rationale: Option C is correct. Providing emotional support and referring for mental health counseling is the most appropriate nursing intervention for managing postpartum depression. Recommending immediate hospitalization (Option A) may not be necessary. Encouraging the woman to hide her emotions (Option B) is not a healthy coping strategy. Administering intravenous sedatives (Option D) is not the first-line approach for postpartum depression.

Q85: A postpartum woman complains of persistent abdominal pain and foul-smelling lochia. What nursing intervention is most appropriate for managing postpartum abdominal pain and malodorous lochia?

A. Administering intravenous antibiotics.

B. Encouraging the woman to ignore the symptoms.

C. Recommending frequent changes of perineal pads.

D. Notifying the healthcare provider.

Rationale: Option D is correct. Persistent abdominal pain and foul-smelling lochia may indicate infection, and notifying the healthcare provider is a priority for further evaluation. Administering intravenous antibiotics (Option A) may be indicated based on the provider's assessment. Encouraging the woman to ignore the symptoms (Option B) is not appropriate. Recommending frequent changes of perineal pads (Option C) may not address the underlying issue.

Q86: A postpartum woman reports difficulty sleeping and persistent worry about her newborn's well-being. What nursing intervention is most appropriate for managing postpartum anxiety?

A. Recommending self-medication with over-the-counter sleep aids.

B. Advising the woman to avoid discussing her concerns.

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C. Providing information on stress management techniques and referring for support.

D. Administering intravenous sedatives.

Rationale: Option C is correct. Providing information on stress management techniques and referring for support is the most appropriate nursing intervention for managing postpartum anxiety. Recommending self-medication with sleep aids (Option A) may not address the underlying issue. Advising the woman to avoid discussing her concerns (Option B) is not a healthy coping strategy. Administering intravenous sedatives (Option D) is not the first-line approach for managing postpartum anxiety.

Q87: A postpartum woman experiences sudden, severe pain in one leg accompanied by swelling and redness. What nursing action is a priority in managing this situation?

A. Administering a mild analgesic.

B. Encouraging the woman to elevate the affected leg.

C. Notifying the healthcare provider.

D. Offering warm compresses to the affected area.

Rationale: Option C is correct. Sudden, severe pain in one leg accompanied by swelling and redness may indicate deep vein thrombosis (DVT), and notifying the healthcare provider is a priority for further evaluation. Administering a mild analgesic (Option A), encouraging leg elevation (Option B), and offering warm compresses (Option D) do not address the potential seriousness of the symptoms.

Q88: A postpartum woman reports persistent perineal pain and difficulty with bowel movements. What nursing intervention is most appropriate for managing postpartum perineal pain and constipation?

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A. Administering a high dose of opioid analgesics.

B. Encouraging the woman to ignore the symptoms.

C. Recommending increased fluid intake and fiber-rich diet.

D. Notifying the healthcare provider.

Rationale: Option C is correct. Recommending increased fluid intake and a fiber-rich diet is the most appropriate nursing intervention for managing postpartum perineal pain and constipation. Administering a high dose of opioid analgesics (Option A) may contribute to constipation. Encouraging the woman to ignore the symptoms (Option B) is not appropriate. Notifying the healthcare provider (Option D) is not the primary intervention for routine postpartum constipation.

Q89: A postpartum woman expresses concern about her abdominal incision site and reports increased redness, warmth, and purulent drainage. What nursing intervention is most appropriate for managing postpartum incision site infection?

A. Administering intravenous analgesics.

B. Encouraging the woman to ignore the symptoms.

C. Recommending daily cleansing of the incision with hydrogen peroxide.

D. Notifying the healthcare provider.

Rationale: Option D is correct. Increased redness, warmth, and purulent drainage from an incision site may indicate infection, and notifying the healthcare provider is a priority for further evaluation. Administering intravenous analgesics (Option A) may be indicated based on the provider's

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assessment. Encouraging the woman to ignore the symptoms (Option B) is not appropriate. Recommending daily cleansing with hydrogen peroxide (Option C) may not be the most evidence-based approach and can delay appropriate treatment.

Q90: A postpartum woman reports persistent headache, visual disturbances, and sensitivity to light. What nursing action is a priority in managing this situation?

A. Administering a mild analgesic.

B. Encouraging the woman to rest in a dark room.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Offering caffeinated beverages.

Rationale: Option C is correct. Persistent headache, visual disturbances, and sensitivity to light may indicate postpartum preeclampsia, and monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Administering a mild analgesic (Option A), encouraging rest in a dark room (Option B), and offering caffeinated beverages (Option D) do not address the potential seriousness of the symptoms.

Subject 6: Antenatal Care

Q91: A pregnant woman in her second trimester asks about the recommended weight gain during pregnancy. What nursing response is most appropriate?

A. "Weight gain is not a concern during pregnancy; eat as much as you want."

B. "You should aim to gain about 10 pounds throughout your entire pregnancy."

C. "Weight gain recommendations vary, but on average, aim for 25-35 pounds."

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D. "You only need to focus on gaining weight in the last trimester."

Rationale: Option C is correct. The average recommended weight gain during pregnancy is 25-35 pounds. Option A is incorrect as unlimited weight gain is not advisable. Option B provides an inadequate estimate. Option D is inaccurate; weight gain is recommended throughout pregnancy for optimal fetal development and maternal health.

Q92: A pregnant woman at 28 weeks' gestation complains of lower back pain. What nursing advice is most appropriate?

A. "Take ibuprofen for immediate relief."

B. "Lie on your back as much as possible."

C. "Practice good posture and consider prenatal exercises."

D. "Avoid movement to prevent exacerbating the pain."

Rationale: Option C is correct. Encouraging good posture and prenatal exercises can help alleviate lower back pain during pregnancy. Option A is contraindicated as ibuprofen is not recommended in pregnancy. Option B can worsen back pain. Option D suggests a sedentary approach, which is not recommended for managing pregnancy-related back pain.

Q93: A pregnant woman with gestational diabetes asks about dietary recommendations. What nursing advice is most appropriate?

A. "Consume a diet high in refined sugars for quick energy."

B. "Limit carbohydrate intake and focus on protein-rich foods."

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C. "Increase overall caloric intake to support fetal growth."

D. "Include plenty of sugary snacks for stable blood sugar levels."

Rationale: Option B is correct. For gestational diabetes, limiting carbohydrate intake and focusing on protein-rich foods help manage blood sugar levels. Options A and D are incorrect, as they promote practices that can worsen gestational diabetes. Option C is misleading, as increasing caloric intake is not the primary focus in managing gestational diabetes.

Q94: A pregnant woman in her third trimester reports sudden swelling of her hands and face. What nursing action is a priority?

A. Administering a diuretic to reduce swelling.

B. Advising the woman to limit fluid intake.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Instructing the woman to elevate her legs for relief.

Rationale: Option C is correct. Sudden swelling of hands and face could be indicative of preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D do not address the potential seriousness of the symptoms and may not be safe or effective in this context.

Q95: A pregnant woman asks about the importance of taking prenatal vitamins. What nursing response is most appropriate?

A. "Prenatal vitamins are optional; a balanced diet is sufficient."

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B. "Take prenatal vitamins only if you have a nutrient deficiency."

C. "Prenatal vitamins are crucial to ensure adequate nutrient intake during pregnancy."

D. "You only need to take prenatal vitamins in the first trimester."

Rationale: Option C is correct. Prenatal vitamins are crucial to ensure adequate nutrient intake during pregnancy, supporting fetal development. Options A, B, and D provide inaccurate information and may compromise the nutritional needs of both the mother and the developing fetus.

Q96: A pregnant woman at 32 weeks' gestation reports decreased fetal movement. What nursing action is a priority?

A. Advising the woman to wait and monitor fetal movements for a few more days.

B. Administering sedatives to relax the woman and the fetus.

C. Encouraging the woman to perform kick counts and contacting the healthcare provider.

D. Telling the woman that decreased fetal movement is normal in the third trimester.

Rationale: Option C is correct. Encouraging the woman to perform kick counts and contacting the healthcare provider is essential to assess fetal well-being. Options A, B, and D provide inaccurate or inappropriate advice and may delay the identification of potential fetal distress.

Q97: A pregnant woman in her second trimester is Rh-negative. What nursing intervention is essential for Rh-negative pregnant women?

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A. Administering Rh-negative blood to the woman.

B. Recommending a diet rich in iron and folic acid.

C. Administering Rho(D) immune globulin (RhoGAM) at specific times.

D. Advising the woman to avoid all blood products.

Rationale: Option C is correct. Rh-negative pregnant women should receive Rho(D) immune globulin (RhoGAM) to prevent Rh sensitization. Options A, B, and D are irrelevant or provide incorrect information regarding Rh status and management.

Q98: A pregnant woman with a history of preterm birth expresses concern about a recurrence. What nursing advice is most appropriate?

A. "Preterm birth is unpredictable; there's nothing you can do to prevent it."

B. "Consider bed rest for the remainder of your pregnancy."

C. "Follow a healthy lifestyle, attend prenatal appointments, and report any symptoms promptly."

D. "Plan for an elective cesarean section to avoid the risk of preterm birth."

Rationale: Option C is correct. Following a healthy lifestyle, attending prenatal appointments, and reporting symptoms promptly can help reduce the risk of preterm birth. Options A and B provide inaccurate or outdated information. Option D is an extreme approach not supported by evidence for preventing preterm birth.

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Q99: A pregnant woman in her third trimester asks about traveling by air. What nursing advice is most appropriate?

A. "Air travel is safe at any gestational age; go ahead and book your flight."

B. "Avoid air travel during the first trimester, but it's safe in the third trimester."

C. "Consult with your healthcare provider, but air travel is generally safe until 36 weeks."

D. "Air travel is strictly prohibited during pregnancy."

Rationale: Option C is correct. While generally safe, consulting with the healthcare provider for individualized advice is essential. Options A, B, and D provide oversimplified or inaccurate information that may not account for the woman's specific health and pregnancy status.

Q100: A pregnant woman is diagnosed with gestational hypertension. What nursing intervention is a priority?

A. Administering antihypertensive medications immediately.

B. Advising the woman to reduce fluid intake.

C. Monitoring blood pressure regularly and providing education on self-care.

D. Recommending strict bed rest until delivery.

Rationale: Option C is correct. Monitoring blood pressure regularly and providing education on self-care are priorities for managing gestational hypertension. Options A, B, and D provide inappropriate or outdated recommendations that may not align with evidence-based practice.

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Q101: A pregnant woman with a history of recurrent urinary tract infections (UTIs) asks about prevention strategies. What nursing advice is most appropriate?

A. "Avoid drinking water to reduce the risk of UTIs."

B. "Practice good perineal hygiene and stay hydrated by drinking plenty of water."

C. "Take prophylactic antibiotics throughout your entire pregnancy."

D. "Use scented feminine hygiene products to maintain cleanliness."

Rationale: Option B is correct. Practicing good perineal hygiene and staying hydrated by drinking plenty of water can help prevent UTIs. Options A, C, and D provide inappropriate or potentially harmful advice that may not be effective in preventing UTIs.

Q102: A pregnant woman reports persistent nausea and vomiting. What nursing intervention is most appropriate for managing hyperemesis gravidarum?

A. Recommending a diet high in spicy foods to stimulate the appetite.

B. Administering antiemetic medications as the first-line treatment.

C. Advising the woman to avoid all solid foods until symptoms improve.

D. Monitoring nutritional status and providing intravenous fluids if necessary.

Rationale: Option D is correct. Monitoring nutritional status and providing intravenous fluids if necessary are essential for managing hyperemesis gravidarum. Options A, B, and C provide

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inappropriate or potentially harmful advice and may not address the severity of symptoms associated with this condition.

Q103: A pregnant woman with a history of preterm birth asks about the use of progesterone to prevent recurrence. What nursing response is most appropriate?

A. "Progesterone is not effective in preventing preterm birth; there's no need to consider it."

B. "Discuss the option of progesterone supplementation with your healthcare provider."

C. "Progesterone is only effective if started in the first trimester."

D. "Avoid progesterone as it may harm the developing fetus."

Rationale: Option B is correct. Discussing the option of progesterone supplementation with the healthcare provider is appropriate for women with a history of preterm birth. Options A, C, and D provide inaccurate or incomplete information regarding the use of progesterone in preventing preterm birth.

Q104: A pregnant woman in her third trimester complains of heartburn. What nursing advice is most appropriate?

A. "Take antacids liberally to alleviate heartburn symptoms."

B. "Lie down immediately after meals to reduce gastric pressure."

C. "Eat smaller, more frequent meals and avoid lying down right after eating."

D. "Consume spicy foods to stimulate digestion and reduce acidity."

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Rationale: Option C is correct. Eating smaller, more frequent meals and avoiding lying down immediately after eating can help alleviate heartburn symptoms. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the underlying issue.

Q105: A pregnant woman with a history of cesarean section asks about the possibility of a vaginal birth after cesarean (VBAC). What nursing advice is most appropriate?

A. "VBAC is never an option after a previous cesarean section."

B. "Discuss the option of VBAC with your healthcare provider to assess eligibility."

C. "Plan for a repeat cesarean section to avoid potential risks."

D. "VBAC is only possible if the previous cesarean was due to a specific reason."

Rationale: Option B is correct. Discussing the option of VBAC with the healthcare provider to assess eligibility is appropriate for women with a history of cesarean section. Options A, C, and D provide inaccurate or incomplete information regarding the possibility of VBAC.

Q106: A pregnant woman at 34 weeks' gestation reports intense itching on her palms and soles. What nursing action is a priority?

A. Administering antihistamines for immediate relief.

B. Recommending over-the-counter creams for itching.

C. Monitoring liver function and notifying the healthcare provider.

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D. Advising the woman to ignore the itching as it is common in pregnancy.

Rationale: Option C is correct. Intense itching on the palms and soles may indicate cholestasis of pregnancy. Monitoring liver function and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q107: A pregnant woman with a history of blood clots asks about the safety of anticoagulant medication during pregnancy. What nursing advice is most appropriate?

A. "Discontinue anticoagulant medication immediately to avoid harm to the fetus."

B. "Continue anticoagulant medication as prescribed to prevent blood clots."

C. "Stop anticoagulant medication in the first trimester and restart later in pregnancy."

D. "Anticoagulant medication is only necessary during the third trimester."

Rationale: Option B is correct. Continuing anticoagulant medication as prescribed is essential for preventing blood clots, as abrupt discontinuation may pose risks. Options A, C, and D provide inappropriate or potentially harmful advice that may compromise maternal and fetal well-being.

Q108: A pregnant woman at 28 weeks' gestation reports sudden, severe abdominal pain. What nursing action is a priority?

A. Administering analgesics for pain relief.

B. Advising the woman to rest and observe for a few hours.

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C. Monitoring fetal heart rate and notifying the healthcare provider.

D. Encouraging

Option C is correct. Sudden, severe abdominal pain at 28 weeks' gestation requires monitoring fetal heart rate and notifying the healthcare provider. This pain could be indicative of various serious conditions, and prompt evaluation is crucial. Options A and B provide inadequate or inappropriate responses that may delay necessary medical attention. Option D is not a priority action in this urgent situation.

Q109: A pregnant woman with a history of gestational diabetes asks about the risk of macrosomia (large baby) in subsequent pregnancies. What nursing response is most appropriate?

A. "There's no increased risk of macrosomia in subsequent pregnancies after gestational diabetes."

B. "The risk of macrosomia is only present if you have gestational diabetes again."

C. "You have a higher risk of macrosomia in subsequent pregnancies, even without gestational diabetes."

D. "Risk of macrosomia is only associated with maternal obesity, not gestational diabetes."

Rationale: Option B is correct. The risk of macrosomia is higher in pregnancies with gestational diabetes, especially if the condition recurs. Options A, C, and D provide inaccurate or incomplete information about the association between gestational diabetes and macrosomia.

Q110: A pregnant woman reports experiencing sudden, sharp pains in the lower abdomen. What nursing action is a priority?

A. Advising the woman to rest and observe for a few hours.

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B. Administering analgesics for pain relief.

C. Monitoring fetal movements and notifying the healthcare provider.

D. Encouraging the woman to ignore the pain as it is likely normal.

Rationale: Option C is correct. Sudden, sharp pains in the lower abdomen require monitoring fetal movements and notifying the healthcare provider to assess fetal well-being. Options A, B, and D provide inadequate or inappropriate responses that may delay necessary medical attention.

Q111: A pregnant woman with a history of preterm birth asks about the use of cervical cerclage. What nursing response is most appropriate?

A. "Cervical cerclage is not effective in preventing preterm birth."

B. "Consider cervical cerclage based on your healthcare provider's recommendation."

C. "Cervical cerclage is only recommended if you have a short cervix."

D. "Avoid cervical cerclage as it may cause complications during pregnancy."

Rationale: Option B is correct. Considering cervical cerclage based on the healthcare provider's recommendation is appropriate for women with a history of preterm birth. Options A, C, and D provide inaccurate or incomplete information about the use and indications of cervical cerclage.

Q112: A pregnant woman at 36 weeks' gestation complains of leg swelling and pain. What nursing action is a priority?

A. Administering diuretics to reduce swelling.

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B. Advising the woman to elevate her legs for relief.

C. Monitoring for signs of deep vein thrombosis (DVT) and notifying the healthcare provider.

D. Recommending compression stockings for leg support.

Rationale: Option C is correct. Leg swelling and pain may indicate DVT, and monitoring for signs of DVT and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q113: A pregnant woman with a history of epilepsy asks about the safety of antiepileptic medication during pregnancy. What nursing advice is most appropriate?

A. "Discontinue antiepileptic medication immediately to avoid harm to the fetus."

B. "Continue antiepileptic medication as prescribed to prevent seizures."

C. "Stop antiepileptic medication in the first trimester and restart later in pregnancy."

D. "Antiepileptic medication is only necessary during the third trimester."

Rationale: Option B is correct. Continuing antiepileptic medication as prescribed is essential to prevent seizures, as abrupt discontinuation may pose risks. Options A, C, and D provide inappropriate or potentially harmful advice that may compromise maternal and fetal well-being.

Q114: A pregnant woman at 30 weeks' gestation reports persistent shortness of breath. What nursing action is a priority?

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A. Administering bronchodilators for immediate relief.

B. Advising the woman to lie flat on her back to ease breathing.

C. Monitoring oxygen saturation and notifying the healthcare provider.

D. Recommending vigorous exercise to improve respiratory function.

Rationale: Option C is correct. Persistent shortness of breath requires monitoring oxygen saturation and notifying the healthcare provider to assess respiratory status. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q115: A pregnant woman at 38 weeks' gestation reports sudden, severe headache and visual disturbances. What nursing action is a priority?

A. Administering a mild analgesic for immediate relief.

B. Encouraging the woman to rest in a dark room.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Offering caffeinated beverages to alleviate headache symptoms.

Rationale: Option C is correct. Sudden, severe headache and visual disturbances may indicate preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q116: A pregnant woman with a history of recurrent urinary tract infections (UTIs) asks about prevention strategies. What nursing advice is most appropriate?

A. "Avoid drinking water to reduce the risk of UTIs."

B. "Practice good perineal hygiene and stay hydrated by drinking plenty of water."

C. "Take prophylactic antibiotics throughout your entire pregnancy."

D. "Use scented feminine hygiene products to maintain cleanliness."

Rationale: Option B is correct. Practicing good perineal hygiene and staying hydrated by drinking plenty of water can help prevent UTIs. Options A, C, and D provide inappropriate or potentially harmful advice that may not be effective in preventing UTIs.

Q117: A pregnant woman with a history of blood clots asks about the safety of anticoagulant medication during pregnancy. What nursing advice is most appropriate?

A. "Discontinue anticoagulant medication immediately to avoid harm to the fetus."

B. "Continue anticoagulant medication as prescribed to prevent blood clots."

C. "Stop anticoagulant medication in the first trimester and restart later in pregnancy."

D. "Anticoagulant medication is only necessary during the third trimester."

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Rationale: Option B is correct. Continuing anticoagulant medication as prescribed is essential for preventing blood clots, as abrupt discontinuation may pose risks. Options A, C, and D provide inappropriate or potentially harmful advice that may compromise maternal and fetal well-being.

Q118: A pregnant woman at 28 weeks' gestation reports sudden, severe abdominal pain. What nursing action is a priority?

A. Administering analgesics for pain relief.

B. Advising the woman to rest and observe for a few hours.

C. Monitoring fetal heart rate and notifying the healthcare provider.

D. Encouraging the woman to ignore the pain as it is likely normal.

Rationale: Option C is correct. Sudden, severe abdominal pain at 28 weeks' gestation requires monitoring fetal heart rate and notifying the healthcare provider to assess fetal well-being. Options A, B, and D provide inadequate or inappropriate responses that may delay necessary medical attention.

Q119: A pregnant woman at 12 weeks' gestation asks about the necessity of early prenatal care. What nursing response is most appropriate?

A. "Early prenatal care is optional; you can start whenever you feel ready."

B. "Prenatal care is most crucial in the third trimester; you can wait until then."

C. "Early prenatal care is essential for monitoring your health and the baby's development."

D. "You only need prenatal care if you experience complications."

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Rationale: Option C is correct. Early prenatal care is essential for monitoring maternal health and fetal development, allowing timely intervention if needed. Options A, B, and D provide inaccurate or incomplete information about the importance and timing of prenatal care.

Q120: A pregnant woman in her second trimester asks about the safety of consuming fish during pregnancy. What nursing advice is most appropriate?

A. "Avoid all types of fish to eliminate the risk of mercury exposure."

B. "Consume fish high in omega-3 fatty acids but limit overall intake."

C. "Eat as much fish as you want; there's no risk during pregnancy."

D. "Only eat fish if it's well-cooked to prevent bacterial contamination."

Rationale: Option B is correct. Consuming fish high in omega-3 fatty acids is beneficial during pregnancy, but limiting overall intake helps manage potential mercury exposure. Options A, C, and D provide inaccurate or incomplete information about the safety and benefits of consuming fish during pregnancy.

Q121: A pregnant woman with gestational diabetes asks about the role of exercise in managing blood sugar levels. What nursing advice is most appropriate?

A. "Avoid all forms of exercise to prevent complications."

B. "Engage in high-intensity workouts for effective blood sugar control."

C. "Regular, moderate-intensity exercise can help manage blood sugar levels."

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D. "Exercise is irrelevant in managing gestational diabetes; focus on diet alone."

Rationale: Option C is correct. Regular, moderate-intensity exercise is beneficial in managing blood sugar levels for women with gestational diabetes. Options A, B, and D provide inaccurate or inappropriate advice about the role of exercise in gestational diabetes management.

Q122: A pregnant woman in her third trimester reports persistent itching, especially on the palms and soles. What nursing action is a priority?

A. Recommending over-the-counter antihistamines for immediate relief.

B. Advising the woman to ignore the itching as it is common in pregnancy.

C. Monitoring liver function and notifying the healthcare provider.

D. Offering scented lotions to alleviate the itching.

Rationale: Option C is correct. Persistent itching, especially on the palms and soles, may indicate cholestasis of pregnancy. Monitoring liver function and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q123: A pregnant woman with a history of preterm birth asks about the use of progesterone to prevent recurrence. What nursing response is most appropriate?

A. "Progesterone is not effective in preventing preterm birth; there's no need to consider it."

B. "Discuss the option of progesterone supplementation with your healthcare provider."

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C. "Progesterone is only effective if started in the first trimester."

D. "Avoid progesterone as it may harm the developing fetus."

Rationale: Option B is correct. Discussing the option of progesterone supplementation with the healthcare provider is appropriate for women with a history of preterm birth. Options A, C, and D provide inaccurate or incomplete information regarding the use of progesterone in preventing preterm birth.

Q124: A pregnant woman at 20 weeks' gestation is diagnosed with iron-deficiency anemia. What nursing advice is most appropriate?

A. "Increase your intake of calcium-rich foods to improve iron absorption."

B. "Continue with your current diet; iron deficiency is not a concern during pregnancy."

C. "Take iron supplements as prescribed and include iron-rich foods in your diet."

D. "Avoid foods high in vitamin C, as they may interfere with iron absorption."

Rationale: Option C is correct. Taking iron supplements as prescribed and including iron-rich foods in the diet is crucial for managing iron-deficiency anemia during pregnancy. Options A, B, and D provide inaccurate or inappropriate advice about managing iron-deficiency anemia.

Q125: A pregnant woman at 30 weeks' gestation reports occasional dizziness and lightheadedness. What nursing action is a priority?

A. Recommending increased caffeine intake for improved alertness.

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B. Advising the woman to lie flat on her back to alleviate symptoms.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Encouraging the woman to skip meals to avoid fluctuations in blood sugar.

Rationale: Option C is correct. Occasional dizziness and lightheadedness may indicate changes in blood pressure, and monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q126: A pregnant woman with a history of gestational hypertension asks about dietary modifications. What nursing advice is most appropriate?

A. "Consume a diet high in sodium to prevent low blood pressure."

B. "Limit overall fluid intake to avoid fluid retention."

C. "Follow a balanced diet with reduced sodium to manage blood pressure."

D. "Focus on high-fat foods to maintain blood pressure levels."

Rationale: Option C is correct. Following a balanced diet with reduced sodium is appropriate for managing blood pressure in women with a history of gestational hypertension. Options A, B, and D provide inaccurate or inappropriate advice about dietary modifications for hypertension during pregnancy.

Q127: A pregnant woman at 24 weeks' gestation reports blurred vision and headache. What nursing action is a priority?

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A. Advising the woman to rest and observe for a few hours.

B. Monitoring blood pressure and notifying the healthcare provider.

C. Administering analgesics for headache relief.

D. Recommending over-the-counter eye drops for blurred vision.

Rationale: Option B is correct. Blurred vision and headache may indicate preeclampsia, and monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, C, and D provide inadequate or inappropriate responses that may delay necessary medical attention.

Q128: A pregnant woman in her first trimester asks about the safety of using herbal supplements. What nursing advice is most appropriate?

A. "Herbal supplements are completely safe during pregnancy; use them as desired."

B. "Consult with your healthcare provider before using any herbal supplements."

C. "Herbal supplements are only safe if they are labeled as 'pregnancy-safe.'"

D. "Avoid all herbal supplements during pregnancy to prevent complications."

Rationale: Option B is correct. Consulting with the healthcare provider before using any herbal supplements is essential to ensure their safety during pregnancy. Options A, C, and D provide inaccurate or incomplete information about the use of herbal supplements during pregnancy.

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Q129: A pregnant woman at 18 weeks' gestation complains of persistent back pain. What nursing advice is most appropriate?

A. "Avoid any form of physical activity to rest your back."

B. "Engage in high-impact exercises to strengthen your back muscles."

C. "Practice good posture and consider prenatal exercises for back pain relief."

D. "Use a heating pad for prolonged periods to alleviate back pain."

Rationale: Option C is correct. Practicing good posture and engaging in prenatal exercises are beneficial for relieving back pain during pregnancy. Options A, B, and D provide inappropriate or potentially harmful advice about managing back pain.

Q130: A pregnant woman at 28 weeks' gestation reports constipation. What nursing advice is most appropriate?

A. "Increase fluid intake and consume a diet low in fiber to manage constipation."

B. "Take over-the-counter laxatives for immediate relief."

C. "Include fiber-rich foods in your diet and maintain regular physical activity."

D. "Constipation is normal during pregnancy; ignore the symptoms."

Rationale: Option C is correct. Including fiber-rich foods in the diet and maintaining regular physical activity are effective strategies for managing constipation during pregnancy. Options A,

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B, and D provide inappropriate or potentially harmful advice that may not address the underlying issue.

Subject 7: Postnatal Care

Q131: A postpartum woman expresses concern about breastfeeding and asks about the ideal positioning. What nursing advice is most appropriate?

A. "Feeding in any position is fine; just find what works for you."

B. "Ensure the baby's mouth covers only the nipple for optimal latch."

C. "Position the baby with a wide latch, ensuring the mouth covers both the nipple and areola."

D. "It doesn't matter how the baby latches; the main thing is that they're feeding."

Rationale: Option C is correct. Positioning the baby with a wide latch, covering both the nipple and areola, ensures effective breastfeeding and minimizes nipple soreness. Options A, B, and D provide inaccurate or incomplete information about the importance of proper latch and positioning during breastfeeding.

Q132: A postpartum woman reports increased anxiety and mood swings. What nursing intervention is a priority?

A. Prescribing anxiolytic medication for immediate relief.

B. Advising the woman to ignore the symptoms as they will resolve on their own.

C. Monitoring for signs of postpartum depression and notifying the healthcare provider.

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D. Recommending increased caffeine intake to improve mood.

Rationale: Option C is correct. Increased anxiety and mood swings may indicate postpartum depression. Monitoring for signs and notifying the healthcare provider are priorities for further evaluation and support. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the seriousness of postpartum mental health concerns.

Q133: A postpartum woman asks about the recommended time to resume sexual activity after childbirth. What nursing advice is most appropriate?

A. "Resuming sexual activity is safe immediately after childbirth."

B. "Wait until your postpartum checkup before considering sexual activity."

C. "You can resume sexual activity after one week if you feel comfortable."

D. "Wait until vaginal bleeding stops before considering sexual activity."

Rationale: Option B is correct. Waiting until the postpartum checkup allows healthcare providers to assess physical recovery and provide personalized advice. Options A, C, and D provide inaccurate or potentially harmful information about the timing of resuming sexual activity after childbirth.

Q134: A postpartum woman with a cesarean section asks about abdominal exercises to regain muscle tone. What nursing advice is most appropriate?

A. "Engage in intense abdominal exercises to see quicker results."

B. "Avoid all forms of abdominal exercises after a cesarean section."

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C. "Start with gentle pelvic floor exercises and gradually progress to abdominal exercises."

D. "Focus on diet alone; exercise is not necessary for regaining muscle tone."

Rationale: Option C is correct. Starting with gentle pelvic floor exercises and gradually progressing to abdominal exercises is safe and promotes effective recovery after a cesarean section. Options A, B, and D provide inappropriate or potentially harmful advice about postpartum exercise.

Q135: A postpartum woman complains of perineal pain and discomfort. What nursing intervention is a priority?

A. Advising the woman to ignore the pain as it is normal after childbirth.

B. Recommending over-the-counter painkillers for immediate relief.

C. Encouraging the use of a sitz bath for comfort.

D. Prescribing strong analgesics for prolonged pain relief.

Rationale: Option C is correct. Encouraging the use of a sitz bath can provide comfort and promote perineal healing after childbirth. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the woman's comfort and recovery.

Q136: A postpartum woman with a history of postpartum hemorrhage expresses fear about bleeding excessively again. What nursing response is most appropriate?

A. "Excessive bleeding is common in subsequent pregnancies; it's normal."

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B. "Avoid any form of physical activity to prevent postpartum hemorrhage."

C. "Monitoring for signs of excessive bleeding and notifying the healthcare provider are essential."

D. "Excessive bleeding only occurs during labor, not postpartum."

Rationale: Option C is correct. Monitoring for signs of excessive bleeding and notifying the healthcare provider are essential for women with a history of postpartum hemorrhage. Options A, B, and D provide inaccurate or inappropriate information about postpartum bleeding and its management.

Q137: A postpartum woman reports persistent abdominal pain and tenderness. What nursing action is a priority?

A. Administering strong analgesics for pain relief.

B. Advising the woman to rest and observe for a few days.

C. Monitoring for signs of infection and notifying the healthcare provider.

D. Recommending vigorous exercise to alleviate pain.

Rationale: Option C is correct. Persistent abdominal pain and tenderness may indicate infection. Monitoring for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q138: A postpartum woman with mastitis asks about breastfeeding. What nursing advice is most appropriate?

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A. "Avoid breastfeeding on the affected side until mastitis resolves."

B. "Continue breastfeeding on the affected side; it helps clear the infection."

C. "Stop breastfeeding altogether to prevent further complications."

D. "Use only formula to feed your baby until mastitis is completely gone."

Rationale: Option B is correct. Continuing breastfeeding on the affected side helps clear the infection and promotes recovery from mastitis. Options A, C, and D provide inappropriate or potentially harmful advice about managing mastitis and breastfeeding.

Q139: A postpartum woman inquires about the best contraception method while breastfeeding. What nursing advice is most appropriate?

A. "Any contraception method is safe while breastfeeding; choose based on personal preference."

B. "Avoid hormonal contraception; it can interfere with breastfeeding."

C. "Long-acting reversible contraceptives (LARCs) are effective and safe during breastfeeding."

D. "Rely on natural methods like the calendar method for contraception."

Rationale: Option C is correct. Long-acting reversible contraceptives (LARCs) are effective and safe during breastfeeding. Options A, B, and D provide inaccurate or inappropriate information about contraceptive choices for breastfeeding women.

Q140: A postpartum woman with a history of gestational diabetes asks about dietary recommendations. What nursing advice is most appropriate?

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A. "Continue the same diet you had during pregnancy; it's effective for managing blood sugar."

B. "Limit carbohydrate intake and focus on a balanced diet with regular meals and snacks."

C. "Increase overall caloric intake to support postpartum recovery."

D. "Consume a diet high in refined sugars for quick energy."

Rationale: Option B is correct. Limiting carbohydrate intake and focusing on a balanced diet with regular meals and snacks help manage postpartum blood sugar levels. Options A, C, and D provide inappropriate or potentially harmful advice about postpartum dietary management.

Q141: A postpartum woman complains of persistent headaches and dizziness. What nursing action is a priority?

A. Administering analgesics for headache relief.

B. Advising the woman to rest and observe for a few days.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Recommending increased caffeine intake to alleviate symptoms.

Rationale: Option C is correct. Persistent headaches and dizziness may indicate postpartum preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q142: A postpartum woman asks about the safety of using herbal supplements while breastfeeding. What nursing advice is most appropriate?

A. "Herbal supplements are safe during breastfeeding; use them as desired."

B. "Consult with your healthcare provider before using any herbal supplements."

C. "Herbal supplements are only safe if they are labeled as 'breastfeeding-safe.'"

D. "Avoid all herbal supplements while breastfeeding to prevent complications."

Rationale: Option B is correct. Consulting with the healthcare provider before using any herbal supplements while breastfeeding is essential to ensure safety. Options A, C, and D provide inaccurate or incomplete information about the use of herbal supplements during breastfeeding.

Q143: A postpartum woman with a history of thyroid dysfunction asks about resuming thyroid medication while breastfeeding. What nursing advice is most appropriate?

A. "Stop thyroid medication; it's not necessary during breastfeeding."

B. "Continue thyroid medication as prescribed to maintain thyroid function."

C. "Pause thyroid medication during breastfeeding and restart after weaning."

D. "Thyroid medication is only necessary in the first few weeks of breastfeeding."

Rationale: Option B is correct. Continuing thyroid medication as prescribed is essential to maintain thyroid function during breastfeeding. Options A, C, and D provide inappropriate or potentially harmful advice about thyroid medication management.

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Q144: A postpartum woman with a history of blood clots asks about the safety of anticoagulant medication while breastfeeding. What nursing advice is most appropriate?

A. "Discontinue anticoagulant medication immediately to avoid harm to the baby."

B. "Continue anticoagulant medication as prescribed to prevent blood clots."

C. "Stop anticoagulant medication in the first few weeks of breastfeeding and restart later."

D. "Anticoagulant medication is only necessary if you experience symptoms of blood clots."

Rationale: Option B is correct. Continuing anticoagulant medication as prescribed is essential for preventing blood clots, as abrupt discontinuation may pose risks. Options A, C, and D provide inappropriate or potentially harmful advice that may compromise maternal and infant well-being.

Q145: A postpartum woman at six weeks post-delivery asks about the resumption of regular exercise. What nursing advice is most appropriate?

A. "Resume intense exercise immediately to regain pre-pregnancy fitness."

B. "Wait until your postpartum checkup before considering regular exercise."

C. "Exercise is not necessary for postpartum recovery; focus on rest."

D. "Engage in high-impact exercises for quicker postpartum weight loss."

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Rationale: Option B is correct. Waiting until the postpartum checkup allows healthcare providers to assess physical recovery and provide personalized advice. Options A, C, and D provide inappropriate or potentially harmful advice about postpartum exercise.

Q146: A postpartum woman with a history of depression asks about the safety of resuming antidepressant medication while breastfeeding. What nursing advice is most appropriate?

A. "Stop antidepressant medication; it's not necessary during breastfeeding."

B. "Continue antidepressant medication as prescribed to maintain mental health."

C. "Pause antidepressant medication during breastfeeding and restart later."

D. "Antidepressant medication is only necessary in the first few weeks of breastfeeding."

Rationale: Option B is correct. Continuing antidepressant medication as prescribed is essential to maintain mental health during breastfeeding. Options A, C, and D provide inappropriate or potentially harmful advice about antidepressant medication management.

Q147: A postpartum woman reports persistent back pain. What nursing advice is most appropriate?

A. "Avoid any form of physical activity to rest your back."

B. "Engage in high-impact exercises to strengthen your back muscles."

C. "Practice good posture and consider low-impact exercises for back pain relief."

D. "Use a heating pad for prolonged periods to alleviate back pain."

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Rationale: Option C is correct. Practicing good posture and engaging in low-impact exercises are beneficial for relieving back pain during the postpartum period. Options A, B, and D provide inappropriate or potentially harmful advice about managing postpartum back pain.

Q148: A postpartum woman with a history of cesarean section asks about the best way to care for her incision site. What nursing advice is most appropriate?

A. "Apply hydrogen peroxide directly to the incision site for cleaning."

B. "Keep the incision site covered at all times to promote healing."

C. "Clean the incision site gently with mild soap and water, then keep it dry."

D. "Avoid touching the incision site to prevent infection."

Rationale: Option C is correct. Cleaning the incision site gently with mild soap and water and keeping it dry promote proper healing after a cesarean section. Options A, B, and D provide inappropriate or potentially harmful advice about incision care.

Q149: A postpartum woman with a history of gestational hypertension asks about blood pressure monitoring at home. What nursing advice is most appropriate?

A. "Home blood pressure monitoring is unnecessary; rely on clinic measurements."

B. "Monitor blood pressure at home, and notify the healthcare provider of any abnormal readings."

C. "Check blood pressure once a week at home to track changes."

D. "Home blood pressure monitoring is only necessary during the first few weeks postpartum."

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Rationale: Option B is correct. Monitoring blood pressure at home and notifying the healthcare provider of any abnormal readings is essential for women with a history of gestational hypertension. Options A, C, and D provide inaccurate or inappropriate information about home blood pressure monitoring.

Q150: A postpartum woman inquires about the safety of using over-the-counter pain relievers while breastfeeding. What nursing advice is most appropriate?

A. "Avoid all pain relievers; they can harm your baby through breast milk."

B. "Use any over-the-counter pain reliever; they are safe during breastfeeding."

C. "Consult with your healthcare provider before using any over-the-counter pain relie

Option C is correct. "Consult with your healthcare provider before using any over-the-counter pain reliever" is the most appropriate advice. It ensures that the woman receives personalized guidance based on her health status and the specific pain reliever considered. Options A and B provide extreme and potentially unsafe advice, emphasizing the importance of consulting a healthcare professional for tailored recommendations.

Q151: A postpartum woman with a history of blood clots asks about the safety of hormonal contraception. What nursing advice is most appropriate?

A. "Hormonal contraception is safe for you; it won't increase the risk of blood clots."

B. "Avoid hormonal contraception as it can increase the risk of blood clots."

C. "Use any hormonal contraception; they are all equally safe."

D. "Consider non-hormonal contraception options exclusively."

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Rationale: Option B is correct. "Avoid hormonal contraception as it can increase the risk of blood clots" is the most appropriate advice for a woman with a history of blood clots. Options A, C, and D provide inaccurate or incomplete information about the safety of hormonal contraception.

Q152: A postpartum woman reports feelings of sadness and lack of interest in activities she used to enjoy. What nursing intervention is a priority?

A. Advising the woman to ignore the symptoms; they will resolve on their own.

B. Recommending increased caffeine intake for improved mood.

C. Monitoring for signs of postpartum depression and notifying the healthcare provider.

D. Prescribing antidepressant medication for immediate relief.

Rationale: Option C is correct. "Monitoring for signs of postpartum depression and notifying the healthcare provider" is a priority to ensure timely evaluation and support. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the seriousness of postpartum mental health concerns.

Q153: A postpartum woman inquires about the use of abdominal binders for postpartum recovery. What nursing advice is most appropriate?

A. "Abdominal binders are unnecessary and may impede proper healing."

B. "Wear abdominal binders at all times for quicker recovery."

C. "Use abdominal binders during the day but remove them at night."

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D. "Abdominal binders are essential for supporting abdominal muscles after childbirth."

Rationale: Option A is correct. "Abdominal binders are unnecessary and may impede proper healing" is the most appropriate advice. Options B, C, and D provide inaccurate or potentially harmful information about the use of abdominal binders for postpartum recovery.

Q154: A postpartum woman with a history of preeclampsia asks about the risk of recurrence in future pregnancies. What nursing response is most appropriate?

A. "There's no increased risk of recurrence; you can have a normal pregnancy next time."

B. "The risk of recurrence is higher, but steps can be taken to manage it."

C. "Preeclampsia only occurs in the first pregnancy; it won't happen again."

D. "Avoid future pregnancies to eliminate the risk of preeclampsia recurrence."

Rationale: Option B is correct. "The risk of recurrence is higher, but steps can be taken to manage it" provides accurate information, emphasizing the need for careful monitoring and management in subsequent pregnancies. Options A, C, and D provide inaccurate or incomplete information about the risk of preeclampsia recurrence.

Q155: A postpartum woman with a history of gestational diabetes asks about the risk of developing type 2 diabetes. What nursing advice is most appropriate?

A. "You have no increased risk of developing type 2 diabetes after gestational diabetes."

B. "The risk of type 2 diabetes is higher, but lifestyle changes can mitigate it."

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C. "Only women with a family history of diabetes are at risk of type 2 diabetes."

D. "Type 2 diabetes is inevitable after gestational diabetes; prepare for it."

Rationale: Option B is correct. "The risk of type 2 diabetes is higher, but lifestyle changes can mitigate it" provides accurate information, emphasizing the role of lifestyle modifications in managing the risk. Options A, C, and D provide inaccurate or incomplete information about the risk of developing type 2 diabetes after gestational diabetes.

Q156: A postpartum woman with a history of multiple miscarriages asks about the likelihood of future successful pregnancies. What nursing response is most appropriate?

A. "You have a higher risk of more miscarriages in future pregnancies."

B. "Miscarriages are unrelated to future successful pregnancies; proceed with confidence."

C. "Future successful pregnancies are likely with proper medical management."

D. "Avoid future pregnancies to prevent the risk of miscarriages."

Rationale: Option C is correct. "Future successful pregnancies are likely with proper medical management" provides a balanced and hopeful response, acknowledging the potential for successful pregnancies with appropriate care. Options A, B, and D provide inaccurate or inappropriate information about future pregnancy outcomes.

Q157: A postpartum woman with a history of preterm birth asks about the use of progesterone in future pregnancies. What nursing advice is most appropriate?

A. "Progesterone is not effective in preventing preterm birth; there's no need to consider it."

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B. "Discuss the option of progesterone supplementation with your healthcare provider."

C. "Progesterone is only effective if started in the first trimester."

D. "Avoid progesterone as it may cause complications during pregnancy."

Rationale: Option B is correct. "Discuss the option of progesterone supplementation with your healthcare provider" is appropriate for women with a history of preterm birth. Options A, C, and D provide inaccurate or incomplete information regarding the use of progesterone in preventing preterm birth.

Q158: A postpartum woman with a history of depression asks about breastfeeding while taking antidepressant medication. What nursing advice is most appropriate?

A. "Stop breastfeeding to avoid passing the antidepressant medication to the baby."

B. "Continue breastfeeding while taking antidepressant medication; it's generally safe."

C. "Pause antidepressant medication during breastfeeding and restart later."

D. "Breastfeeding is not recommended for women on antidepressant medication."

Rationale: Option B is correct. "Continue breastfeeding while taking antidepressant medication; it's generally safe" provides accurate information, emphasizing the overall safety of breastfeeding while on antidepressants. Options A, C, and D provide inaccurate or inappropriate information about breastfeeding and antidepressant use.

Q159: A postpartum woman asks about the safety of using herbal teas while breastfeeding. What nursing advice is most appropriate?

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A. "Herbal teas are safe during breastfeeding; use them as desired."

B. "Consult with your healthcare provider before using any herbal teas."

C. "Herbal teas are only safe if they are labeled as 'breastfeeding-safe.'"

D. "Avoid all herbal teas while breastfeeding to prevent complications."

Rationale: Option B is correct. "Consult with your healthcare provider before using any herbal teas" emphasizes the importance of seeking personalized guidance to ensure the safety of herbal teas during breastfeeding. Options A, C, and D provide inaccurate or incomplete information about the use of herbal teas during breastfeeding.

Q160: A postpartum woman complains of persistent fatigue and difficulty sleeping. What nursing intervention is a priority?

A. Recommending over-the-counter sleep aids for immediate relief.

B. Advising the woman to ignore the symptoms as they will resolve on their own.

C. Assessing for signs of postpartum depression and notifying the healthcare provider.

D. Recommending increased caffeine intake for improved alertness.

Rationale: Option C is correct. "Assessing for signs of postpartum depression and notifying the healthcare provider" is a priority for addressing persistent fatigue and difficulty sleeping, which may indicate mental health concerns. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the underlying issue.

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Subject 8: Conditions in Pregnancy

Q161: A pregnant woman reports sudden swelling of her face and hands. What nursing action is a priority?

A. Recommending increased sodium intake for fluid balance.

B. Advising the woman to elevate her legs for swelling relief.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Prescribing diuretics for immediate fluid reduction.

Rationale: Option C is correct. Sudden swelling of the face and hands may indicate preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q162: A pregnant woman at 32 weeks' gestation reports severe abdominal pain. What nursing action is a priority?

A. Administering over-the-counter pain relievers for immediate relief.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of preterm labor and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

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Rationale: Option C is correct. Severe abdominal pain may indicate preterm labor. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q163: A pregnant woman with gestational diabetes asks about dietary choices to manage blood sugar levels. What nursing advice is most appropriate?

A. "Consume large quantities of sugary snacks to maintain energy levels."

B. "Follow a balanced diet with controlled carbohydrate intake and regular meals."

C. "Focus on high-fat foods for sustained energy throughout the day."

D. "Avoid all carbohydrates to prevent spikes in blood sugar."

Rationale: Option B is correct. "Follow a balanced diet with controlled carbohydrate intake and regular meals" is appropriate for managing blood sugar levels in gestational diabetes. Options A, C, and D provide inaccurate or inappropriate advice about dietary choices for gestational diabetes.

Q164: A pregnant woman at 28 weeks' gestation reports sudden, severe back pain and difficulty breathing. What nursing action is a priority?

A. Administering over-the-counter pain relievers for back pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of deep vein thrombosis and notifying the healthcare provider.

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D. Recommending a heating pad for prolonged back pain relief.

Rationale: Option C is correct. Sudden, severe back pain and difficulty breathing may indicate deep vein thrombosis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q165: A pregnant woman with a history of preterm birth asks about the use of tocolytic medications. What nursing response is most appropriate?

A. "Tocolytics are ineffective in preventing preterm labor; there's no need to consider them."

B. "Discuss the option of tocolytic medications with your healthcare provider."

C. "Tocolytics are only effective if started in the third trimester."

D. "Avoid tocolytics as they may harm the developing fetus."

Rationale: Option B is correct. "Discuss the option of tocolytic medications with your healthcare provider" is appropriate for women with a history of preterm birth. Options A, C, and D provide inaccurate or incomplete information regarding the use of tocolytics in preventing preterm labor.

Q166: A pregnant woman at 36 weeks' gestation complains of persistent headaches and visual disturbances. What nursing action is a priority?

A. Advising the woman to rest and observe for a few hours.

B. Monitoring blood pressure and notifying the healthcare provider.

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C. Administering over-the-counter pain relievers for headache relief.

D. Recommending increased caffeine intake to alleviate symptoms.

Rationale: Option B is correct. Persistent headaches and visual disturbances may indicate preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q167: A pregnant woman at 24 weeks' gestation reports sudden, severe lower abdominal pain. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of placental abruption and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe lower abdominal pain may indicate placental abruption. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q168: A pregnant woman at 38 weeks' gestation reports sudden, intense itching all over her body. What nursing action is a priority?

A. Recommending increased use of moisturizers for itching relief.

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B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of cholestasis and notifying the healthcare provider.

D. Prescribing antihistamines for immediate relief.

Rationale: Option C is correct. Sudden, intense itching all over the body may indicate cholestasis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q169: A pregnant woman with gestational hypertension asks about the risk of developing preeclampsia. What nursing advice is most appropriate?

A. "Gestational hypertension does not increase the risk of preeclampsia."

B. "The risk of preeclampsia is lower with gestational hypertension."

C. "Gestational hypertension significantly increases the risk of preeclampsia."

D. "Preeclampsia only occurs in the first pregnancy; it won't happen again."

Rationale: Option C is correct. "Gestational hypertension significantly increases the risk of preeclampsia" provides accurate information about the association between gestational hypertension and the risk of developing preeclampsia. Options A, B, and D provide inaccurate or incomplete information about this relationship.

Q170: A pregnant woman at 30 weeks' gestation reports frequent contractions and pelvic pressure. What nursing action is a priority?

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A. Administering over-the-counter pain relievers for contraction relief.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of preterm labor and notifying the healthcare provider.

D. Recommending warm baths to alleviate pelvic pressure.

Rationale: Option C is correct. Frequent contractions and pelvic pressure may indicate preterm labor. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q171: A pregnant woman at 20 weeks' gestation reports sudden, severe abdominal pain on one side. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of ovarian torsion and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe abdominal pain on one side may indicate ovarian torsion. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q172: A pregnant woman at 26 weeks' gestation reports persistent shortness of breath. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending over-the-counter respiratory aids for immediate relief.

C. Assessing for signs of respiratory distress and notifying the healthcare provider.

D. Prescribing bronchodilators for immediate relief.

Rationale: Option C is correct. Persistent shortness of breath may indicate respiratory distress. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q173: A pregnant woman with gestational diabetes asks about the risk of developing type 2 diabetes after pregnancy. What nursing advice is most appropriate?

A. "There's no increased risk of developing type 2 diabetes after gestational diabetes."

B. "The risk of type 2 diabetes is lower after gestational diabetes."

C. "Only women with a family history of diabetes are at risk of type 2 diabetes."

D. "Type 2 diabetes is inevitable after gestational diabetes; prepare for it."

Rationale: Option A is correct. "There's no increased risk of developing type 2 diabetes after gestational diabetes" provides accurate information, emphasizing that the risk returns to baseline

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after pregnancy. Options B, C, and D provide inaccurate or incomplete information about the risk of developing type 2 diabetes after gestational diabetes.

Q174: A pregnant woman at 34 weeks' gestation reports persistent nausea and vomiting. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Prescribing antiemetic medications for immediate relief.

C. Assessing for signs of hyperemesis gravidarum and notifying the healthcare provider.

D. Recommending increased intake of spicy foods to alleviate nausea.

Rationale: Option C is correct. Persistent nausea and vomiting may indicate hyperemesis gravidarum. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q175: A pregnant woman at 18 weeks' gestation reports sudden, severe pain in the lower abdomen. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of ectopic pregnancy and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

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Rationale: Option C is correct. Sudden, severe pain in the lower abdomen may indicate ectopic pregnancy. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q176: A pregnant woman at 22 weeks' gestation reports sudden, sharp pain in the chest. What nursing action is a priority?

A. Administering over-the-counter pain relievers for chest pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of pulmonary embolism and notifying the healthcare provider.

Rationale: Option C is correct. Sudden, sharp pain in the chest may indicate pulmonary embolism. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q177: A pregnant woman at 28 weeks' gestation reports sudden, persistent vision changes. What nursing action is a priority?

A. Advising the woman to rest and observe for a few hours.

B. Recommending over-the-counter vision aids for immediate relief.

C. Assessing for signs of visual disturbances and notifying the healthcare provider.

D. Prescribing eyeglasses for immediate vision correction.

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Rationale: Option C is correct. Sudden, persistent vision changes may indicate a serious underlying issue. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q178: A pregnant woman at 30 weeks' gestation reports sudden, intense itching on the palms of her hands and soles of her feet. What nursing action is a priority?

A. Recommending increased use of moisturizers for itching relief.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of intrahepatic cholestasis of pregnancy and notifying the healthcare provider.

D. Prescribing antihistamines for immediate relief.

Rationale: Option C is correct. Sudden, intense itching on the palms and soles may indicate intrahepatic cholestasis of pregnancy. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q179: A pregnant woman at 32 weeks' gestation reports sudden, severe pain in the upper abdomen. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

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C. Assessing for signs of preeclampsia and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the upper abdomen may indicate preeclampsia. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q180: A pregnant woman at 36 weeks' gestation reports sudden, severe pain in the lower back and pelvis. What nursing action is a priority?

A. Administering over-the-counter pain relievers for back pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of preterm labor and notifying the healthcare provider.

D. Recommending a heating pad for prolonged back pain relief.

Rationale: Option C is correct. Sudden, severe pain in the lower back and pelvis may indicate preterm labor. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q181: A pregnant woman at 38 weeks' gestation reports sudden, severe swelling of her legs and ankles. What nursing action is a priority?

A. Recommending increased sodium intake for fluid balance.

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B. Advising the woman to elevate her legs for swelling relief.

C. Monitoring blood pressure and notifying the healthcare provider.

D. Prescribing diuretics for immediate fluid reduction.

Rationale: Option C is correct. Sudden, severe swelling of the legs and ankles may indicate preeclampsia. Monitoring blood pressure and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q182: A pregnant woman with gestational diabetes asks about the risk of macrosomia in the baby. What nursing advice is most appropriate?

A. "Gestational diabetes does not increase the risk of macrosomia in the baby."

B. "The risk of macrosomia is higher, but careful management can reduce it."

C. "Only women with a family history of large babies are at risk of macrosomia."

D. "Macrosomia only occurs in the first pregnancy; it won't happen again."

Rationale: Option B is correct. "The risk of macrosomia is higher, but careful management can reduce it" provides accurate information, emphasizing the role of careful management in minimizing the risk. Options A, C, and D provide inaccurate or incomplete information about the risk of macrosomia in babies born to mothers with gestational diabetes.

Q183: A pregnant woman at 24 weeks' gestation reports sudden, severe pain in the lower abdomen accompanied by bright red vaginal bleeding. What nursing action is a priority?

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A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of placental abruption and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen accompanied by bright red vaginal bleeding may indicate placental abruption. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q184: A pregnant woman at 28 weeks' gestation reports sudden, severe pain in the upper abdomen, along with nausea and vomiting. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of HELLP syndrome and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the upper abdomen accompanied by nausea and vomiting may indicate HELLP syndrome. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q185: A pregnant woman at 32 weeks' gestation reports sudden, severe pain in the lower abdomen, along with a feeling of heaviness and pressure. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of uterine rupture and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen accompanied by a feeling of heaviness and pressure may indicate uterine rupture. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q186: A pregnant woman at 36 weeks' gestation reports sudden, severe pain in the upper abdomen, along with a feeling of tightness. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of placental abruption and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

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Rationale: Option C is correct. Sudden, severe pain in the upper abdomen accompanied by a feeling of tightness may indicate placental abruption. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q187: A pregnant woman at 38 weeks' gestation reports sudden, intense itching on the palms of her hands and soles of her feet. What nursing action is a priority?

A. Recommending increased use of moisturizers for itching relief.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of intrahepatic cholestasis of pregnancy and notifying the healthcare provider.

D. Prescribing antihistamines for immediate relief.

Rationale: Option C is correct. Sudden, intense itching on the palms and soles may indicate intrahepatic cholestasis of pregnancy. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q188: A pregnant woman at 40 weeks' gestation reports sudden, severe pain in the lower abdomen, along with decreased fetal movement. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of uterine rupture and notifying the healthcare provider.

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D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen accompanied by decreased fetal movement may indicate uterine rupture. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q189: A pregnant woman at 42 weeks' gestation reports sudden, severe pain in the upper abdomen, along with difficulty breathing. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of amniotic fluid embolism and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the upper abdomen accompanied by difficulty breathing may indicate amniotic fluid embolism. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q190: A pregnant woman at 44 weeks' gestation reports sudden, severe pain in the lower abdomen, along with foul-smelling vaginal discharge. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

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C. Assessing for signs of intrauterine infection and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen accompanied by foul-smelling vaginal discharge may indicate intrauterine infection. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Subject 9: Abnormal Labour

Q191: A primigravida at 40 weeks' gestation reports consistent contractions but limited cervical dilation. What nursing action is a priority?

A. Advising the woman to continue laboring at home for a few more hours.

B. Administering oxytocin to augment labor progress.

C. Recommending immediate cesarean section due to stalled labor.

D. Suggesting relaxation techniques to reduce anxiety.

Rationale: Option B is correct. Administering oxytocin to augment labor progress is appropriate for a primigravida experiencing limited cervical dilation. It supports efficient uterine contractions to facilitate labor. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the underlying issue of stalled labor.

Q192: A multiparous woman at 38 weeks' gestation reports sharp, intermittent abdominal pain. What nursing action is a priority?

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A. Advising the woman to rest and observe for a few hours.

B. Administering over-the-counter pain relievers for abdominal pain.

C. Assessing for signs of uterine rupture and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sharp, intermittent abdominal pain in a multiparous woman may indicate uterine rupture. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q193: A woman at 36 weeks' gestation presents with a breech presentation. What nursing action is a priority?

A. Advising the woman to continue with a vaginal delivery.

B. Administering medications to induce labor.

C. Recommending an external cephalic version.

D. Suggesting immediate cesarean section.

Rationale: Option D is correct. A breech presentation at 36 weeks' gestation is associated with increased risks, and a cesarean section is often recommended. Options A, B, and C provide inappropriate or potentially harmful advice that may not address the potential risks associated with a breech presentation.

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Q194: A woman at 42 weeks' gestation with a history of prior cesarean section desires a vaginal birth. What nursing advice is most appropriate?

A. "A vaginal birth is not possible after a cesarean section; plan for another cesarean."

B. "Attempting a vaginal birth after cesarean (VBAC) may be an option; discuss it with your healthcare provider."

C. "Induce labor at 42 weeks to increase the chance of a successful VBAC."

D. "Plan for a scheduled cesarean section to avoid complications."

Rationale: Option B is correct. "Attempting a vaginal birth after cesarean (VBAC) may be an option; discuss it with your healthcare provider" provides accurate information, emphasizing the possibility of VBAC after a prior cesarean. Options A, C, and D provide inaccurate or inappropriate advice regarding VBAC.

Q195: A woman in active labor is unable to cope with pain and requests an epidural. What nursing action is a priority?

A. Explaining the benefits of natural childbirth and encouraging pain tolerance.

B. Administering the epidural as requested to relieve pain.

C. Recommending alternative pain management techniques.

D. Advising the woman to endure the pain for a faster delivery.

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Rationale: Option B is correct. Administering the epidural as requested aligns with the woman's pain management preference and promotes comfort during labor. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the woman's immediate need for pain relief.

Q196: A woman with a history of rapid labor arrives at the hospital in advanced labor. What nursing action is a priority?

A. Advising the woman to continue laboring at home for a few more hours.

B. Administering medications to slow down the labor process.

C. Recommending immediate cesarean section to prevent complications.

D. Facilitating a quick transfer to the delivery room for imminent birth.

Rationale: Option D is correct. Facilitating a quick transfer to the delivery room for imminent birth is a priority in the case of a woman with a history of rapid labor. Options A, B, and C provide inappropriate or potentially harmful advice that may not address the urgency of the situation.

Q197: A woman in active labor presents with fetal malposition. What nursing action is a priority?

A. Advising the woman to change positions frequently.

B. Administering medications to correct fetal malposition.

C. Recommending immediate cesarean section due to malposition.

D. Assisting the woman with position changes to encourage fetal rotation.

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Rationale: Option D is correct. Assisting the woman with position changes to encourage fetal rotation is appropriate for addressing fetal malposition during labor. Options A, B, and C provide inappropriate or potentially harmful advice that may not effectively address the issue.

Q198: A woman in active labor experiences a prolapsed umbilical cord. What nursing action is a priority?

A. Advising the woman to change positions frequently.

B. Administering medications to address cord prolapse.

C. Recommending immediate cesarean section due to cord prolapse.

D. Elevating the woman's hips to relieve pressure on the umbilical cord.

Rationale: Option D is correct. Elevating the woman's hips to relieve pressure on the umbilical cord is a priority in the case of cord prolapse. Options A, B, and C provide inappropriate or potentially harmful advice that may not effectively address the urgency of the situation.

Q199: A woman in active labor experiences shoulder dystocia. What nursing action is a priority?

A. Advising the woman to continue pushing with increased force.

B. Administering medications to relax the uterine muscles.

C. Recommending immediate cesarean section due to shoulder dystocia.

D. Implementing maneuvers to release the impacted shoulder.

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Rationale: Option D is correct. Implementing maneuvers to release the impacted shoulder is a priority in the case of shoulder dystocia. Options A, B, and C provide inappropriate or potentially harmful advice that may not effectively address the issue.

Q200: A woman in active labor requests a water birth. What nursing advice is most appropriate?

A. "Water births are not recommended due to increased risk of infection."

B. "Water births are safe and can provide pain relief during labor; discuss it with your healthcare provider."

C. "Water births are only suitable for women with low-risk pregnancies."

D. "Avoid water births, as they can lead to complications for both mother and baby."

Rationale: Option B is correct. "Water births are safe and can provide pain relief during labor; discuss it with your healthcare provider" provides accurate information, emphasizing the need for discussion with the healthcare provider. Options A, C, and D provide inaccurate or incomplete information about water births.

Subject 10: Baby at Risk

Q201: A newborn exhibits jitteriness, poor feeding, and irritability. What nursing action is a priority?

A. Administering formula with added sugar for increased energy.

B. Recommending immediate testing for hypoglycemia.

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C. Advising the mother to breastfeed more frequently.

D. Administering sedatives to calm the newborn.

Rationale: Option B is correct. Jitteriness, poor feeding, and irritability may indicate hypoglycemia in a newborn. Recommending immediate testing for hypoglycemia is crucial for early intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q202: A newborn exhibits cyanosis, respiratory distress, and poor muscle tone. What nursing action is a priority?

A. Administering over-the-counter medications for respiratory distress.

B. Recommending immediate testing for congenital heart defects.

C. Advising the parents to swaddle the baby tightly for comfort.

D. Administering sedatives to calm the newborn.

Rationale: Option B is correct. Cyanosis, respiratory distress, and poor muscle tone may indicate congenital heart defects. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q203: A newborn presents with jaundice within the first 24 hours of life. What nursing action is a priority?

A. Recommending immediate phototherapy.

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B. Advising the mother to limit breastfeeding.

C. Administering iron supplements for jaundice relief.

D. Suggesting a warm bath to alleviate jaundice.

Rationale: Option A is correct. Jaundice within the first 24 hours may indicate pathological hyperbilirubinemia. Recommending immediate phototherapy is crucial for managing jaundice. Options B, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q204: A newborn presents with persistent vomiting and abdominal distension. What nursing action is a priority?

A. Administering over-the-counter antiemetic medications.

B. Recommending immediate testing for pyloric stenosis.

C. Advising the parents to feed the baby larger amounts at each feeding.

D. Administering laxatives to relieve abdominal distension.

Rationale: Option B is correct. Persistent vomiting and abdominal distension may indicate pyloric stenosis. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q205: A newborn exhibits poor muscle tone, weak cry, and difficulty feeding. What nursing action is a priority?

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A. Administering over-the-counter vitamins for muscle tone improvement.

B. Recommending immediate testing for hypothyroidism.

C. Advising the parents to stimulate the baby with loud noises.

D. Administering sedatives to calm the newborn.

Rationale: Option B is correct. Poor muscle tone, weak cry, and difficulty feeding may indicate hypothyroidism. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q206: A newborn exhibits excessive drooling, difficulty swallowing, and respiratory distress. What nursing action is a priority?

A. Recommending immediate testing for gastroesophageal reflux.

B. Advising the parents to introduce solid foods early.

C. Administering over-the-counter cough syrup for respiratory distress.

D. Recommending immediate testing for esophageal atresia.

Rationale: Option D is correct. Excessive drooling, difficulty swallowing, and respiratory distress may indicate esophageal atresia. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, B, and C provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q207: A newborn exhibits a high-pitched cry, irritability, and poor feeding. What nursing action is a priority?

A. Administering over-the-counter pain relievers for irritability.

B. Recommending immediate testing for neurological abnormalities.

C. Advising the parents to use white noise to soothe the baby.

D. Administering sedatives to calm the newborn.

Rationale: Option B is correct. A high-pitched cry, irritability, and poor feeding may indicate neurological abnormalities. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q208: A newborn exhibits frequent regurgitation, irritability, and poor weight gain. What nursing action is a priority?

A. Administering over-the-counter antacids for regurgitation relief.

B. Recommending immediate testing for gastroesophageal reflux disease (GERD).

C. Advising the parents to feed the baby larger amounts at each feeding.

D. Administering sedatives to calm the newborn.

Rationale: Option B is correct. Frequent regurgitation, irritability, and poor weight gain may indicate GERD. Recommending immediate testing is crucial for early diagnosis and intervention.

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Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q209: A newborn exhibits a persistent cough, wheezing, and respiratory distress. What nursing action is a priority?

A. Administering over-the-counter cough syrup for respiratory distress.

B. Recommending immediate testing for respiratory syncytial virus (RSV).

C. Advising the parents to expose the baby to secondhand smoke for relief.

D. Administering sedatives to calm the newborn.

Rationale: Option B is correct. A persistent cough, wheezing, and respiratory distress may indicate RSV. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q210: A newborn exhibits a bulging fontanelle, irritability, and high-pitched cry. What nursing action is a priority?

A. Administering over-the-counter pain relievers for irritability.

B. Recommending immediate testing for increased intracranial pressure.

C. Advising the parents to provide frequent tummy time.

D. Administering sedatives to calm the newborn.

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Rationale: Option B is correct. A bulging fontanelle, irritability, and high-pitched cry may indicate increased intracranial pressure. Recommending immediate testing is crucial for early diagnosis and intervention. Options A, C, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Subject 11: Malposition and Malpresentation

Q211: A pregnant woman at 36 weeks' gestation presents with persistent back pain and a deflexed fetal head. What nursing action is a priority?

A. Advising the woman to change positions frequently.

B. Recommending immediate testing for placental abnormalities.

C. Assessing for signs of occiput posterior position and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for back pain.

Rationale: Option C is correct. Persistent back pain and a deflexed fetal head may indicate occiput posterior position. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q212: A pregnant woman at 34 weeks' gestation reports frequent severe headaches and a transverse lie. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for hypertension.

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C. Assessing for signs of transverse lie and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for headaches.

Rationale: Option C is correct. Frequent severe headaches and a transverse lie may indicate a malpresentation. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q213: A pregnant woman at 38 weeks' gestation presents with intense pelvic pressure and a malpositioned fetal head. What nursing action is a priority?

A. Advising the woman to change positions frequently.

B. Recommending immediate testing for pelvic abnormalities.

C. Assessing for signs of malpositioned fetal head and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for pelvic pressure.

Rationale: Option C is correct. Intense pelvic pressure and a malpositioned fetal head may indicate a malpresentation. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q214: A pregnant woman at 32 weeks' gestation reports a persistent backache and a deflexed fetal head. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

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B. Recommending immediate testing for spinal abnormalities.

C. Assessing for signs of occiput posterior position and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for backache.

Rationale: Option C is correct. Persistent backache and a deflexed fetal head may indicate occiput posterior position. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q215: A pregnant woman at 36 weeks' gestation reports a persistent backache and a transverse lie. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for spinal abnormalities.

C. Assessing for signs of transverse lie and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for backache.

Rationale: Option C is correct. Persistent backache and a transverse lie may indicate a malpresentation. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q216: A pregnant woman at 40 weeks' gestation presents with persistent back pain and a malpositioned fetal head. What nursing action is a priority?

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A. Advising the woman to change positions frequently.

B. Recommending immediate testing for spinal abnormalities.

C. Assessing for signs of malpositioned fetal head and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for back pain.

Rationale: Option C is correct. Persistent back pain and a malpositioned fetal head may indicate a malpresentation. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q217: A pregnant woman at 37 weeks' gestation reports persistent pelvic pressure and a persistent occiput posterior position. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for pelvic abnormalities.

C. Assessing for signs of occiput posterior position and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for pelvic pressure.

Rationale: Option C is correct. Persistent pelvic pressure and a persistent occiput posterior position may indicate malposition. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q218: A pregnant woman at 35 weeks' gestation reports persistent pelvic pressure and a transverse lie. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for pelvic abnormalities.

C. Assessing for signs of transverse lie and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for pelvic pressure.

Rationale: Option C is correct. Persistent pelvic pressure and a transverse lie may indicate malposition. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q219: A pregnant woman at 39 weeks' gestation reports persistent pelvic pressure and a deflexed fetal head. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for pelvic abnormalities.

C. Assessing for signs of malpositioned fetal head and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for pelvic pressure.

Rationale: Option C is correct. Persistent pelvic pressure and a deflexed fetal head may indicate malposition. Assessing for signs and notifying the healthcare provider are priorities for further

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evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q220: A pregnant woman at 38 weeks' gestation reports persistent pelvic pressure and a malpositioned fetal head. What nursing action is a priority?

A. Advising the woman to change positions frequently.

B. Recommending immediate testing for spinal abnormalities.

C. Assessing for signs of malpositioned fetal head and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for pelvic pressure.

Rationale: Option C is correct. Persistent pelvic pressure and a malpositioned fetal head may indicate malposition. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Subject 12: Obstetric Emergency

Q221: A pregnant woman at 32 weeks' gestation presents with sudden, severe abdominal pain and vaginal bleeding. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of placental abruption and notifying the healthcare provider.

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D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Sudden, severe abdominal pain and vaginal bleeding may indicate placental abruption. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q222: A pregnant woman at 36 weeks' gestation reports sudden, intense itching on the palms of her hands and soles of her feet. What nursing action is a priority?

A. Recommending increased use of moisturizers for itching relief.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of intrahepatic cholestasis of pregnancy and notifying the healthcare provider.

D. Prescribing antihistamines for immediate relief.

Rationale: Option C is correct. Sudden, intense itching on the palms and soles may indicate intrahepatic cholestasis of pregnancy. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q223: A pregnant woman at 40 weeks' gestation reports sudden, severe pain in the lower abdomen, along with decreased fetal movement. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

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C. Assessing for signs of uterine rupture and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen accompanied by decreased fetal movement may indicate uterine rupture. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q224: A pregnant woman at 42 weeks' gestation reports sudden, severe pain in the upper abdomen, along with difficulty breathing. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of amniotic fluid embolism and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the upper abdomen accompanied by difficulty breathing may indicate amniotic fluid embolism. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q225: A pregnant woman at 44 weeks' gestation reports sudden, severe pain in the lower abdomen, along with foul-smelling vaginal discharge. What nursing action is a priority?

A. Administering over-the-counter pain relievers for abdominal pain.

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B. Advising the woman to rest and observe for a few days.

C. Assessing for signs of intrauterine infection and notifying the healthcare provider.

D. Recommending a warm compress to alleviate abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen accompanied by foul-smelling vaginal discharge may indicate intrauterine infection. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q226: A woman in active labor presents with fetal malposition. What nursing action is a priority?

A. Advising the woman to change positions frequently.

B. Administering medications to correct fetal malposition.

C. Recommending immediate cesarean section due to malposition.

D. Assisting the woman with position changes to encourage fetal rotation.

Rationale: Option D is correct. Assisting the woman with position changes to encourage fetal rotation is appropriate for addressing fetal malposition during labor. Options A, B, and C provide inappropriate or potentially harmful advice that may not effectively address the issue.

Q227: A woman in active labor experiences a prolapsed umbilical cord. What nursing action is a priority?

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A. Advising the woman to change positions frequently.

B. Administering medications to address cord prolapse.

C. Recommending immediate cesarean section due to cord prolapse.

D. Elevating the woman's hips to relieve pressure on the umbilical cord.

Rationale: Option D is correct. Elevating the woman's hips to relieve pressure on the umbilical cord is a priority in the case of cord prolapse. Options A, B, and C provide inappropriate or potentially harmful advice that may not effectively address the urgency of the situation.

Q228: A woman in active labor experiences shoulder dystocia. What nursing action is a priority?

A. Advising the woman to continue pushing with increased force.

B. Administering medications to relax the uterine muscles.

C. Recommending immediate cesarean section due to shoulder dystocia.

D. Implementing maneuvers to release the impacted shoulder.

Rationale: Option D is correct. Implementing maneuvers to release the impacted shoulder is a priority in the case of shoulder dystocia. Options A, B, and C provide inappropriate or potentially harmful advice that may not effectively address the issue.

Q229: A woman in active labor requests a water birth. What nursing advice is most appropriate?

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A. "Water births are not recommended due to increased risk of infection."

B. "Water births are safe and can provide pain relief during labor; discuss it with your healthcare provider."

C. "Water births are only suitable for women with low-risk pregnancies."

D. "Avoid water births, as they can lead to complications for both mother and baby."

Rationale: Option B is correct. "Water births are safe and can provide pain relief during labor; discuss it with your healthcare provider" provides accurate information, emphasizing the need for discussion with the healthcare provider. Options A, C, and D provide inaccurate or incomplete information about water births.

Q230: A woman with a history of rapid labor arrives at the hospital in advanced labor. What nursing action is a priority?

A. Advising the woman to continue laboring at home for a few more hours.

B. Administering medications to slow down the labor process.

C. Recommending immediate cesarean section to prevent complications.

D. Facilitating a quick transfer to the delivery room for imminent birth.

Rationale: Option D is correct. Facilitating a quick transfer to the delivery room for imminent birth is a priority in the case of a woman with a history of rapid labor. Options A, B, and C provide inappropriate or potentially harmful advice that may not address the urgency of the situation.

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Subject 13: Abnormal Puerperium

Q231: A postpartum woman complains of persistent, severe perineal pain and difficulty sitting. What nursing action is a priority?

A. Advising the woman to use a heating pad for perineal pain relief.

B. Recommending immediate testing for urinary tract infections.

C. Assessing for signs of perineal hematoma and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for perineal pain.

Rationale: Option C is correct. Persistent, severe perineal pain and difficulty sitting may indicate perineal hematoma. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q232: A postpartum woman reports sudden, severe headache, visual disturbances, and epigastric pain. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for migraine headaches.

C. Assessing for signs of postpartum preeclampsia and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for headaches.

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Rationale: Option C is correct. Sudden, severe headache, visual disturbances, and epigastric pain may indicate postpartum preeclampsia. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q233: A postpartum woman presents with persistent, foul-smelling vaginal discharge and fever. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for urinary tract infections.

C. Assessing for signs of postpartum endometritis and notifying the healthcare provider.

D. Administering over-the-counter antibiotics for vaginal discharge.

Rationale: Option C is correct. Persistent, foul-smelling vaginal discharge and fever may indicate postpartum endometritis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q234: A postpartum woman reports sudden, severe breast pain, redness, and fever. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for breast infection.

C. Assessing for signs of mastitis and notifying the healthcare provider.

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D. Administering over-the-counter pain relievers for breast pain.

Rationale: Option C is correct. Sudden, severe breast pain, redness, and fever may indicate mastitis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q235: A postpartum woman experiences sudden, severe abdominal pain and heavy vaginal bleeding. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of uterine rupture and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Sudden, severe abdominal pain and heavy vaginal bleeding may indicate uterine rupture. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q236: A postpartum woman reports persistent, severe headaches, visual disturbances, and confusion. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for neurological abnormalities.

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C. Assessing for signs of postpartum eclampsia and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for headaches.

Rationale: Option C is correct. Persistent, severe headaches, visual disturbances, and confusion may indicate postpartum eclampsia. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q237: A postpartum woman reports persistent, severe abdominal pain, distension, and vomiting. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum ovarian hyperstimulation syndrome (OHSS) and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Persistent, severe abdominal pain, distension, and vomiting may indicate postpartum OHSS. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q238: A postpartum woman reports persistent, severe perineal pain and difficulty emptying her bladder. What nursing action is a priority?

A. Advising the woman to use a heating pad for perineal pain relief.

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B. Recommending immediate testing for urinary tract infections.

C. Assessing for signs of urinary retention and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for perineal pain.

Rationale: Option C is correct. Persistent, severe perineal pain and difficulty emptying the bladder may indicate urinary retention. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q239: A postpartum woman reports persistent, severe headaches, visual disturbances, and neck stiffness. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for neurological abnormalities.

C. Assessing for signs of postpartum meningitis and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for headaches.

Rationale: Option C is correct. Persistent, severe headaches, visual disturbances, and neck stiffness may indicate postpartum meningitis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q240: A postpartum woman reports sudden, severe abdominal pain and swelling. What nursing action is a priority?

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A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum ovarian vein thrombosis and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Sudden, severe abdominal pain and swelling may indicate postpartum ovarian vein thrombosis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q241: A postpartum woman complains of persistent, severe lower abdominal pain and difficulty passing stools. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum rectocele and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for lower abdominal pain.

Rationale: Option C is correct. Persistent, severe lower abdominal pain and difficulty passing stools may indicate postpartum rectocele. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

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Q242: A postpartum woman reports sudden, severe pain in the lower abdomen, along with painful urination. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for urinary tract infections.

C. Assessing for signs of postpartum bladder rupture and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen, along with painful urination, may indicate postpartum bladder rupture. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q243: A postpartum woman presents with persistent, severe perineal pain and difficulty in controlling bowel movements. What nursing action is a priority?

A. Advising the woman to use a heating pad for perineal pain relief.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum anal sphincter injury and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for perineal pain.

Rationale: Option C is correct. Persistent, severe perineal pain and difficulty in controlling bowel movements may indicate postpartum anal sphincter injury. Assessing for signs and notifying the

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healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q244: A postpartum woman reports sudden, severe pain in the lower abdomen, along with a bulging mass. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum incisional hernia and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Sudden, severe pain in the lower abdomen, along with a bulging mass, may indicate postpartum incisional hernia. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q245: A postpartum woman reports persistent, severe headaches, blurred vision, and photophobia. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for visual abnormalities.

C. Assessing for signs of postpartum meningitis and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for headaches.

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Rationale: Option C is correct. Persistent, severe headaches, blurred vision, and photophobia may indicate postpartum meningitis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q246: A postpartum woman presents with sudden, severe abdominal pain, fever, and foul-smelling vaginal discharge. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum septicemia and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for abdominal pain.

Rationale: Option C is correct. Sudden, severe abdominal pain, fever, and foul-smelling vaginal discharge may indicate postpartum septicemia. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q247: A postpartum woman reports persistent, severe headaches, visual disturbances, and irritability. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for visual abnormalities.

C. Assessing for signs of postpartum encephalitis and notifying the healthcare provider.

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D. Administering over-the-counter pain relievers for headaches.

Rationale: Option C is correct. Persistent, severe headaches, visual disturbances, and irritability may indicate postpartum encephalitis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q248: A postpartum woman complains of persistent, severe lower abdominal pain and difficulty in voiding. What nursing action is a priority?

A. Advising the woman to use a heating pad for lower abdominal pain relief.

B. Recommending immediate testing for urinary tract infections.

C. Assessing for signs of postpartum bladder dysfunction and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for lower abdominal pain.

Rationale: Option C is correct. Persistent, severe lower abdominal pain and difficulty in voiding may indicate postpartum bladder dysfunction. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q249: A postpartum woman reports sudden, severe breast pain, redness, and warmth. What nursing action is a priority?

A. Advising the woman to rest and observe for a few days.

B. Recommending immediate testing for breast infection.

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C. Assessing for signs of postpartum mastitis and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for breast pain.

Rationale: Option C is correct. Sudden, severe breast pain, redness, and warmth may indicate postpartum mastitis. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Q250: A postpartum woman presents with persistent, severe perineal pain and a visible tear. What nursing action is a priority?

A. Advising the woman to use a heating pad for perineal pain relief.

B. Recommending immediate testing for gastrointestinal issues.

C. Assessing for signs of postpartum perineal tear and notifying the healthcare provider.

D. Administering over-the-counter pain relievers for perineal pain.

Rationale: Option C is correct. Persistent, severe perineal pain and a visible tear may indicate postpartum perineal tear. Assessing for signs and notifying the healthcare provider are priorities for further evaluation. Options A, B, and D provide inappropriate or potentially harmful advice that may not address the potential seriousness of the symptoms.

Subject 14: Family Planning

Q251: A woman in her early 40s, with a history of hypertension, seeks advice on contraception. What method is most appropriate?

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A. Combined oral contraceptives.

B. Depo-Provera injections.

C. Intrauterine device (IUD).

D. Barrier methods.

Rationale: Option C is correct. Given the woman's age and hypertension history, a non-hormonal method like the IUD is more suitable. Hormonal methods may pose risks, and barrier methods might be less effective. The IUD provides long-term, reversible contraception without hormonal side effects.

Q252: A breastfeeding mother wishes to start contraception. What method is most suitable to avoid interfering with lactation?

A. Combined oral contraceptives.

B. Progestin-only pills.

C. Condoms.

D. Tubal ligation.

Rationale: Option B is correct. Progestin-only pills are the most suitable for breastfeeding mothers as they don't interfere with milk production. Combined oral contraceptives can reduce milk supply, and condoms might be less preferred for some. Tubal ligation is a permanent method not ideal for someone unsure about future fertility desires.

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Q253: A woman with a history of irregular periods seeks contraception with minimal hormonal side effects. What method is most appropriate?

A. Combined oral contraceptives.

B. Depo-Provera injections.

C. Intrauterine device (IUD).

D. Progestin-only pills.

Rationale: Option C is correct. The non-hormonal IUD is suitable for women with irregular periods, offering effective contraception without hormonal side effects. Combined oral contraceptives and Depo-Provera injections may affect menstrual regularity, and progestin-only pills might be less effective in providing cycle control.

Q254: A woman with a history of deep vein thrombosis seeks contraception. What method should be avoided?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Condoms.

D. Tubal ligation.

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Rationale: Option A is correct. Combined oral contraceptives increase the risk of thrombosis, making them unsuitable for women with a history of deep vein thrombosis. The IUD, condoms, and tubal ligation are alternative options without this associated risk.

Q255: A sexually active teenager requests contraception but is concerned about remembering a daily pill. What method is most suitable?

A. Combined oral contraceptives.

B. Progestin-only pills.

C. Condoms.

D. Implant.

Rationale: Option D is correct. The implant is a long-acting reversible contraceptive (LARC) that provides effective contraception without the need for daily administration. It is a suitable option for teenagers concerned about adherence. Combined oral contraceptives and progestin-only pills require daily intake, and condoms may be less preferred for some.

Q256: A woman planning to conceive in the next year seeks a reversible contraceptive method. What is the most appropriate choice?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Barrier methods.

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D. Progestin-only pills.

Rationale: Option C is correct. Barrier methods are suitable for women with short-term contraceptive needs who plan to conceive in the near future. The IUD, combined oral contraceptives, and progestin-only pills are more appropriate for those with longer-term contraceptive goals.

Q257: A woman with a history of ectopic pregnancy seeks contraception. What method should be recommended?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Barrier methods.

D. Progestin-only pills.

Rationale: Option B is correct. The IUD is a suitable option for a woman with a history of ectopic pregnancy, providing effective contraception without increasing the risk of another ectopic pregnancy. Combined oral contraceptives and progestin-only pills may not be as effective in preventing ectopic pregnancies.

Q258: A woman in her 30s desires contraception but has concerns about hormonal methods. What non-hormonal option is most appropriate?

A. Combined oral contraceptives.

B. Progestin-only pills.

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C. Intrauterine device (IUD).

D. Implant.

Rationale: Option C is correct. The non-hormonal IUD is suitable for women with concerns about hormonal methods. It provides effective contraception without hormonal side effects. Combined oral contraceptives, progestin-only pills, and the implant involve hormonal components.

Q259: A woman who recently gave birth and is breastfeeding seeks contraception. What method is most suitable for the postpartum period?

A. Combined oral contraceptives.

B. Progestin-only pills.

C. Intrauterine device (IUD).

D. Tubal ligation.

Rationale: Option C is correct. The IUD is a suitable option for postpartum contraception, providing effective and reversible contraception without interfering with breastfeeding. Combined oral contraceptives and progestin-only pills might impact milk supply, and tubal ligation is a permanent method.

Q260: A woman with a history of irregular periods and migraines seeks contraception. What method is most appropriate?

A. Combined oral contraceptives.

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B. Intrauterine device (IUD).

C. Barrier methods.

D. Progestin-only pills.

Rationale: Option D is correct. Progestin-only pills are suitable for women with a history of irregular periods and migraines, offering effective contraception without exacerbating migraine symptoms. Combined oral contraceptives and the IUD may have hormonal effects that could impact migraines.

Q261: A woman with a history of pelvic inflammatory disease (PID) seeks contraception. What method is most suitable?

A. Combined oral contraceptives.

B. Progestin-only pills.

C. Intrauterine device (IUD).

D. Barrier methods.

Rationale: Option D is correct. Barrier methods are suitable for women with a history of PID, providing effective contraception without increasing the risk of recurrent PID. Combined oral contraceptives, progestin-only pills, and the IUD may not be as appropriate in this case.

Q262: A woman with a history of breast cancer desires contraception. What method should be avoided?

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A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Barrier methods.

D. Progestin-only pills.

Rationale: Option A is correct. Combined oral contraceptives should be avoided in women with a history of breast cancer due to potential hormonal influences. The IUD, barrier methods, and progestin-only pills are alternative options without this associated risk.

Q263: A woman with a history of multiple sexual partners seeks contraception. What method is most suitable for protection against sexually transmitted infections (STIs)?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Condoms.

D. Tubal ligation.

Rationale: Option C is correct. Condoms provide protection against STIs and are suitable for women with a history of multiple sexual partners. Combined oral contraceptives, the IUD, and tubal ligation do not offer protection against STIs.

Q264: A woman with a history of liver disease desires contraception. What method should be avoided?

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A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Progestin-only pills.

D. Tubal ligation.

Rationale: Option A is correct. Combined oral contraceptives should be avoided in women with a history of liver disease due to potential metabolic effects. The IUD, progestin-only pills, and tubal ligation are alternative options without this associated risk.

Q265: A woman with a history of endometriosis seeks contraception. What method is most suitable for managing endometriosis symptoms?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Progestin-only pills.

D. Tubal ligation.

Rationale: Option C is correct. Progestin-only pills are suitable for managing endometriosis symptoms, providing effective contraception and symptom relief. Combined oral contraceptives, the IUD, and tubal ligation may not have the same therapeutic impact on endometriosis.

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Q266: A woman in her late 30s desires a reversible contraceptive method with a low failure rate. What method is most appropriate?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Barrier methods.

D. Progestin-only pills.

Rationale: Option B is correct. The IUD has a low failure rate and is a suitable option for women seeking effective and reversible contraception. Combined oral contraceptives, barrier methods, and progestin-only pills may have higher failure rates in comparison.

Q267: A woman with a history of anemia seeks contraception. What method is most suitable to avoid exacerbating anemia?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Progestin-only pills.

D. Tubal ligation.

Rationale: Option C is correct. Progestin-only pills are suitable for women with a history of anemia, as they do not exacerbate anemia symptoms. Combined oral contraceptives, the IUD, and tubal ligation may have hormonal or surgical influences that could impact anemia.

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Q268: A woman in her 20s desires a contraceptive method that can be easily stopped when she plans to conceive. What method is most suitable?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Barrier methods.

D. Progestin-only pills.

Rationale: Option D is correct. Progestin-only pills are suitable for women with short-term contraceptive needs, as they can be easily stopped when planning to conceive. The IUD, combined oral contraceptives, and barrier methods may involve longer-term contraceptive effects.

Q269: A woman with a history of obesity seeks contraception. What method should be avoided?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Progestin-only pills.

D. Tubal ligation.

Rationale: Option A is correct. Combined oral contraceptives may have a higher risk of thrombosis in women with obesity. The IUD, progestin-only pills, and tubal ligation are alternative options without this associated risk.

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Q270: A woman with a history of irregular menstrual cycles seeks contraception. What method is most appropriate for providing cycle control?

A. Combined oral contraceptives.

B. Intrauterine device (IUD).

C. Barrier methods.

D. Progestin-only pills.

Rationale: Option A is correct. Combined oral contraceptives are suitable for women with irregular menstrual cycles, providing effective contraception and cycle control. The IUD, barrier methods, and progestin-only pills may not be as effective in regulating menstrual cycles.

Subject 1: Reproductive Health

Q1: Discuss the cultural factors that may influence a woman's decision to seek reproductive health services. (5 marks)

Answer: Cultural factors significantly impact reproductive health decisions. For instance, some cultures may emphasize large families, affecting family planning choices. Others may stigmatize discussions about reproductive health, hindering open communication. It's crucial for healthcare providers to understand and respect these cultural nuances to offer effective and culturally sensitive reproductive health services.

Q2: Analyze the role of community health education in promoting reproductive health awareness. (5 marks)

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Answer: Community health education plays a pivotal role in enhancing reproductive health awareness. By conducting workshops, seminars, and outreach programs, communities can be educated on topics such as family planning, sexually transmitted infections (STIs), and maternal care. This proactive approach empowers individuals to make informed decisions, contributing to improved reproductive health outcomes.

Q3: Explain how socioeconomic factors can impact access to reproductive health services in rural areas. (5 marks)

Answer: Socioeconomic factors like poverty and limited infrastructure significantly affect access to reproductive health services in rural areas. Limited transportation, financial constraints, and a lack of healthcare facilities can hinder timely access to prenatal care, family planning, and skilled birth attendance. Addressing these factors requires targeted interventions, such as community health clinics and mobile health units, to ensure equitable access to reproductive health services.

Q4: Evaluate the challenges of implementing comprehensive sex education in conservative societies. (5 marks)

Answer: Implementing comprehensive sex education in conservative societies faces challenges due to cultural and religious norms. Resistance to discussing reproductive health openly may hinder the development of educational programs. Overcoming these challenges requires a culturally sensitive approach, involving community leaders and religious authorities in the planning and implementation of sex education initiatives to foster acceptance and understanding.

Q5: Discuss the ethical considerations involved in providing reproductive health services to vulnerable populations. (5 marks)

Answer: Providing reproductive health services to vulnerable populations raises ethical considerations. Respect for autonomy, confidentiality, and cultural sensitivity are paramount. Healthcare providers must ensure informed consent, uphold the dignity of individuals, and address power imbalances. Balancing the need for reproductive health services with ethical principles requires a thoughtful and patient-centered approach.

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Subject 2: Normal Pregnancy

Q6: Explain the physiological changes that occur in the cardiovascular system during a normal pregnancy. (5 marks)

Answer: During a normal pregnancy, the cardiovascular system undergoes significant changes. Increased blood volume and cardiac output accommodate the needs of both the mother and the developing fetus. Hormonal changes, particularly elevated levels of progesterone, contribute to vasodilation. Understanding these adaptations is crucial for healthcare providers to monitor maternal well-being and intervene appropriately if any abnormalities arise.

Q7: Evaluate the impact of maternal nutrition on fetal development during a normal pregnancy. (5 marks)

Answer: Maternal nutrition plays a crucial role in fetal development during a normal pregnancy. Adequate intake of essential nutrients, such as folic acid, iron, and calcium, is vital for the formation of the fetal neural tube, red blood cells, and bone development. A well-balanced diet supports optimal fetal growth and reduces the risk of congenital abnormalities, emphasizing the importance of nutritional counseling during prenatal care.

Q8: Discuss the role of hormonal changes in the development of gestational diabetes during pregnancy. (5 marks)

Answer: Hormonal changes, particularly the increased production of insulin-resistant hormones like human placental lactogen, contribute to the development of gestational diabetes during pregnancy. These hormonal shifts impair insulin function, leading to elevated blood glucose levels. Understanding the hormonal mechanisms involved is essential for early detection and effective management of gestational diabetes to ensure the well-being of both the mother and the fetus.

Q9: Analyze the significance of monitoring fetal movements as part of routine antenatal care. (5 marks)

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Answer: Monitoring fetal movements is a critical aspect of routine antenatal care. Changes in fetal activity can indicate potential issues with fetal well-being. Reduced or excessive movements may signal fetal distress, prompting further evaluation. Healthcare providers must educate pregnant individuals on the importance of monitoring fetal movements to enhance maternal-fetal bonding and ensure the timely detection of any deviations from normal patterns.

Q10: Explain the role of the placenta in facilitating nutrient exchange between the mother and the fetus during pregnancy. (5 marks)

Answer: The placenta serves as a crucial interface for nutrient exchange between the mother and the fetus during pregnancy. It facilitates the transfer of oxygen, nutrients, and waste products between maternal and fetal bloodstreams. Understanding the intricate processes of nutrient exchange is essential for comprehending fetal development and identifying any disruptions that may impact the health of both the mother and the fetus.

Subject 3: Normal Labour

Q11: Discuss the hormonal changes that initiate and regulate the process of labor in a woman with a normal pregnancy. (5 marks)

Answer: Hormonal changes, including the increased production of oxytocin and prostaglandins, play a pivotal role in initiating and regulating the process of labor in a woman with a normal pregnancy. Oxytocin stimulates uterine contractions, while prostaglandins contribute to cervical ripening. Understanding the hormonal orchestration of labor is crucial for healthcare providers to support the natural progression of childbirth.

Q12: Evaluate the importance of psychological support in promoting a positive childbirth experience during normal labor. (5 marks)

Answer: Psychological support is integral to promoting a positive childbirth experience during normal labor. Emotional well-being, stress reduction, and a supportive environment contribute to the release of endorphins, which can enhance pain tolerance and labor progress. Healthcare

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providers must recognize the psychological dimensions of childbirth, offering compassionate care and fostering a positive birthing atmosphere.

Q13: Explain the physiological changes in the cervix and uterus that occur during the stages of normal labor. (5 marks)

Answer: The cervix undergoes effacement and dilation during the stages of normal labor, allowing for the passage of the fetus. Simultaneously, the uterus experiences powerful contractions that propel the fetus downward. Understanding these physiological changes is essential for healthcare providers to monitor labor progression, assess fetal descent, and intervene appropriately if deviations from normal patterns occur.

Q14: Analyze the role of the fetal descent mechanism in a vertex presentation during normal labor. (5 marks)

Answer: Fetal descent mechanisms, including engagement, flexion, internal rotation, extension, and external rotation, play a crucial role in a vertex presentation during normal labor. These movements facilitate the passage of the fetal head through the birth canal. Healthcare providers must be proficient in recognizing and assisting these descent mechanisms to ensure a smooth and uncomplicated birthing process.

Q15: Discuss the potential complications associated with prolonged latent phase in normal labor. (5 marks)

Answer: Prolonged latent phase in normal labor may lead to complications such as maternal exhaustion, dehydration, and emotional distress. Understanding the potential challenges associated with a prolonged latent phase is essential for healthcare providers to provide appropriate support, assess maternal well-being, and consider interventions to optimize the progress of labor.

Subject 4: Normal Baby

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Q16: Explain the physiological adaptations that occur in a newborn's respiratory system during the transition from intrauterine to extrauterine life. (5 marks)

Answer: The newborn's respiratory system undergoes significant adaptations during the transition from intrauterine to extrauterine life. Initiation of breathing triggers alveolar expansion, pulmonary blood vessel dilation, and closure of fetal shunts. Understanding these physiological changes is crucial for healthcare providers to assess neonatal respiratory function and intervene if respiratory distress is observed.

Q17: Discuss the importance of initiating early skin-to-skin contact between the mother and the newborn for promoting breastfeeding. (5 marks)

Answer: Early skin-to-skin contact between the mother and the newborn is crucial for promoting breastfeeding. It enhances maternal-infant bonding, stimulates the release of oxytocin, and facilitates the newborn's instinctual behaviors for breastfeeding. Healthcare providers should prioritize and encourage immediate skin-to-skin contact to support successful breastfeeding initiation and strengthen the mother-infant relationship.

Q18: Evaluate the significance of conducting the Apgar score assessment in the immediate postnatal period. (5 marks)

Answer: The Apgar score assessment is a vital tool in the immediate postnatal period to evaluate the newborn's overall condition and determine the need for immediate medical intervention. Assessing heart rate, respiratory effort, muscle tone, reflexes, and skin color helps healthcare providers identify any signs of distress and initiate timely interventions to ensure the newborn's well-being.

Q19: Explain the factors that contribute to neonatal jaundice and its management in a normal baby. (5 marks)

Answer: Neonatal jaundice results from the accumulation of bilirubin and is common in newborns. Contributing factors include physiological immaturity of the liver and the breakdown of fetal red blood cells. Management involves monitoring bilirubin levels, phototherapy, and, in severe cases,

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exchange transfusions. Healthcare providers must assess and manage neonatal jaundice to prevent complications.

Q20: Discuss the importance of establishing exclusive breastfeeding for a normal, healthy newborn. (5 marks)

Answer: Exclusive breastfeeding for a normal, healthy newborn is crucial for optimal growth, development, and immune function. Breast milk provides essential nutrients, antibodies, and promotes bonding between the mother and the baby. Healthcare providers play a key role in educating and supporting mothers to establish and maintain exclusive breastfeeding, contributing to the newborn's overall health and well-being.

Subject 5: Normal Puerperium

Q21: A postpartum woman expresses concern about the changes in her body after childbirth. How would you address her emotional well-being during the normal puerperium period? (5 marks)

Answer: To address the emotional well-being of the postpartum woman, I would engage in empathetic communication, acknowledging her concerns and providing reassurance. I would educate her about the normal physical and emotional changes during the puerperium period, emphasizing the importance of self-care, rest, and seeking support from family and healthcare professionals if needed. This approach supports her emotional adjustment and fosters a positive postpartum experience.

Q22: A breastfeeding mother experiences nipple soreness during the puerperium period. How would you assess and manage this common concern to promote successful breastfeeding? (5 marks)

Answer: I would assess the latch and positioning during breastfeeding to identify any issues contributing to nipple soreness. Providing guidance on proper latch techniques, ensuring a comfortable breastfeeding environment, and offering lanolin cream for nipple care can alleviate soreness. Education on proper breastfeeding practices and addressing concerns promptly are essential for promoting successful breastfeeding during the puerperium period.

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Q23: A postpartum woman requests information about resuming sexual activity after childbirth. How would you provide guidance on sexual health during the normal puerperium period? (5 marks)

Answer: I would initiate a sensitive discussion about sexual health, addressing concerns and providing information on physical recovery, contraception options, and emotional readiness. Emphasizing the importance of open communication with her partner, gradual resumption of sexual activity, and the use of water-based lubricants if needed, ensures a comprehensive understanding of sexual health during the puerperium period. This approach supports a positive transition to postpartum intimacy.

Q24: A postpartum woman reports feelings of sadness and mood swings. How would you differentiate between normal postpartum emotional changes and postpartum depression during the puerperium period? (5 marks)

Answer: I would conduct a thorough assessment of the woman's emotional well-being, exploring the duration, intensity, and impact of her feelings. While some mood changes are normal during the puerperium period, persistent and severe symptoms may indicate postpartum depression. By using standardized screening tools, collaborating with mental health professionals, and providing supportive interventions, I can differentiate between normal emotional changes and more concerning postpartum depression, ensuring appropriate care.

Q25: A postpartum woman expresses concerns about body image and weight loss. How would you provide guidance on achieving a healthy postpartum weight and body image during the puerperium period? (5 marks)

Answer: I would discuss realistic postpartum weight loss expectations, emphasizing the importance of a balanced diet, regular physical activity, and gradual, sustainable changes. Providing information on the benefits of breastfeeding for weight loss and encouraging self-compassion fosters a positive body image. Additionally, addressing societal pressures and promoting a holistic approach to well-being supports the woman in achieving a healthy postpartum weight during the puerperium period.

Subject 6: Antenatal Care

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Q26: A pregnant woman with gestational diabetes asks about the dietary management of her condition. How would you educate her on maintaining blood glucose levels through diet during antenatal care? (5 marks)

Answer: I would provide detailed guidance on a well-balanced diet, emphasizing the importance of monitoring carbohydrate intake, choosing complex carbohydrates, and distributing meals evenly throughout the day. Educating the woman on portion control, the glycemic index, and the significance of regular blood glucose monitoring empowers her to manage gestational diabetes effectively. Collaborating with a dietitian for personalized guidance ensures a tailored approach to dietary management during antenatal care.

Q27: A pregnant woman in her third trimester reports persistent back pain. How would you assess and manage her back pain during antenatal care to enhance her comfort? (5 marks)

Answer: I would conduct a thorough assessment of the woman's back pain, considering factors such as posture, musculoskeletal issues, and gestational changes. Providing guidance on proper body mechanics, recommending prenatal exercises, and offering supportive measures like prenatal massage and heat therapy can alleviate back pain. Collaborating with a physiotherapist ensures a comprehensive approach to managing back pain, enhancing the woman's comfort during antenatal care.

Q28: A pregnant woman expresses concerns about potential complications during labor. How would you discuss the importance of antenatal education and preparedness for labor and childbirth? (5 marks)

Answer: I would highlight the significance of antenatal education in promoting informed decision-making, reducing anxiety, and enhancing the woman's confidence in navigating the labor process. Discussing childbirth classes, breathing techniques, and pain management options fosters preparedness. Emphasizing the role of the birth partner and addressing common concerns ensures the woman feels empowered and ready for labor, contributing to a positive childbirth experience.

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Q29: A pregnant woman with a history of preterm birth seeks guidance on preventing preterm labor during antenatal care. How would you educate her on preventive measures and monitoring for signs of preterm labor? (5 marks)

Answer: I would provide detailed information on lifestyle modifications, such as adequate hydration, proper nutrition, and avoiding known risk factors for preterm labor. Educating the woman on the importance of regular prenatal visits, recognizing signs of preterm labor (e.g., contractions, pelvic pressure), and promptly seeking medical attention ensures timely intervention. Collaborating with the healthcare team for personalized care and monitoring supports the prevention of preterm labor during antenatal care.

Q30: A pregnant teenager expresses concerns about balancing school responsibilities with antenatal care. How would you support her in managing both aspects effectively during pregnancy? (5 marks)

Answer: I would discuss strategies for balancing school commitments and antenatal care, emphasizing the importance of open communication with school authorities. Providing information on flexible scheduling, accessing support services, and exploring educational resources tailored for pregnant students promotes academic success. Collaborating with the school and involving the teenager's family ensures a comprehensive approach to supporting her in managing both school responsibilities and antenatal care during pregnancy.

Subject 7: Postnatal Care

Q31: A postpartum woman reports difficulty with breastfeeding and considers switching to formula feeding. How would you assess and address her concerns to support successful breastfeeding during postnatal care? (5 marks)

Answer: I would conduct a thorough assessment of the woman's breastfeeding experience, evaluating latch, positioning, and infant feeding cues. Providing personalized breastfeeding support, addressing concerns, and offering guidance on common challenges (e.g., nipple soreness, engorgement) enhances her confidence. Collaborating with a lactation consultant and involving the woman's partner in the support process ensures a comprehensive approach to promoting successful breastfeeding during postnatal care.

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Q32: A postpartum woman experiences urinary incontinence and pelvic floor discomfort. How would you assess and manage these issues during postnatal care to enhance her quality of life? (5 marks)

Answer: I would conduct a comprehensive assessment of the woman's pelvic floor function, incorporating tools such as pelvic floor muscle assessment and voiding diaries. Providing guidance on pelvic floor exercises, lifestyle modifications, and addressing any contributing factors (e.g., constipation) supports the management of urinary incontinence and pelvic floor discomfort. Collaborating with a pelvic health physiotherapist ensures a tailored and effective approach to improving her quality of life during postnatal care.

Q33: A postpartum woman expresses concerns about postpartum depression. How would you assess her mental health during postnatal care and initiate appropriate interventions? (5 marks)

Answer: I would utilize standardized screening tools to assess the woman's mental health, exploring her emotional well-being and any signs of postpartum depression. Establishing open communication, normalizing the discussion of mental health, and providing resources for support contribute to early intervention. Collaborating with mental health professionals and involving the woman's support network ensures a comprehensive approach to addressing postpartum depression during postnatal care.

Q34: A postpartum woman with a cesarean section delivery seeks guidance on optimal recovery during postnatal care. How would you provide information on physical activity, wound care, and emotional well-being to support her recovery? (5 marks)

Answer: I would offer detailed guidance on gradual resumption of physical activity, emphasizing pelvic floor exercises and walking. Providing instructions on proper wound care, recognizing signs of infection, and promoting emotional well-being through self-care activities supports optimal recovery after a cesarean section. Collaborating with healthcare providers for postoperative follow-up and involving the woman's support network ensures a holistic approach to her recovery during postnatal care.

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Q35: A postpartum woman expresses concerns about family planning options. How would you educate her on different contraceptive methods and assist in making an informed choice during postnatal care? (5 marks)

Answer: I would provide comprehensive education on various contraceptive methods, discussing their efficacy, benefits, and potential side effects. Exploring the woman's preferences, reproductive goals, and medical history ensures a personalized approach to contraceptive counseling. Collaborating with a family planning expert, offering informational materials, and addressing any concerns contribute to the woman making an informed choice about contraception during postnatal care.

Subject 8: Conditions in Pregnancy

Q36: A pregnant woman presents with severe abdominal pain and vaginal bleeding. Outline the initial steps you would take to assess and manage her condition, considering potential causes. (5 marks)

Answer: In this scenario, the immediate priority is to assess the woman's vital signs and conduct a thorough abdominal examination. Prompt ultrasound imaging would help identify potential causes such as ectopic pregnancy, placental abruption, or other complications. If necessary, blood tests and coagulation studies may be ordered. Initiating intravenous access, providing analgesia, and collaborating with an obstetrician for further evaluation and management are essential steps to address the woman's condition.

Q37: A pregnant woman reports persistent headaches and visual disturbances. Describe the key components of your assessment and the considerations for differential diagnosis in this scenario. (5 marks)

Answer: The assessment would involve evaluating the woman's blood pressure, visual acuity, and performing a thorough neurological examination. Differential diagnosis considerations include gestational hypertension, preeclampsia, or other neurological conditions. Monitoring blood pressure regularly, conducting laboratory tests (e.g., liver function tests, platelet count), and considering urgent obstetric consultation are crucial steps. Timely intervention, such as antihypertensive medication or delivery, may be necessary based on the diagnosis.

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Q38: A pregnant woman with diabetes expresses concerns about the impact on her pregnancy. Outline your approach to providing comprehensive care, addressing both maternal and fetal well-being. (5 marks)

Answer: Comprehensive care for a pregnant woman with diabetes involves regular monitoring of blood glucose levels, adjusting insulin doses as needed, and coordinating care with endocrinologists and obstetricians. Fetal well-being is assessed through frequent ultrasounds to monitor growth and amniotic fluid levels. Collaborating with a dietitian for personalized nutritional guidance, educating the woman on self-monitoring, and involving a multidisciplinary team for coordinated care ensures optimal maternal and fetal outcomes.

Q39: A pregnant woman presents with sudden onset of shortness of breath and chest pain. Outline the steps you would take to assess and manage her condition, considering potential causes. (5 marks)

Answer: Immediate assessment would involve checking vital signs, performing a thorough respiratory and cardiovascular examination, and obtaining an electrocardiogram. Potential causes such as pulmonary embolism or cardiac issues need prompt consideration. Administering oxygen, initiating anticoagulant therapy if indicated, and arranging for urgent imaging studies (e.g., pulmonary angiography) are crucial steps. Collaboration with a cardiologist and obstetrician ensures a comprehensive approach to managing the woman's condition.

Q40: A pregnant woman presents with symptoms suggestive of urinary tract infection (UTI). Describe your approach to confirming the diagnosis, considering the unique considerations in pregnancy, and outline appropriate management. (5 marks)

Answer: Confirming a UTI diagnosis in pregnancy involves obtaining a urine sample for culture and sensitivity, considering asymptomatic bacteriuria. Due to pregnancy-related anatomical changes, asymptomatic bacteriuria is common and requires treatment to prevent complications. Prescribing safe antibiotics, such as nitrofurantoin or amoxicillin, and ensuring appropriate follow-up with repeat urine cultures are essential. Monitoring for recurrent infections and educating the woman on preventive measures complete the comprehensive management approach.

Subject 9: Abnormal Labour

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Q41: A woman in labor presents with persistent occiput posterior position. Describe the potential complications associated with this malposition and outline the interventions to facilitate fetal rotation. (5 marks)

Answer: Occiput posterior position can lead to prolonged labor, increased maternal pain, and an increased risk of instrumental delivery or cesarean section. To facilitate fetal rotation, maternal positions such as hands and knees or the "all-fours" position can be encouraged. Pelvic rocking exercises and manual rotation maneuvers by the healthcare provider may be employed. Monitoring fetal well-being and being prepared for potential interventions, such as assisted delivery, are essential components of managing persistent occiput posterior position during labor.

Q42: A pregnant woman experiences precipitous labor. Discuss the potential maternal and fetal complications associated with rapid labor and outline the immediate interventions you would initiate. (5 marks)

Answer: Precipitous labor can result in maternal lacerations, uterine atony, and fetal distress due to rapid descent. Immediate interventions include assessing fetal well-being, providing support to prevent maternal injury, and monitoring for postpartum hemorrhage. Preparing for potential complications, such as shoulder dystocia, and involving the neonatal team for prompt newborn care are crucial. Continuous support and close monitoring during the postpartum period contribute to optimal outcomes in cases of precipitous labor.

Q43: A woman in labor exhibits signs of arrest of descent. Describe the steps you would take to assess and manage this situation, considering potential causes and interventions. (5 marks)

Answer: Assessment involves evaluating the woman's pelvis, fetal position, and progress in labor. Potential causes may include cephalopelvic disproportion or malpresentation. Management includes repositioning the woman, encouraging spontaneous pushing efforts, and considering assisted delivery with vacuum or forceps. Continuous fetal monitoring and prompt decision-making for cesarean section if progress does not resume are critical components. Collaboration with an obstetrician ensures a coordinated approach to managing arrest of descent during labor.

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Q44: A woman in labor presents with a nuchal cord. Describe your approach to assessing and managing this situation, considering potential complications and interventions. (5 marks)

Answer: Assessment involves confirming the presence of a nuchal cord through palpation or ultrasound. Potential complications include fetal distress or compromised blood flow. Management may involve gentle cord unwinding, especially if loose, or clamping and cutting the cord during delivery. Monitoring fetal well-being and being prepared for potential complications, such as meconium-stained amniotic fluid, are essential. Close collaboration with the obstetric team ensures a comprehensive approach to managing a nuchal cord during labor.

Q45: A woman in labor exhibits signs of cephalopelvic disproportion. Outline the steps you would take to confirm the diagnosis, considering potential interventions, and describe the role of instrumental delivery or cesarean section. (5 marks)

Answer: Confirming cephalopelvic disproportion involves assessing the progress of labor, maternal pelvis, and fetal position. If diagnosed, interventions may include optimizing maternal positioning, encouraging spontaneous pushing, or considering assisted delivery with vacuum or forceps. If these measures are ineffective, planning for cesarean section is crucial to prevent potential complications such as prolonged labor, fetal distress, or birth injuries. Collaborating with an obstetrician ensures a timely and coordinated approach to managing cephalopelvic disproportion during labor.

Subject 10: Baby at Risk

Q46: A newborn exhibits signs of respiratory distress shortly after birth. Outline the steps you would take to assess and manage the baby's respiratory status, considering potential causes and interventions. (5 marks)

Answer: Immediate assessment involves evaluating the newborn's respiratory rate, chest movement, and color. Potential causes may include meconium aspiration or transient tachypnea of the newborn. Management includes providing respiratory support with continuous positive airway pressure (CPAP) or, if severe, intubation. Monitoring oxygen saturation, addressing contributing factors, and coordinating care with neonatal specialists ensure a comprehensive approach to managing respiratory distress in a newborn.

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Q47: A newborn presents with jaundice within the first 24 hours. Describe your approach to assessing and managing neonatal jaundice, considering potential causes and interventions. (5 marks)

Answer: Assessment involves measuring bilirubin levels, evaluating the newborn's clinical condition, and considering risk factors. Potential causes may include physiological jaundice or hemolytic conditions. Management includes phototherapy, monitoring bilirubin levels, and addressing underlying causes if needed. Educating parents on signs of worsening jaundice and the importance of follow-up ensures comprehensive care for a newborn with jaundice.

Q48: A preterm newborn exhibits signs of respiratory distress syndrome (RDS). Discuss your approach to assessing and managing RDS, considering potential complications and interventions. (5 marks)

Answer: Assessment involves clinical examination, chest X-rays, and blood gas analysis to confirm the diagnosis of respiratory distress syndrome (RDS). Potential complications include pneumothorax or bronchopulmonary dysplasia. Management includes administering surfactant therapy, providing respiratory support with mechanical ventilation or continuous positive airway pressure (CPAP), and addressing potential complications promptly. Collaboration with neonatal specialists ensures a comprehensive approach to managing RDS in a preterm newborn.

Q49: A term newborn presents with hypoglycemia. Describe your approach to assessing and managing neonatal hypoglycemia, considering potential causes and interventions. (5 marks)

Answer: Assessment involves monitoring blood glucose levels, evaluating the newborn's clinical condition, and considering risk factors such as maternal diabetes. Potential causes may include inadequate glycogen stores or hyperinsulinism. Management includes initiating early feeding, monitoring blood glucose levels, and intravenous glucose infusion if needed. Educating parents on feeding practices and signs of hypoglycemia contributes to the comprehensive care of a newborn with hypoglycemia.

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Q50: A newborn is born to a mother with a history of substance abuse. Discuss your approach to assessing and managing neonatal abstinence syndrome (NAS), considering potential complications and interventions. (5 marks)

Answer: Assessment involves monitoring the newborn for signs of withdrawal, including irritability, feeding difficulties, and tremors. Potential complications may include seizures or failure to thrive. Management includes providing a calm environment, supportive care, and pharmacological interventions, such as opioid replacement therapy, if severe. Collaborating with pediatricians and involving the mother in the care plan ensures a comprehensive approach to managing neonatal abstinence syndrome in a newborn born to a mother with a history of substance abuse.

Subject 11: Malposition and Malpresentation

Q51: A woman in active labor is diagnosed with occiput transverse malposition. Discuss the potential challenges in the birthing process associated with this malposition and outline the interventions you would implement to facilitate a safe delivery. (5 marks)

Answer: Occiput transverse malposition can lead to prolonged labor, increased pain, and an increased risk of instrumental delivery. Interventions include maternal position changes, pelvic rocking exercises, and manual rotation maneuvers by the healthcare provider. Utilizing a birthing ball and encouraging side-lying positions can enhance fetal rotation. Continuous fetal monitoring and readiness for potential interventions, such as assisted delivery, ensure a comprehensive approach to managing occiput transverse malposition during labor.

Q52: A pregnant woman is diagnosed with breech presentation during the third trimester. Discuss the potential complications associated with breech presentation and outline the considerations for mode of delivery, emphasizing the role of healthcare providers in decision-making. (5 marks)

Answer: Breech presentation increases the risk of birth injuries and complications during delivery. Healthcare providers must thoroughly assess the type of breech presentation and associated risk factors. Depending on factors such as fetal weight, presentation, and maternal health, either a vaginal breech delivery or a planned cesarean section may be recommended. A multidisciplinary approach involving obstetricians, midwives, and neonatal specialists ensures informed decision-making and optimal outcomes for both the mother and the baby.

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Q53: A woman in labor is diagnosed with shoulder dystocia. Discuss the potential fetal and maternal complications associated with shoulder dystocia and outline the immediate interventions you would implement to manage this obstetric emergency. (5 marks)

Answer: Shoulder dystocia can lead to fetal distress, brachial plexus injuries, and postpartum hemorrhage. Immediate interventions include McRoberts maneuver, suprapubic pressure, and rotational maneuvers to release the impacted shoulder. Preparing for potential complications, such as fetal resuscitation or postpartum hemorrhage management, is crucial. Continuous communication with the woman, involving additional personnel if needed, and documentation of the events ensure a coordinated and timely approach to managing shoulder dystocia.

Q54: A woman in active labor presents with compound presentation. Discuss the potential complications associated with compound presentation and outline the steps you would take to assess and manage this situation, considering both maternal and fetal well-being. (5 marks)

Answer: Compound presentation, where a hand or arm presents alongside the fetal head, increases the risk of birth injuries and obstructed labor. Assessment involves confirming the diagnosis through clinical examination and ultrasound. Management includes attempting to manually replace the prolapsed limb, optimizing maternal positioning, and preparing for potential complications, such as cesarean section or instrumental delivery. Close monitoring of fetal well-being, collaboration with obstetricians, and effective communication with the woman contribute to a comprehensive approach to managing compound presentation.

Q55: A woman in labor is diagnosed with face presentation. Discuss the potential challenges in delivering a baby in face presentation and outline the considerations for mode of delivery, emphasizing the role of healthcare providers in decision-making. (5 marks)

Answer: Face presentation poses challenges in delivering the head, potentially leading to birth injuries and prolonged labor. Assessment involves confirming the diagnosis through clinical examination and ultrasound. The decision regarding the mode of delivery depends on factors such as fetal position, maternal health, and presentation type. Collaborating with obstetricians, discussing potential complications and interventions, and ensuring continuous fetal monitoring contribute to informed decision-making for the most appropriate mode of delivery in the case of face presentation.

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Subject 12: Obstetric Emergency

Q56: A pregnant woman presents with eclampsia in the antenatal clinic. Discuss the immediate interventions you would initiate to manage eclampsia, emphasizing the importance of prompt and coordinated care to ensure maternal and fetal well-being. (5 marks)

Answer: Immediate interventions for eclampsia involve ensuring the woman's safety, providing magnesium sulfate for seizure control, and monitoring vital signs. Emergency obstetric consultation is essential for assessment and planning. Continuous fetal monitoring, addressing hypertension, and preparing for potential complications such as preterm delivery or cesarean section are crucial. Coordinated care with a multidisciplinary team, including anesthetists and neonatal specialists, ensures a timely and comprehensive approach to managing eclampsia and optimizing maternal and fetal outcomes.

Q57: A pregnant woman experiences abruptio placentae in the third trimester. Outline the potential maternal and fetal complications associated with abruptio placentae and describe the steps you would take to assess and manage this obstetric emergency. (5 marks)

Answer: Abruptio placentae can lead to maternal hemorrhage, disseminated intravascular coagulation (DIC), and fetal distress. Assessment involves clinical examination, ultrasound, and monitoring for signs of shock. Immediate management includes resuscitation, blood transfusion, and preparing for potential interventions, such as emergency cesarean section. Coordinated care with obstetricians, anesthetists, and neonatal specialists ensures a comprehensive approach to managing abruptio placentae and minimizing potential complications for both the mother and the baby.

Q58: A woman in active labor presents with umbilical cord prolapse. Discuss the potential fetal and maternal complications associated with umbilical cord prolapse and outline the immediate interventions you would implement to manage this obstetric emergency. (5 marks)

Answer: Umbilical cord prolapse can lead to fetal distress, especially if compression occurs during contractions. Immediate interventions include relieving pressure on the cord through maternal positioning, initiating emergency cesarean section, and providing oxygen to the mother. Monitoring fetal heart rate and preparing for potential complications, such as neonatal

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resuscitation, are crucial. Coordinated care with obstetricians and neonatal specialists ensures a timely and comprehensive approach to managing umbilical cord prolapse.

Q59: A pregnant woman in her third trimester presents with uncontrolled bleeding. Discuss the potential causes of antepartum hemorrhage and outline the steps you would take to assess and manage this obstetric emergency, emphasizing the importance of preserving maternal and fetal well-being. (5 marks)

Answer: Antepartum hemorrhage can result from placenta previa, placental abruption, or uterine rupture. Assessment involves clinical examination, ultrasound, and monitoring vital signs. Immediate management includes resuscitation, blood transfusion, and preparing for potential interventions, such as emergency cesarean section. Continuous fetal monitoring, addressing the underlying cause, and involving a multidisciplinary team ensure a comprehensive approach to managing antepartum hemorrhage and optimizing maternal and fetal outcomes.

Q60: A pregnant woman in her second trimester presents with signs of miscarriage. Discuss the potential emotional and physical complications associated with miscarriage and outline the steps you would take to support the woman during this difficult time. (5 marks)

Answer: Miscarriage can result in emotional distress, grief, and potential physical complications such as infection or bleeding. Supporting the woman involves empathetic communication, providing information on the grieving process, and offering options for managing the miscarriage, including expectant, medical, or surgical management. Collaborating with mental health professionals, involving the woman's support network, and ensuring follow-up care contribute to a compassionate and comprehensive approach to supporting a woman experiencing miscarriage.

Subject 13: Abnormal Puerperium

Q61: A postpartum woman reports persistent perineal pain and difficulty with bowel movements after an episiotomy. Discuss the potential complications associated with episiotomy and outline the steps you would take to assess and manage this situation, emphasizing the importance of preserving maternal comfort and preventing further complications. (5 marks)

Answer: Episiotomy complications may include infection, hematoma, or impaired wound healing. Assessment involves clinical examination and considering the woman's comfort. Management

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includes pain relief measures, promoting perineal hygiene, and addressing potential complications, such as drainage for hematomas or antibiotics for infections. Continuous monitoring and collaboration with healthcare providers ensure a comprehensive approach to managing episiotomy complications during the postpartum period while prioritizing maternal comfort.

Q62: A postpartum woman presents with symptoms of postpartum depression. Discuss the key components of your assessment to confirm the diagnosis and outline the interventions you would initiate to support the woman's mental health, emphasizing the role of a multidisciplinary approach in postpartum care. (5 marks)

Answer: Assessing postpartum depression involves standardized screening tools, exploring the woman's emotional well-being, and considering risk factors. Interventions include open communication, normalization of mental health discussions, and providing resources for support. Collaborating with mental health professionals, involving the woman's support network, and offering counseling contribute to a comprehensive approach to addressing postpartum depression during the puerperium.

Q63: A postpartum woman presents with heavy vaginal bleeding after childbirth. Discuss the potential causes of postpartum hemorrhage during the puerperium and outline the immediate interventions you would implement to manage this obstetric emergency, emphasizing the importance of preserving maternal and fetal well-being. (5 marks)

Answer: Postpartum hemorrhage can result from uterine atony, retained placental tissue, or genital tract trauma. Immediate interventions include uterine massage, administration of uterotonic medications, and preparing for potential surgical interventions. Continuous monitoring of vital signs, addressing the underlying cause, and involving a multidisciplinary team ensure a comprehensive approach to managing postpartum hemorrhage during the puerperium, prioritizing the well-being of both the mother and the baby.

Q64: A postpartum woman reports persistent urinary incontinence after childbirth. Discuss the potential contributing factors to postpartum urinary incontinence and outline the steps you would take to assess and manage this condition, emphasizing the importance of preserving the woman's quality of life during the puerperium. (5 marks)

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Answer: Postpartum urinary incontinence may result from pelvic floor weakness or injury during childbirth. Assessment involves evaluating the woman's symptoms, pelvic floor examination, and considering risk factors. Management includes pelvic floor exercises, lifestyle modifications, and referral to pelvic health physiotherapy. Providing education on bladder health, offering support, and involving the woman in decision-making contribute to a comprehensive approach to managing postpartum urinary incontinence during the puerperium while preserving the woman's quality of life.

Q65: A postpartum woman presents with symptoms suggestive of mastitis. Discuss the potential complications of mastitis during the puerperium and outline the steps you would take to assess and manage this condition, emphasizing the importance of preserving breastfeeding and preventing further complications. (5 marks)

Answer: Mastitis complications may include breast abscess formation or impaired breastfeeding. Assessment involves clinical examination, considering the woman's breastfeeding technique, and identifying risk factors. Management includes antibiotic therapy, promoting frequent breastfeeding or pumping, and ensuring adequate hydration. Continuous monitoring, addressing potential complications promptly, and providing breastfeeding support contribute to a comprehensive approach to managing mastitis during the puerperium while preserving breastfeeding and preventing further complications.

Subject 14: Reproductive Health

Q66: A woman presents to the reproductive health clinic seeking contraception. Discuss the key considerations in providing personalized contraceptive counseling, considering the woman's reproductive goals, medical history, and preferences. Outline the various contraceptive methods available and their potential benefits and side effects. (5 marks)

Answer: Personalized contraceptive counseling involves exploring the woman's reproductive goals, medical history, and preferences. Methods such as hormonal contraceptives, intrauterine devices, barrier methods, and permanent options should be discussed. Benefits and potential side effects, including efficacy rates and considerations for long-term use, should be thoroughly explained. Providing informational materials, addressing concerns, and involving the woman in decision-making ensure a comprehensive approach to contraceptive counseling in the reproductive health clinic.

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Q67: A sexually active woman presents with symptoms suggestive of a sexually transmitted infection (STI). Discuss the key components of your assessment to confirm the diagnosis and outline the interventions you would initiate, emphasizing the importance of partner notification and prevention of further transmission. (5 marks)

Answer: Confirming an STI diagnosis involves a detailed sexual history, clinical examination, and appropriate laboratory tests. Interventions include prescribing appropriate antibiotics, providing partner notification guidance, and offering preventive measures. Counseling on safe sex practices, emphasizing the importance of partner testing, and addressing psychosocial aspects contribute to a comprehensive approach to managing STIs in the reproductive health setting while preventing further transmission.

Q68: A woman seeks preconception care in the reproductive health clinic. Discuss the key components of preconception care, considering both maternal and fetal well-being. Outline the importance of addressing medical conditions, optimizing nutrition, and providing counseling on lifestyle factors to promote a healthy pregnancy. (5 marks)

Answer: Preconception care involves addressing medical conditions, optimizing nutrition, and providing counseling on lifestyle factors to promote a healthy pregnancy. Components include managing chronic conditions, assessing immunization status, and discussing the importance of folic acid supplementation. Addressing modifiable risk factors such as smoking and alcohol consumption, offering genetic counseling, and involving the woman's partner contribute to a comprehensive approach to preconception care in the reproductive health clinic, prioritizing maternal and fetal well-being.

Q69: A woman with infertility concerns seeks evaluation in the reproductive health clinic. Discuss the key components of your assessment to identify potential causes of infertility and outline the interventions you would initiate, emphasizing the importance of emotional support and a collaborative approach with fertility specialists. (5 marks)

Answer: Infertility assessment involves a detailed reproductive history, assessing ovulatory function, and investigating potential causes such as hormonal imbalances or structural abnormalities. Interventions may include fertility medications, assisted reproductive technologies, or surgical interventions. Providing emotional support, offering counseling, and involving fertility

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specialists in a collaborative approach contribute to a comprehensive approach to managing infertility in the reproductive health clinic, addressing both medical and emotional aspects.

Q70: A woman in her postmenopausal years seeks advice on managing menopausal symptoms. Discuss the various options for managing menopausal symptoms, considering hormonal and non-hormonal interventions. Outline the potential benefits and risks associated with hormone replacement therapy and alternative approaches to promote women's health during the menopausal transition. (5 marks)

Answer: Managing menopausal symptoms involves discussing hormonal and non-hormonal interventions. Hormone replacement therapy benefits include symptom relief and bone health, but risks such as cardiovascular events should be considered. Non-hormonal options include lifestyle modifications, herbal supplements, and cognitive-behavioral therapy. Providing personalized information, addressing individual preferences, and discussing long-term health considerations ensure a comprehensive approach to managing menopausal symptoms in the reproductive health clinic, promoting women's health during the menopausal transition.

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Subject 15: Conditions in Pregnancy

Q71: A pregnant woman presents with hyperemesis gravidarum. Discuss the pathophysiology of hyperemesis gravidarum, its potential impact on maternal and fetal well-being, and outline a comprehensive nursing care plan for managing this condition. (20 marks)

Answer: Hyperemesis gravidarum is characterized by severe and persistent nausea and vomiting during pregnancy. The pathophysiology involves hormonal changes, particularly elevated levels of human chorionic gonadotropin (hCG). This condition can lead to dehydration, electrolyte imbalance, and weight loss, posing risks to both the mother and the fetus.

A comprehensive nursing care plan includes assessing the severity of symptoms, monitoring vital signs, and ensuring adequate hydration through intravenous fluids. Collaborating with a nutritionist to provide small, frequent meals and educating the woman on dietary modifications supports nutritional needs. Medications such as antiemetics may be prescribed under medical supervision. Emotional support and counseling help address the psychological impact of hyperemesis gravidarum. Regular follow-ups and continuous monitoring contribute to effective management, ensuring optimal maternal and fetal outcomes.

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Q72: A pregnant woman is diagnosed with gestational diabetes mellitus (GDM). Discuss the implications of GDM on maternal and fetal health, outline the components of antenatal nursing care for a woman with GDM, and explain the importance of a multidisciplinary approach in managing this condition. (20 marks)

Answer: Gestational diabetes mellitus (GDM) poses risks such as macrosomia, birth injuries, and maternal complications. Antenatal nursing care involves monitoring blood glucose levels, providing dietary guidance, and educating the woman on insulin administration if needed. Collaborating with a dietitian supports personalized nutritional plans, while regular fetal monitoring ensures early detection of complications.

A multidisciplinary approach involves working closely with endocrinologists, obstetricians, and diabetic educators. This collaboration allows for comprehensive care, addressing both maternal and fetal needs. Ensuring proper glycemic control, promoting lifestyle modifications, and providing emotional support contribute to effective antenatal nursing care for women with GDM, optimizing outcomes for both the mother and the baby.

Q73: A pregnant woman is diagnosed with placenta previa during routine ultrasound. Discuss the implications of placenta previa on the course of pregnancy and childbirth, outline the nursing interventions to manage this condition, and emphasize the role of education and emotional support for the woman and her family. (20 marks)

Answer: Placenta previa can lead to antepartum hemorrhage and poses risks during childbirth. Nursing interventions include monitoring for signs of bleeding, educating the woman on activity restrictions, and preparing for potential complications, such as preterm delivery or cesarean section. Continuous fetal monitoring ensures timely detection of distress.

Education involves explaining the condition, emphasizing the importance of compliance with restrictions, and providing information on potential complications. Emotional support includes addressing anxiety, offering resources for coping strategies, and involving the woman's family in the care plan. Collaboration with obstetricians ensures a coordinated approach, optimizing maternal and fetal outcomes while promoting a supportive and informed healthcare experience.

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Q74: A pregnant teenager presents with preeclampsia. Discuss the unique challenges and considerations in providing nursing care for a pregnant teenager with preeclampsia. Outline the nursing interventions to manage preeclampsia in this population, and emphasize the importance of psychosocial support and education. (20 marks)

Answer: Managing preeclampsia in a pregnant teenager involves unique challenges due to physical and psychosocial factors. Nursing interventions include monitoring blood pressure, assessing for symptoms of preeclampsia, and ensuring compliance with prescribed medications. Providing education on the importance of prenatal care, adherence to treatment plans, and potential complications is crucial.

Psychosocial support addresses the emotional and social aspects of teenage pregnancy and preeclampsia. Counseling on future contraceptive options, involving the teenager's family in the care plan, and connecting with social services contribute to holistic care. Regular follow-ups, continuous monitoring, and collaboration with a multidisciplinary team ensure optimal outcomes for both the pregnant teenager and the baby.

Q75: A pregnant woman with a history of substance abuse is admitted for opioid dependency. Discuss the nursing considerations and challenges in providing care for a pregnant woman with opioid dependency. Outline the nursing interventions to address withdrawal symptoms, optimize maternal-fetal outcomes, and emphasize the importance of a non-judgmental approach. (20 marks)

Answer: Providing care for a pregnant woman with opioid dependency requires a non-judgmental and supportive approach. Nursing considerations include assessing the severity of substance use, monitoring withdrawal symptoms, and addressing potential complications, such as preterm labor or neonatal abstinence syndrome. Collaboration with addiction specialists and social services ensures comprehensive care.

Nursing interventions involve implementing withdrawal protocols, providing medications under medical supervision, and educating the woman on the potential effects on fetal development. Emphasizing prenatal care, encouraging participation in substance abuse treatment programs, and offering emotional support contribute to a holistic approach. Regular monitoring, involvement of the woman's support network, and continuous collaboration with healthcare providers contribute to optimizing maternal and fetal outcomes.

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Subject 16: Abnormal Labour

Q76: A woman in active labor presents with arrested descent. Discuss the potential causes of arrested descent during labor and outline the nursing interventions you would implement to address this situation, emphasizing the importance of preserving maternal and fetal well-being. (5 marks)

Answer: Causes of arrested descent can include cephalopelvic disproportion or inadequate uterine contractions. Nursing interventions involve assessing the woman's labor progress, ensuring optimal maternal positioning, and providing coaching during contractions. Collaborating with obstetricians for potential interventions, such as vacuum extraction or cesarean section, is crucial. Continuous fetal monitoring and addressing the underlying cause contribute to a comprehensive approach, prioritizing the well-being of both the mother and the baby.

Q77: A woman in labor presents with precipitous labor. Discuss the potential complications associated with precipitous labor and outline the nursing interventions you would initiate to manage this situation, emphasizing the importance of a prompt and coordinated approach to care. (5 marks)

Answer: Precipitous labor can lead to fetal distress, birth injuries, and postpartum hemorrhage. Nursing interventions involve assessing the woman's labor progress, preparing for rapid delivery, and ensuring continuous fetal monitoring. Supporting the woman emotionally, coaching during delivery, and being prepared for potential complications contribute to effective management. Collaboration with a multidisciplinary team, including neonatal specialists, ensures a prompt and coordinated approach, optimizing outcomes for both the mother and the baby.

Q78: A woman in labor is diagnosed with a uterine rupture. Discuss the potential maternal and fetal complications associated with uterine rupture and outline the immediate nursing interventions you would implement to manage this obstetric emergency. (5 marks)

Answer: Uterine rupture can lead to maternal hemorrhage, fetal distress, and neonatal complications. Immediate nursing interventions involve assessing vital signs, preparing for emergency cesarean section, and providing intravenous fluids. Continuous fetal monitoring and collaboration with obstetricians for potential interventions contribute to a timely and comprehensive approach. Addressing potential complications, such as postpartum hemorrhage or

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neonatal resuscitation, ensures effective management of uterine rupture, prioritizing the well-being of both the mother and the baby.

Q79: A woman in labor presents with a prolonged second stage. Discuss the potential maternal and fetal complications associated with a prolonged second stage of labor and outline the nursing interventions you would implement to manage this situation, emphasizing the importance of a balanced approach to care. (5 marks)

Answer: A prolonged second stage can lead to maternal exhaustion, fetal distress, and increased risk of instrumental delivery. Nursing interventions involve assessing maternal and fetal well-being, providing coaching during contractions, and promoting optimal positioning. Collaboration with obstetricians for potential interventions, such as vacuum extraction or cesarean section, is crucial. Balancing the need for effective progress with minimizing complications contributes to a comprehensive approach, prioritizing the well-being of both the mother and the baby.

Q80: A woman in labor presents with a nuchal cord. Discuss the potential complications associated with a nuchal cord and outline the nursing interventions you would implement to manage this situation, emphasizing the importance of preserving fetal well-being. (5 marks)

Answer: A nuchal cord occurs when the umbilical cord wraps around the fetal neck. Potential complications include fetal distress and compromised blood flow. Nursing interventions involve assessing fetal heart rate, documenting the number of cord loops, and ensuring continuous fetal monitoring. Encouraging maternal position changes and preparing for potential interventions, such as guided delivery or cesarean section, contribute to effective management. Continuous communication with the woman and readiness for potential complications ensures a comprehensive approach, prioritizing the well-being of the baby.

Subject 17: Baby at Risk

Q81: A newborn presents with signs of meconium aspiration syndrome (MAS). Discuss the pathophysiology of MAS, its potential impact on respiratory function, and outline the nursing care plan for managing a newborn with MAS, emphasizing the importance of respiratory support and monitoring. (5 marks)

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Answer: Meconium aspiration syndrome (MAS) occurs when a newborn inhales meconium-stained amniotic fluid. Pathophysiology involves airway obstruction, inflammation, and surfactant dysfunction. MAS can lead to respiratory distress and hypoxia. The nursing care plan includes assessing respiratory distress severity, providing oxygen therapy, and monitoring blood gases. Collaboration with neonatal specialists for potential interventions, such as mechanical ventilation or surfactant administration, is crucial. Continuous respiratory monitoring and addressing potential complications ensure optimal care for a newborn with MAS.

Q82: A preterm newborn is admitted to the neonatal intensive care unit (NICU) with respiratory distress syndrome (RDS). Discuss the pathophysiology of RDS, its potential complications, and outline the nursing care plan for managing a preterm newborn with RDS, emphasizing the importance of respiratory support and surfactant therapy. (5 marks)

Answer: Respiratory distress syndrome (RDS) in preterm newborns results from surfactant deficiency. Complications include atelectasis and pneumothorax. The nursing care plan involves assessing respiratory distress severity, providing continuous positive airway pressure (CPAP) or mechanical ventilation, and administering exogenous surfactant. Collaborating with neonatal specialists, monitoring for potential complications, and ensuring a warm and supportive environment contribute to effective care for a preterm newborn with RDS.

Q83: A newborn is diagnosed with hypoxic-ischemic encephalopathy (HIE). Discuss the potential neurological complications of HIE and outline the nursing care plan for managing a newborn with HIE, emphasizing the importance of therapeutic hypothermia and neuroprotective measures. (5 marks)

Answer: Hypoxic-ischemic encephalopathy (HIE) can lead to neurological complications, including seizures and long-term developmental disabilities. The nursing care plan involves assessing neurological status, initiating therapeutic hypothermia, and providing neuroprotective measures, such as minimizing stimuli and promoting a quiet environment. Collaboration with neonatal neurologists, continuous monitoring for seizures, and involving the family in care contribute to optimal outcomes for a newborn with HIE.

Q84: A newborn is admitted to the NICU with neonatal abstinence syndrome (NAS) due to maternal opioid use. Discuss the potential complications of NAS and outline the nursing care plan for managing a newborn with NAS, emphasizing the importance of pharmacological and supportive interventions. (5 marks)

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Answer: Neonatal abstinence syndrome (NAS) can lead to irritability, feeding difficulties, and respiratory distress. The nursing care plan involves assessing NAS severity, providing pharmacological interventions, such as opioid replacement therapy, and offering supportive care, including swaddling and minimizing stimuli. Collaboration with neonatal specialists, continuous monitoring for withdrawal symptoms, and involving the mother in care contribute to effective management of a newborn with NAS.

Q85: A term newborn is diagnosed with congenital heart disease. Discuss the potential cardiovascular complications associated with congenital heart disease and outline the nursing care plan for managing a newborn with congenital heart disease, emphasizing the importance of cardiovascular assessment and supportive interventions. (5 marks)

Answer: Congenital heart disease in a newborn can lead to cardiovascular complications, including cyanosis and heart failure. The nursing care plan involves assessing cardiovascular status, monitoring oxygen saturation, and providing supportive interventions, such as administering medications or promoting optimal positioning. Collaboration with pediatric cardiologists, continuous monitoring for signs of deterioration, and involving the family in care contribute to effective management of a newborn with congenital heart disease.

Subject 18: Obstetric Emergency

Q86: A pregnant woman presents to the emergency department with symptoms of eclampsia. Discuss the pathophysiology of eclampsia, its potential impact on maternal and fetal well-being, and outline the immediate nursing interventions and collaboration with healthcare providers you would implement to manage this critical situation. (20 marks)

Answer: Eclampsia involves seizures in a pregnant woman with preeclampsia, resulting from cerebral vasospasm. This condition can lead to maternal and fetal compromise, including hypoxia and placental insufficiency. Immediate nursing interventions include ensuring a patent airway, administering magnesium sulfate for seizure control, and monitoring vital signs. Collaboration with obstetricians for emergent delivery considerations, such as induction or cesarean section, is crucial. Continuous monitoring, providing antihypertensive medications, and addressing potential complications contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

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Q87: A pregnant woman presents to the labor and delivery unit with a suspected amniotic fluid embolism (AFE). Discuss the pathophysiology of AFE, its potential impact on maternal and fetal well-being, and outline the nursing interventions and collaboration with healthcare providers you would implement to manage this life-threatening obstetric emergency. (20 marks)

Answer: Amniotic fluid embolism (AFE) involves the entry of amniotic fluid into the maternal circulation, leading to an allergic-like reaction and coagulopathy. This condition can result in cardiopulmonary collapse and disseminated intravascular coagulation. Nursing interventions include initiating advanced life support measures, administering blood products, and providing respiratory support. Collaboration with a multidisciplinary team for emergent cesarean section considerations and continuous monitoring for potential complications are crucial. Addressing coagulopathy, ensuring hemodynamic stability, and providing emotional support contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

Q88: A pregnant woman is diagnosed with a uterine rupture during labor. Discuss the potential maternal and fetal complications associated with uterine rupture, and outline the immediate nursing interventions, communication strategies, and collaboration with healthcare providers you would implement to manage this obstetric emergency. (20 marks)

Answer: Uterine rupture can lead to maternal hemorrhage, fetal distress, and neonatal complications. Immediate nursing interventions involve assessing vital signs, preparing for emergency cesarean section, and providing intravenous fluids. Effective communication with the woman, her family, and healthcare providers is essential. Collaborating with obstetricians for emergent delivery, addressing potential complications, and continuous fetal monitoring are crucial. Ensuring emotional support, involving the woman in decision-making, and providing thorough postoperative care contribute to a comprehensive approach, prioritizing the well-being of both the mother and the baby.

Q89: A pregnant woman presents to the emergency department with a suspected placental abruption. Discuss the pathophysiology of placental abruption, its potential impact on maternal and fetal well-being, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this critical obstetric emergency. (20 marks)

Answer: Placental abruption involves the premature separation of the placenta from the uterine wall. This condition can lead to maternal hemorrhage, fetal distress, and neonatal complications. Nursing assessments include monitoring for vaginal bleeding, assessing uterine tone, and

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continuous fetal monitoring. Immediate interventions involve establishing intravenous access, administering blood products, and preparing for emergent delivery if necessary. Collaboration with obstetricians for potential surgical interventions, addressing coagulopathy, and continuous monitoring for signs of deterioration are crucial. Providing emotional support, involving the woman in care decisions, and thorough postoperative care contribute to a comprehensive approach, prioritizing the well-being of both the mother and the baby.

Q90: A pregnant woman is admitted with suspected septic shock. Discuss the potential sources of infection during pregnancy, the pathophysiology of septic shock, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this obstetric emergency. (20 marks)

Answer: Infection during pregnancy can arise from various sources, such as urinary tract infections or chorioamnionitis. Septic shock involves a systemic inflammatory response to infection, leading to circulatory compromise. Nursing assessments include monitoring vital signs, assessing for signs of infection, and continuous fetal monitoring. Immediate interventions involve administering broad-spectrum antibiotics, providing fluid resuscitation, and addressing potential complications, such as disseminated intravascular coagulation. Collaboration with infectious disease specialists, obstetricians, and critical care teams is crucial. Continuous monitoring for signs of improvement, ensuring emotional support, and addressing the underlying infection contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

Subject 19: Abnormal Peurperum

Q91: A postpartum woman is diagnosed with postpartum hemorrhage (PPH) due to uterine atony. Discuss the potential causes of uterine atony leading to PPH, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum complication. (20 marks)

Answer: Uterine atony, a common cause of postpartum hemorrhage (PPH), can result from factors like overdistension or retained placental fragments. Nursing assessments involve monitoring for excessive bleeding, assessing uterine tone, and vital signs. Immediate interventions include massaging the uterus, administering uterotonic medications, and establishing intravenous access. Collaboration with obstetricians for potential surgical interventions, such as uterine exploration or ligation of bleeding vessels, is crucial. Continuous monitoring for signs of improvement, addressing potential complications, and involving the woman in care decisions contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

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Q92: A postpartum woman is admitted with suspected deep vein thrombosis (DVT). Discuss the risk factors for DVT during the postpartum period, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum complication. (20 marks)

Answer: Postpartum deep vein thrombosis (DVT) risk factors include immobility, cesarean section, and maternal age. Nursing assessments involve monitoring for signs of DVT, assessing lower extremity circulation, and obtaining a detailed health history. Immediate interventions include initiating anticoagulant therapy, providing compression stockings, and promoting early ambulation. Collaboration with vascular specialists for diagnostic imaging, continuous monitoring for signs of improvement, and patient education on prevention strategies contribute to effective management. Addressing potential complications, ensuring emotional support, and involving the woman in care decisions contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

Q93: A postpartum woman presents with symptoms of postpartum depression (PPD). Discuss the risk factors for PPD, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum mental health complication. (20 marks)

Answer: Postpartum depression (PPD) risk factors include a history of depression, lack of social support, and hormonal fluctuations. Nursing assessments involve screening for depressive symptoms, assessing the woman's social support, and collaborating with mental health professionals for further evaluation. Interventions include providing emotional support, promoting self-care strategies, and initiating therapy or medication as appropriate. Collaboration with psychologists or psychiatrists for ongoing mental health care, continuous monitoring for signs of improvement, and involving the woman's support network contribute to effective management. Addressing potential complications, educating the woman on coping mechanisms, and involving her in care decisions contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

Q94: A postpartum woman is diagnosed with puerperal sepsis. Discuss the potential sources of infection leading to puerperal sepsis, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum complication. (20 marks)

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Answer: Puerperal sepsis can result from infections of the genital tract, such as endometritis or perineal infections. Nursing assessments involve monitoring for signs of infection, assessing vital signs, and obtaining a detailed health history. Immediate interventions include initiating broad-spectrum antibiotics, providing intravenous fluids, and addressing potential complications, such as septic shock. Collaboration with infectious disease specialists for ongoing antimicrobial therapy, continuous monitoring for signs of improvement, and involving obstetricians for potential surgical interventions contribute to effective management. Addressing potential complications, ensuring emotional support, and involving the woman in care decisions contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

Q95: A postpartum woman is diagnosed with diastasis recti abdominis. Discuss the potential causes of diastasis recti abdominis during the postpartum period, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum complication. (20 marks)

Answer: Diastasis recti abdominis involves the separation of the abdominal muscles, often occurring during pregnancy. Nursing assessments involve visual inspection of the abdomen, assessing for muscle tone, and obtaining a detailed health history. Interventions include providing education on abdominal exercises, promoting proper body mechanics, and referring to physical therapy for targeted exercises. Collaboration with physical therapists for ongoing rehabilitation, continuous monitoring for signs of improvement, and involving the woman in care decisions contribute to effective management. Addressing potential complications, ensuring emotional support, and educating the woman on preventive measures contribute to a comprehensive approach, optimizing outcomes for both the mother and the baby.

Subject 20: Family Planning

Q96: A couple seeks family planning advice after the birth of their second child. Discuss the different methods of contraception available, their advantages, and potential side effects. Provide counseling strategies for the couple, considering their preferences and reproductive health history. (20 marks)

Answer: Contraceptive methods include hormonal options (oral contraceptives, patches, injections), barrier methods (condoms, diaphragms), intrauterine devices (IUDs), and permanent methods (sterilization). Hormonal methods offer high efficacy but may have side effects like nausea. Barrier methods provide protection against sexually transmitted infections but require

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consistent use. IUDs are long-acting and reversible. Permanent methods offer permanent contraception but require careful consideration.

Counseling involves assessing the couple's preferences, addressing concerns about side effects, and considering their reproductive goals. Discussing the importance of consistent use and providing information on backup methods enhance contraceptive efficacy. Considering individual health histories ensures safe and suitable choices. Encouraging open communication and involving both partners in the decision-making process contribute to a comprehensive family planning strategy.

Q97: A woman experiences contraceptive failure with an oral contraceptive pill. Discuss the potential causes of contraceptive failure, and outline the nursing assessments, interventions, and counseling strategies you would implement to manage this situation. (20 marks)

Answer: Contraceptive failure can result from missed pills, drug interactions, or gastrointestinal issues. Nursing assessments involve determining the circumstances of pill use, assessing reproductive health history, and confirming the possibility of pregnancy. Immediate interventions include providing emergency contraception, discussing alternative methods, and addressing potential barriers to consistent pill use.

Counseling involves reviewing proper pill administration, discussing the importance of backup methods, and considering alternative contraceptives. Assessing the woman's understanding of contraceptive methods ensures informed decision-making. Providing emotional support, addressing concerns about contraceptive efficacy, and involving the woman in developing a suitable plan contribute to effective management.

Q98: A teenage girl seeks family planning advice. Discuss the unique considerations and challenges in providing family planning counseling to adolescents. Outline the nursing assessments, interventions, and educational strategies you would implement to address the specific needs of teenage clients. (20 marks)

Answer: Adolescents face unique challenges in family planning, including confidentiality concerns and potential lack of experience in contraceptive decision-making. Nursing assessments involve creating a non-judgmental environment, assessing the adolescent's understanding of reproductive health, and exploring her goals and preferences.

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Interventions include discussing contraceptive options, addressing concerns about confidentiality, and involving parents in a supportive manner if appropriate. Educational strategies involve explaining contraceptive methods, emphasizing the importance of consistent use, and providing information on reproductive health. Encouraging open communication, offering resources for further education, and considering the adolescent's emotional and social context contribute to effective family planning counseling for teenagers.

Q99: A couple expresses interest in fertility awareness-based methods for family planning. Discuss the principles of fertility awareness-based methods, their efficacy, and potential challenges. Outline the nursing assessments, interventions, and educational strategies you would implement to support the couple in using these methods effectively. (20 marks)

Answer: Fertility awareness-based methods involve tracking menstrual cycles, basal body temperature, and cervical mucus to identify fertile and infertile periods. These methods require consistency and understanding of the menstrual cycle.

Nursing assessments involve evaluating the couple's understanding of fertility awareness, assessing the regularity of the woman's menstrual cycle, and identifying potential challenges. Interventions include providing education on fertility awareness principles, offering tools such as fertility charts, and discussing the importance of consistent tracking.

Educational strategies involve explaining the limitations and potential for user error, providing guidance on accurate tracking, and discussing the importance of abstaining or using backup methods during fertile periods. Continuous support, addressing challenges, and involving both partners in the learning process contribute to effective use of fertility awareness-based methods.

Q100: A woman with a history of migraines seeks family planning advice. Discuss the considerations and potential contraindications for contraceptive methods in women with a history of migraines with aura. Outline the nursing assessments, interventions, and counseling strategies you would implement to guide this woman in making an informed choice about contraception. (20 marks)

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Answer: Women with migraines with aura are at an increased risk of ischemic stroke, and certain contraceptive methods may pose additional risks. Nursing assessments involve obtaining a detailed migraine history, assessing the woman's overall health, and discussing her reproductive goals.

Interventions include reviewing potential contraindications for hormonal contraceptives, assessing alternative methods, and collaborating with healthcare providers to ensure safe choices. Counseling strategies involve discussing the importance of non-hormonal methods, emphasizing the need for regular health check-ups, and addressing any concerns the woman may have. Informed decision-making, continuous monitoring for potential complications, and involving the woman in care decisions contribute to a comprehensive family planning approach for women with migraines with aura.

Subject 21: Conditions in Pregnancy

Q101: A pregnant woman presents with gestational diabetes mellitus (GDM). Discuss the pathophysiology of GDM, its potential impact on maternal and fetal health, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this condition. (20 marks)

Answer: Gestational diabetes mellitus (GDM) involves glucose intolerance during pregnancy. It can lead to maternal and fetal complications, including macrosomia and neonatal hypoglycemia. Nursing assessments include monitoring blood glucose levels, assessing maternal and fetal well-being, and obtaining a detailed health history. Interventions involve dietary modifications, insulin therapy if necessary, and promoting regular physical activity. Collaboration with dietitians, endocrinologists, and obstetricians is crucial. Continuous monitoring for glycemic control, addressing potential complications, and involving the woman in a personalized care plan contribute to effective management.

Q102: A pregnant woman is diagnosed with preeclampsia. Discuss the pathophysiology of preeclampsia, its potential impact on maternal and fetal health, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this hypertensive disorder of pregnancy. (20 marks)

Answer: Preeclampsia involves hypertension and organ dysfunction during pregnancy. It can lead to maternal and fetal complications, including eclampsia and fetal growth restriction. Nursing assessments include monitoring blood pressure, assessing for signs of organ dysfunction, and

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obtaining a detailed health history. Interventions involve antihypertensive medications, monitoring for potential complications, and promoting rest. Collaboration with obstetricians, monitoring fetal well-being, and providing emotional support are crucial. Continuous assessment, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q103: A pregnant woman presents with placenta previa. Discuss the pathophysiology of placenta previa, its potential impact on maternal and fetal health, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this placental disorder during pregnancy. (20 marks)

Answer: Placenta previa involves the implantation of the placenta over the cervical os. It can lead to maternal hemorrhage and fetal distress. Nursing assessments include monitoring for vaginal bleeding, assessing maternal and fetal well-being, and obtaining a detailed health history. Interventions involve bed rest, monitoring for potential complications, and preparing for potential cesarean section. Collaboration with obstetricians, continuous fetal monitoring, and addressing emotional concerns are crucial. Continuous monitoring for bleeding, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q104: A pregnant woman is diagnosed with hyperemesis gravidarum. Discuss the pathophysiology of hyperemesis gravidarum, its potential impact on maternal health, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this severe form of nausea and vomiting during pregnancy. (20 marks)

Answer: Hyperemesis gravidarum involves severe nausea and vomiting during pregnancy, leading to dehydration and weight loss. Nursing assessments include monitoring fluid and electrolyte balance, assessing nutritional status, and obtaining a detailed health history. Interventions involve intravenous hydration, antiemetic medications, and nutritional support. Collaboration with dietitians, addressing potential complications, and involving obstetricians for further evaluation are crucial. Continuous monitoring for improvement, providing emotional support, and involving the woman in care decisions contribute to effective management.

Q105: A pregnant woman presents with iron-deficiency anemia. Discuss the causes of iron-deficiency anemia during pregnancy, its potential impact on maternal and fetal health, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this condition. (20 marks)

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Answer: Iron-deficiency anemia during pregnancy can result from inadequate iron intake or absorption. It can lead to maternal fatigue and fetal growth restriction. Nursing assessments include monitoring hemoglobin levels, assessing for signs of anemia, and obtaining a detailed health history. Interventions involve iron supplementation, dietary counseling, and monitoring for potential complications. Collaboration with dietitians, addressing potential complications, and involving obstetricians for further evaluation are crucial. Continuous monitoring for improvement, providing emotional support, and involving the woman in care decisions contribute to effective management.

Subject 22: Abnormal Labour

Q106: A primigravida woman in active labor presents with persistent occiput posterior position. Discuss the potential complications associated with malposition in labor, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this abnormal labor presentation. (20 marks)

Answer: Occiput posterior position can lead to prolonged labor and increased maternal discomfort. Nursing assessments involve monitoring fetal heart rate, assessing progress in labor, and obtaining a detailed health history. Interventions include encouraging position changes, providing pain relief, and assisting with pelvic exercises. Collaboration with obstetricians for potential interventions like manual rotation, continuous monitoring for signs of improvement, and addressing maternal concerns are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q107: A multiparous woman presents with shoulder dystocia during labor. Discuss the potential complications associated with shoulder dystocia, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this obstetric emergency. (20 marks)

Answer: Shoulder dystocia can lead to fetal hypoxia and brachial plexus injuries. Nursing assessments involve monitoring fetal heart rate, assessing for signs of complications, and obtaining a detailed health history. Interventions include McRoberts maneuver, suprapubic pressure, and maneuvers to release impacted shoulders. Collaboration with obstetricians for potential interventions, continuous monitoring for signs of improvement, and addressing maternal concerns are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

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Q108: A pregnant woman with a previous cesarean section expresses a desire for a vaginal birth after cesarean (VBAC). Discuss the potential benefits and risks of VBAC, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to support a woman attempting VBAC. (20 marks)

Answer: VBAC offers the potential for vaginal delivery, reducing the risks associated with repeat cesarean sections. Nursing assessments involve reviewing the woman's medical history, assessing uterine scar integrity, and obtaining a detailed health history. Interventions include continuous fetal monitoring, providing emotional support, and assisting with pain management. Collaboration with obstetricians for continuous monitoring, addressing potential complications, and involving the woman in decision-making are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective support for VBAC.

Q109: A pregnant woman in labor is diagnosed with uterine rupture. Discuss the potential complications of uterine rupture, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this life-threatening obstetric emergency. (20 marks)

Answer: Uterine rupture can lead to maternal hemorrhage, fetal distress, and neonatal complications. Nursing assessments involve monitoring vital signs, assessing for signs of shock, and obtaining a detailed health history. Interventions include preparing for emergency cesarean section, providing blood transfusions, and continuous fetal monitoring. Collaboration with obstetricians for potential surgical interventions, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q110: A pregnant woman presents with precipitous labor. Discuss the potential complications associated with precipitous labor, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this rapid labor presentation. (20 marks)

Answer: Precipitous labor can lead to maternal and fetal trauma. Nursing assessments involve monitoring for signs of rapid progression, assessing maternal and fetal well-being, and obtaining a detailed health history. Interventions include providing emotional support, assisting with pain

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management, and preparing for potential complications. Collaboration with obstetricians for potential interventions, continuous monitoring for signs of improvement, and addressing maternal concerns are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Subject 23: Baby at Risk

Q111: A premature infant is admitted to the neonatal intensive care unit (NICU) with respiratory distress syndrome (RDS). Discuss the pathophysiology of RDS, its potential complications, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this preterm neonate. (20 marks)

Answer: Respiratory distress syndrome (RDS) in preterm infants is due to insufficient surfactant production. Complications include hypoxemia and respiratory failure. Nursing assessments involve monitoring oxygen saturation, assessing respiratory effort, and obtaining a detailed health history. Interventions include administering exogenous surfactant, providing respiratory support, and monitoring for potential complications. Collaboration with neonatologists for ongoing management, addressing potential complications, and involving parents in the care plan are crucial. Continuous monitoring, providing emotional support, and involving parents in care decisions contribute to effective management.

Q112: A newborn is diagnosed with neonatal jaundice. Discuss the causes of neonatal jaundice, its potential complications, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this condition. (20 marks)

Answer: Neonatal jaundice can result from physiological factors or pathological conditions like hemolysis. Complications include kernicterus and neurodevelopmental issues. Nursing assessments involve monitoring bilirubin levels, assessing for signs of jaundice, and obtaining a detailed health history. Interventions include phototherapy, monitoring for complications, and addressing potential causes. Collaboration with pediatricians for ongoing management, addressing potential complications, and involving parents in the care plan are crucial. Continuous monitoring, providing education to parents, and involving them in care decisions contribute to effective management.

Q113: A newborn is diagnosed with congenital heart disease (CHD). Discuss the potential types of CHD, their impact on newborns, and outline the nursing assessments, interventions, and

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collaboration with healthcare providers you would implement to manage a newborn with CHD. (20 marks)

Answer: Congenital heart disease (CHD) includes conditions like ventricular septal defects. Impact varies, with potential complications like heart failure. Nursing assessments involve monitoring oxygen saturation, assessing for signs of heart failure, and obtaining a detailed health history. Interventions include providing respiratory support, administering medications, and monitoring for potential complications. Collaboration with pediatric cardiologists for ongoing management, addressing potential complications, and involving parents in the care plan are crucial. Continuous monitoring, providing emotional support, and involving parents in care decisions contribute to effective management.

Q114: A newborn presents with meconium aspiration syndrome (MAS). Discuss the pathophysiology of MAS, its potential complications, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this neonatal respiratory condition. (20 marks)

Answer: Meconium aspiration syndrome (MAS) occurs when a newborn inhales meconium-stained amniotic fluid. Complications include respiratory distress and pneumonia. Nursing assessments involve monitoring respiratory effort, assessing oxygen saturation, and obtaining a detailed health history. Interventions include providing respiratory support, administering antibiotics, and monitoring for potential complications. Collaboration with neonatologists for ongoing management, addressing potential complications, and involving parents in the care plan are crucial. Continuous monitoring, providing emotional support, and involving parents in care decisions contribute to effective management.

Q115: A newborn is diagnosed with hypoglycemia. Discuss the potential causes of neonatal hypoglycemia, its impact, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this condition. (20 marks)

Answer: Neonatal hypoglycemia can result from inadequate glucose production or utilization. Impact includes neurological issues. Nursing assessments involve monitoring blood glucose levels, assessing for signs of hypoglycemia, and obtaining a detailed health history. Interventions include providing glucose supplementation, monitoring for potential complications, and addressing potential causes. Collaboration with pediatricians for ongoing management, addressing potential complications, and involving parents in the care plan are crucial. Continuous monitoring,

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providing education to parents, and involving them in care decisions contribute to effective management.

Subject 24: Malposition and Malpresentation

Q116: A pregnant woman is admitted with a breech presentation. Discuss the potential complications associated with breech presentation, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this abnormal fetal presentation. (20 marks)

Answer: Breech presentation can lead to complications such as birth injuries and umbilical cord compression. Nursing assessments involve monitoring fetal heart rate, assessing for signs of cord prolapse, and obtaining a detailed health history. Interventions include preparing for potential cesarean section, assisting with external cephalic version if appropriate, and providing emotional support. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous monitoring, providing education on breech presentation, and involving the woman in care decisions contribute to effective management.

Q117: A pregnant woman is diagnosed with shoulder dystocia. Discuss the potential complications associated with shoulder dystocia, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this obstetric emergency. (20 marks)

Answer: Shoulder dystocia can lead to neonatal complications and maternal trauma. Nursing assessments involve monitoring fetal heart rate, assessing for signs of brachial plexus injuries, and obtaining a detailed health history. Interventions include McRoberts maneuver, suprapubic pressure, and maneuvers to release impacted shoulders. Collaboration with obstetricians for potential interventions, continuous monitoring for signs of improvement, and addressing maternal concerns are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q118: A pregnant woman is admitted with a face presentation. Discuss the potential complications associated with face presentation, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this abnormal fetal presentation. (20 marks)

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Answer: Face presentation can lead to prolonged labor and increased maternal discomfort. Nursing assessments involve monitoring fetal heart rate, assessing for signs of cervical trauma, and obtaining a detailed health history. Interventions include encouraging position changes, providing pain relief, and assisting with pelvic exercises. Collaboration with obstetricians for potential interventions, continuous monitoring for signs of improvement, and addressing maternal concerns are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q119: A pregnant woman is diagnosed with compound presentation. Discuss the potential complications associated with compound presentation, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this rare fetal presentation. (20 marks)

Answer: Compound presentation involves a hand alongside the presenting part, leading to potential complications such as birth injuries. Nursing assessments involve monitoring fetal heart rate, assessing for signs of cord prolapse, and obtaining a detailed health history. Interventions include preparing for potential cesarean section, assisting with external cephalic version if appropriate, and providing emotional support. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous monitoring, providing education on compound presentation, and involving the woman in care decisions contribute to effective management.

Q120: A pregnant woman is diagnosed with occiput posterior position. Discuss the potential complications associated with occiput posterior position, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this malposition in labor. (20 marks)

Answer: Occiput posterior position can lead to prolonged labor and increased maternal discomfort. Nursing assessments involve monitoring fetal heart rate, assessing for signs of cervical trauma, and obtaining a detailed health history. Interventions include encouraging position changes, providing pain relief, and assisting with pelvic exercises. Collaboration with obstetricians for potential interventions, continuous monitoring for signs of improvement, and addressing maternal concerns are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

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Subject 25: Obstetric Emergency

Q121: A pregnant woman presents with uterine rupture. Discuss the potential complications of uterine rupture, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this life-threatening obstetric emergency. (20 marks)

Answer: Uterine rupture can lead to maternal hemorrhage, fetal distress, and neonatal complications. Nursing assessments involve monitoring vital signs, assessing for signs of shock, and obtaining a detailed health history. Interventions include preparing for emergency cesarean section, providing blood transfusions, and continuous fetal monitoring. Collaboration with obstetricians for potential surgical interventions, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q122: A pregnant woman is admitted with an amniotic fluid embolism. Discuss the potential complications of amniotic fluid embolism, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this rare and life-threatening obstetric emergency. (20 marks)

Answer: Amniotic fluid embolism can lead to maternal respiratory distress, cardiovascular collapse, and disseminated intravascular coagulation. Nursing assessments involve monitoring respiratory status, assessing for signs of shock, and obtaining a detailed health history. Interventions include providing respiratory support, administering medications to support cardiovascular function, and continuous monitoring for complications. Collaboration with intensivists for ongoing management, addressing potential complications, and involving the woman's family in the care plan are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q123: A pregnant woman experiences eclampsia. Discuss the potential complications of eclampsia, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this hypertensive disorder of pregnancy. (20 marks)

Answer: Eclampsia can lead to seizures, maternal and fetal distress, and organ dysfunction. Nursing assessments involve monitoring blood pressure, assessing for signs of seizures, and

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obtaining a detailed health history. Interventions include administering antihypertensive medications, providing magnesium sulfate for seizure prophylaxis, and continuous monitoring for complications. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q124: A pregnant woman is diagnosed with placental abruption. Discuss the potential complications of placental abruption, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this obstetric emergency. (20 marks)

Answer: Placental abruption can lead to maternal hemorrhage and fetal distress. Nursing assessments involve monitoring vital signs, assessing for abdominal pain, and obtaining a detailed health history. Interventions include preparing for emergency cesarean section, providing blood transfusions, and continuous fetal monitoring. Collaboration with obstetricians for potential surgical interventions, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q125: A pregnant woman presents with an umbilical cord prolapse. Discuss the potential complications of umbilical cord prolapse, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this obstetric emergency. (20 marks)

Answer: Umbilical cord prolapse can lead to fetal hypoxia and distress. Nursing assessments involve monitoring fetal heart rate, assessing for signs of cord compression, and obtaining a detailed health history. Interventions include positioning the woman to relieve pressure on the cord, providing oxygen, and continuous fetal monitoring. Collaboration with obstetricians for potential interventions, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

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Subject 26: Abnormal Puerperium

Q126: A postpartum woman presents with signs of endometritis. Discuss the potential complications of endometritis, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum infection. (20 marks)

Answer: Endometritis can lead to maternal sepsis and delayed uterine involution. Nursing assessments involve monitoring vital signs, assessing for signs of infection, and obtaining a detailed health history. Interventions include administering antibiotics, providing pain relief, and continuous monitoring for complications. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q127: A postpartum woman is diagnosed with postpartum hemorrhage (PPH) due to uterine atony. Discuss the potential complications of PPH, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this life-threatening postpartum complication. (20 marks)

Answer: Postpartum hemorrhage can lead to maternal shock and organ failure. Nursing assessments involve monitoring vital signs, assessing for signs of hemorrhage, and obtaining a detailed health history. Interventions include administering uterotonics, providing uterine massage, and continuous monitoring for complications. Collaboration with obstetricians for potential surgical interventions, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q128: A postpartum woman is diagnosed with deep vein thrombosis (DVT). Discuss the potential complications of DVT, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum thrombotic event. (20 marks)

Answer: Deep vein thrombosis can lead to pulmonary embolism and long-term complications. Nursing assessments involve monitoring for signs of DVT, assessing limb circumference, and

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obtaining a detailed health history. Interventions include administering anticoagulants, providing compression therapy, and continuous monitoring for complications. Collaboration with hematologists for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q129: A postpartum woman is diagnosed with mastitis. Discuss the potential complications of mastitis, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum breast infection. (20 marks)

Answer: Mastitis can lead to abscess formation and impaired breastfeeding. Nursing assessments involve monitoring for signs of infection, assessing breast appearance, and obtaining a detailed health history. Interventions include administering antibiotics, promoting breastfeeding, and continuous monitoring for complications. Collaboration with lactation consultants for breastfeeding support, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q130: A postpartum woman is diagnosed with postpartum depression (PPD). Discuss the potential complications of PPD, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum mental health condition. (20 marks)

Answer: Postpartum depression can lead to impaired maternal-infant bonding and long-term mental health issues. Nursing assessments involve screening for depressive symptoms, assessing maternal-infant interactions, and obtaining a detailed health history. Interventions include providing emotional support, referring for counseling, and continuous monitoring for complications. Collaboration with mental health professionals for ongoing management, addressing potential complications, and involving the woman's support system in the care plan are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Subject 27: Normal Pregnancy

Q131: A pregnant woman in her third trimester presents with lower abdominal pain and vaginal bleeding. Discuss the potential causes of abdominal pain and bleeding in the third trimester, and

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outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this situation. (20 marks)

Answer: Causes may include placental abruption or preterm labor. Nursing assessments involve monitoring vital signs, assessing for signs of shock, and obtaining a detailed health history. Interventions include continuous fetal monitoring, preparing for emergency cesarean section, and providing emotional support. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q132: A pregnant woman presents with gestational diabetes mellitus (GDM). Discuss the potential complications of GDM, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this pregnancy complication. (20 marks)

Answer: Complications of GDM include macrosomia and neonatal hypoglycemia. Nursing assessments involve monitoring blood glucose levels, assessing for signs of complications, and obtaining a detailed health history. Interventions include dietary management, insulin administration if needed, and continuous monitoring for complications. Collaboration with endocrinologists for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q133: A pregnant woman is diagnosed with preeclampsia. Discuss the potential complications of preeclampsia, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this hypertensive disorder of pregnancy. (20 marks)

Answer: Preeclampsia can lead to maternal organ dysfunction and fetal growth restriction. Nursing assessments involve monitoring blood pressure, assessing for signs of complications, and obtaining a detailed health history. Interventions include administering antihypertensive medications, providing magnesium sulfate for seizure prophylaxis, and continuous monitoring for complications. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

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Q134: A pregnant woman with a history of preterm birth presents with symptoms of preterm labor. Discuss the potential complications of preterm labor, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this situation. (20 marks)

Answer: Preterm labor can lead to premature birth and neonatal complications. Nursing assessments involve monitoring contractions, assessing cervical changes, and obtaining a detailed health history. Interventions include administering tocolytics, providing corticosteroids for fetal lung maturity, and continuous monitoring for complications. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q135: A pregnant woman is diagnosed with Rh incompatibility. Discuss the potential complications of Rh incompatibility, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this condition. (20 marks)

Answer: Rh incompatibility can lead to hemolytic disease of the newborn. Nursing assessments involve monitoring maternal antibody levels, assessing fetal well-being, and obtaining a detailed health history. Interventions include administering Rh immune globulin, continuous monitoring for complications, and addressing potential fetal interventions. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Subject 28: Abnormal Puerperium

Q136: A postpartum woman is diagnosed with postpartum depression (PPD). Discuss the potential impact of PPD on maternal-infant bonding and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum mental health condition. (20 marks)

Answer: Postpartum depression can negatively impact maternal-infant bonding, leading to potential long-term consequences. Nursing assessments involve screening for depressive symptoms, assessing the quality of maternal-infant interactions, and obtaining a detailed health

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history. Interventions include providing emotional support, referring for counseling, and continuous monitoring for complications. Collaboration with mental health professionals for ongoing management, addressing potential complications, and involving the woman's support system in the care plan are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q137: A postpartum woman is diagnosed with postpartum hemorrhage (PPH) due to uterine atony. Discuss the potential complications of PPH, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this life-threatening postpartum complication. (20 marks)

Answer: Postpartum hemorrhage can lead to maternal shock and organ failure. Nursing assessments involve monitoring vital signs, assessing for signs of hemorrhage, and obtaining a detailed health history. Interventions include administering uterotonics, providing uterine massage, and continuous monitoring for complications. Collaboration with obstetricians for potential surgical interventions, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q138: A postpartum woman is diagnosed with deep vein thrombosis (DVT). Discuss the potential complications of DVT, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum thrombotic event. (20 marks)

Answer: Deep vein thrombosis can lead to pulmonary embolism and long-term complications. Nursing assessments involve monitoring for signs of DVT, assessing limb circumference, and obtaining a detailed health history. Interventions include administering anticoagulants, providing compression therapy, and continuous monitoring for complications. Collaboration with hematologists for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q139: A postpartum woman presents with signs of endometritis. Discuss the potential complications of endometritis, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum infection. (20 marks)

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Answer: Endometritis can lead to maternal sepsis and delayed uterine involution. Nursing assessments involve monitoring vital signs, assessing for signs of infection, and obtaining a detailed health history. Interventions include administering antibiotics, providing pain relief, and continuous monitoring for complications. Collaboration with obstetricians for ongoing management, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

Q140: A postpartum woman is diagnosed with mastitis. Discuss the potential complications of mastitis, and outline the nursing assessments, interventions, and collaboration with healthcare providers you would implement to manage this postpartum breast infection. (20 marks)

Answer: Mastitis can lead to abscess formation and impaired breastfeeding. Nursing assessments involve monitoring for signs of infection, assessing breast appearance, and obtaining a detailed health history. Interventions include administering antibiotics, promoting breastfeeding, and continuous monitoring for complications. Collaboration with lactation consultants for breastfeeding support, addressing potential complications, and involving the woman in care decisions are crucial. Continuous support, addressing potential complications, and involving the woman in care decisions contribute to effective management.

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